CHAPTER

6 Risk Assessment and Intervention: Suicide, Nonsuicidal Self-Injury, and Homicide

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PREVIEW

Suicide and homicide are viewed as preventable global public health problems. Suicides and homicides have a mental health impact on family members, friends, coworkers, neighbors, and members of extended social networks, and even the professional community struggles with the implications of these preventable deaths. Nonsuicidal self-injury (NSSI) has always been a concern, but only recently has it received a similar clinical focus and formal assessment to suicide and homicide. Even after inclusion in the DSM-5-TR, assessment of NSSI occurs less frequently or is more targeted in certain populations, like adolescents or persons diagnosed with borderline personality disorder. However, an NSSI assessment should also be part of all suicide risk assessments and is now required as a standard of care. Whether intervening before suicide, NSSI, or homicide (prevention), while occurring (intervention), or after it has occurred (postvention), professional counselors play a vital role in society's response to helping those in need of violence-free coping strategies, intervention based on sound practices, or, if necessary, the best and most productive short-term and long-term response to survivors of suicide or homicide. This chapter is explicitly for crisis counselors to learn how to be effective in responding to members of their communities touched by suicide, NSSI, and homicide.

LEARNING OBJECTIVES

Upon completion of this chapter, readers will be able to:

6.1 Describe the prevalence and rates of suicide, homicide, and NSSI.
6.2 Describe risk factors, protective factors, and warning signs of suicide.
6.3 Distinguish between acute and chronic suicidal risk.
6.4 Describe how counselors assess a client's suicidal risk as no, low, moderate, or high.
6.5 Describe effective questions and assessment strategies for suicide intervention.
6.6 Describe appropriate treatment planning and care management with a suicidal client, including a safety plan, outpatient and inpatient treatment, and postvention.
6.7 Describe assessments and treatment for NSSI, including the use of a Q-tree.
6.8 Describe threat assessment and treatment options for intervention with people who are homicidal or are threats to others.

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PREVALENCE AND RATES OF SUICIDE, HOMICIDE, AND NONSUICIDAL SELF-INJURY

Learning Objective 6.1 Describe the prevalence and rates of suicide, homicide, and NSSI.

Clinicians and epidemiologists alike watch closely the rates and prevalence of suicide and homicide to monitor the presence of conditions that may increase risk factors for certain populations. (Nonsuicidal self-injury historically was not included in these prevalence reports and more research needs to be conducted.) Counselors should know what the terms *prevalence* and *rate* mean to interpret how members of certain at-risk groups might require more attentive care:

- *Prevalence* is a nominal figure that states how many lives were lost to suicide (or any other type of malady). For example, the prevalence of suicide in County RST was 75 persons, contrasted with County XYZ, which lost 33 persons.
- *Rate* or *crude rate* is a value explaining data that has been interpreted by leveling the playing field, so to speak; in other words, if all members of a population (or an age group, gender, or ethnic group) were divided into groups of 100,000, the rate would indicate how many people died per 100,000 people (sometimes called per capita). For example, County RST, a very populated county, lost 75 persons to suicide or homicide, but its rate was 10.9 per 100,000, or 10.9. County XYZ, a small, sparsely populated county, lost 16 persons to suicide or homicide, but its rate was 19.7 per 100,000, or 19.7. Due to the common denominator of 100,000, the higher rate for County XYZ is considered more alarming, even though the prevalence of suicide was smaller in the rural county.

Epidemiologists also study the *causes of death* and the *rankings* of these causes, and they track the rates of suicide and homicide, as rates provide a more uniform view for analysis to which prevention efforts may be targeted. For example, the suicide rate for Americans age 65 years and older is approximately 18 in 100,000, but the prevalence is lower than in other groups because the population number (or size of this group) of older persons is smaller. A rate of 18 in 100,000 is considered alarming because it is higher than the national overall rate. For suicide, the national rate is 14.5 per 100,000. Professionals want to learn why persons in each group are more prone to die by suicide or homicide. Youth have a lower *rate* of death than older Americans; however, suicide is ranked as the second leading *cause of death ranking* among youth, and homicide is the sixth and third leading cause of death among 0 to 14-year-olds and 15 to 24-year-olds, respectively (Drapeau & McIntosh, 2020; World Life Expectancy, 2020). Cause of death ranking is often illustrated in chart form listing the 10 highest prevalence of deaths per cause (e.g., heart disease, malignancies, respiratory disease, liver disease, unintentional injury).

Drapeau and McIntosh (2020) reported an estimated 1,124,125 annual suicide attempts in the United States or 25 suicide attempts for every suicide. Cerel et al. (2014) estimated that for every suicide, 147 individuals (survivors) are connected to the person who died by suicide (6.6 million annually). When examining the vast effects of those affected by suicides, mental health professionals must be prepared to serve the high numbers of survivors.

Suicide is a public health problem that all crisis counselors, regardless of setting, must learn to assess and treat because of its prevalence in society. In the United States in 2020, suicide claimed 45,979 lives, with about one person dying from suicide every 11.1 minutes (Centers for Disease Control and Prevention [CDC], 2022a). Consider these data:

- In 2020 in the United States, 12.2 million adults experienced thoughts of suicide (CDC, 2022a).
- In 2020 in the United States, 3.2 million people planned to attempt suicide (CDC, 2022a).
- In 2020 in the United States, 1.2 million persons attempted and survived a suicide (CDC, 2022a).
- Though suicide happens in all age groups and is the 10th leading cause of death across all age brackets, it is the ninth leading cause of death in the United States among persons age 10–64, and the second leading cause of death for youth age 10–14 and adults age 25–34 (CDC, 2022a).
- Men die from suicide at a ratio of 3.6 for every 1 loss of life to suicide among women (American Association of Suicidology [AAS], 2021).
- Women attempt suicide more often than men, at a ratio of 3:1; men die more often than women, mainly because of more lethal means (AAS, 2021).
- The impact of suicide loss has staggering personal and economic impact on individuals and communities. The CDC (2022a) estimated an annual cost of $70 billion in the United States from lifetime medical and work-related expenses due to suicide attempts and suicide deaths.
- AAS (2021) estimated that 135 individuals are exposed to or affected by each suicide death, which is 6.9 million persons annually in the United States.
- The rates of suicide per 100,000 persons across the United States ranged from 6.2 per 100,000 (Washington, DC) to 29.4 per 100,000 (Wyoming) (Drapeau & McIntosh, 2020).
- Suicide rates have been increasing. From 2014 to 2019, the rates per 100,000 persons climbed respectively from 13.4, 13.8, 13.9, 14.5, to 14.8, with a slight decrease in 2019, to 14.5.

Furthermore, more than 1.2 million nonfatal suicide attempts occurred in the United States in 2020, sometimes resulting in permanent injury or disabling conditions and sometimes imperiling a concerned person who tried to prevent the suicide from happening (CDC, 2022a).

For a worldwide analysis of suicide deaths and homicide deaths, consult the World Health Organization (WHO, 2021c) and the World Population Review (2022). The number of suicides worldwide in 2019 was 703,000 (WHO, 2021c) and the number of homicides globally in 2017 was 464,000 (World Population Review, 2022).

Nonsuicidal self-injury (NSSI) behaviors are less often reported in the literature and often not the focus of a risk assessment unless it is the presenting problem. This does not mean the prevalence of NSSI is not occurring at alarming rates. NSSI is reported to occur among 13–17% of American adolescents and young adults (Swannell et al., 2014). Furthermore, NSSI relates to other mental health issues including depression, anxiety, posttraumatic stress disorder (PTSD), and suicidal risk. The recent inclusion of NSSI

behaviors in the fifth edition, text revision of the *Diagnostic and Statistical Manual of Mental Disorders* (DSM-5-TR; American Psychiatric Association, 2022) reveals the need for a thorough risk assessment that includes NSSI behaviors.

Effective risk prevention requires input from multiple systems and increased involvement from governments; the professions of mental health, public health, and medical health; law enforcement; schools, universities, and other institutions of learning; the media; and community leaders. Effective risk assessment also begins with the ability of the crisis counselor to initiate and ask direct questions about risk assessment. If the crisis worker does not bring up any of these issues or feels uncomfortable bringing these issues up, the client may not feel comfortable bringing up risk issues. The first step to providing a competent risk assessment is to be aware of comfortability in asking about risk assessment. Activity 6.1 provides structured practice in asking clients about suicide. You can also use the same activity to practice assessing NSSI and homicidal risk.

ACTIVITY 6.1

Form a group in which participants stand in two lines facing each other. At the direction of a leader, the participants in line 1 ask their partners in line 2, "Are you having thoughts of suicide?" The members of line 1 should then move to the right (or circle to the other end of the line if they are at the far-right end of the line) and ask the next partner the same question. Continue doing this until all line 1 members have asked everyone in line 2 the same question. Then repeat the activity, but this time, every member of line 2 now asks members in line 1, "Are you having thoughts of suicide?" After the exercise, discuss together as a group the following questions:

- Which role did you prefer: being the person asked or being the person doing the asking? Explain.
- Why do you think it is important to practice asking the question, "Are you having thoughts of suicide?"
- How do you feel about using the word *suicide* or about looking someone in the eye and asking directly about suicide?
- What are some other equally direct and nonjudgmental ways that you could ask about someone's intention to self-injure?
- What has been your experience with suicide, both personally and professionally? Consider what you do and do not know about working with a client in a suicidal crisis or emergency. What more do you need to know before you work with a client who is suicidal? What personal attitudes, values, opinions, or views do you have about suicide?

Suicide Terminology

The use of terminology is important when discussing suicide since the language chosen could reflect personal attitudes and affect those impacted by suicide. For example, the phrase "committed suicide," although once common, is less acceptable as it implies a negative perspective of the person. Compare the phrase to "committed adultery" or "committed murder," as they often carry a negative view of the person who participated in or "committed" the act. If we define a person solely by the label of a mental illness, we diminish the whole person.

In comparison to other types of medical deaths, we often refer to the cause of death rather than placing responsibility on the person, such as "cancer-related death" or "died of cancer." Following these guidelines, language such as "died by suicide," "suicide death," or "took their own life" are more acceptable phrases as they offer a more humanistic approach to those grieving the death of a suicide and those who may

TABLE 6.1 Acceptable Language to Describe Suicide and Language to Avoid

Acceptable Language	Language to Avoid
Died by/of suicide	Committed/Completed suicide
Suicide death	Successful attempt
Suicide attempt (attempt suicide)	Unsuccessful attempt
Person experiencing suicidal thoughts or behaviors	Suicide attempter

have attempted or taken their own life. Terminology now considered unacceptable has been used for decades by professionals, so be patient and use this as a time to provide educational guidance to those in the field who may still use what now is viewed as unacceptable terminology. Professional literature will still include what is now viewed as unacceptable language. Try using the examples in Table 6.1 of "acceptable language" in place of what is now viewed as "unacceptable language" around suicide.

Many individuals are impacted by the loss of a loved one from suicide, so it is essential to understand correct terminology when referring to those impacted by suicide and with other professionals. *Primary victims* are the individuals who took their own life or died by suicide. *Attempt survivors* are those persons who have attempted to take their own life but did not die. *Survivors* are loved ones affected by the loss of a person who died by suicide.

RISK FACTORS, PROTECTIVE FACTORS, AND WARNING SIGNS

Learning Objective 6.2 Describe risk factors, protective factors, and warning signs of suicide.

Most often, counselors attempting to provide a thorough risk assessment for suicide will assess and report only on the risk factors of a client experiencing thoughts of suicide or who has attempted suicide. Although this is an essential part of the suicide assessment, protective factors are just as clinically essential to explore because they offer insights into what is actually protecting or preventing the individual from taking their life. So specifically ask, "What has kept you from following through on taking your life?"

Counselors should discern these protective factors from the overall assessment and build them into the care management aspect of treatment. If a counselor only focuses on the risk factors and does not integrate the protective factors, the level of risk and lethality may not be accurate. Risk and protective factors are individualized and depend on unique client circumstances. Therefore, no single risk or protective factor can be used across clients to indicate certain lethality and risk. Rather, the entire assessment profile helps a counselor determine the most effective client care management action.

Risk Factors

Risk factors increase the likelihood of a person experiencing suicidal thoughts or behaviors. Risk factors are often seen as the absence of something (e.g., supports, medication compliance, medical wellness, loss of loved ones) but may include underlying

meaning-related factors (e.g., feelings of guilt/shame, abandonment, being a burden) that often increase risk lethality.

Many special groups have a heightened risk for suicide. These include Native Americans; members of the military or veterans; first responders; older persons; persons with life-threatening depression and/or bipolar disorder; victims of bullying; and youth who are lesbian, gay, bisexual, transgender, questioning, and other (LGBTQ+). For these groups, suicide death rates are higher than average for the population. It is important to note that a certain group does not have traits that make them more suicidal; rather, other external determinants elevate the risk that members of some groups may become self-violent. These factors may include marginalization, trauma events or complex trauma, poverty, and exposure to self-violence or violence, among other factors.

Although agencies and clinicians may have different ways to inquire about risk factors, for the following discussion, risk factors are placed into three main categories: health factors, environmental factors, and historical factors. These categories may have some factors in common.

HEALTH FACTORS Health factors include medical or mental health concerns. Often, physical illnesses such as a terminal disease may also affect a person emotionally and existentially. Thus, such illnesses may become complex risk factors. For example, an individual with cancer may not be able to work, may become dependent on others, and this may affect their family (e.g., financially or via role changes). In addition, the individual may perceive they have become a burden to family members for their care, which may be a significant change from their previous role of taking care of others. Therefore, it is important that counselors understand not only the surface risk factor (medical diagnosis) but also other factors that may complicate the risk (e.g., financial, job status, perception of being a burden on others, independence).

Some mental health concerns may also impact the risk for an individual. Often when the client is not able to think through rational situations or is impulsive, a higher risk of acting on suicidal thoughts exists. Mood disorders, schizophrenia, anxiety disorders, substance use, and certain personality disorders may also present a higher risk for suicidal behaviors.

ENVIRONMENTAL FACTORS Environmental factors can include cultural, behavioral, and personality issues. Cultural issues include, but are not limited to, those relating to race/ethnicity, gender identity, socioeconomic status, political affiliation, religious/spiritual affiliation, sexual orientation/identity, age, and sex. Other environmental factors may include, but are not limited to:

- Hopelessness
- Impulsive or aggressive tendencies
- Family history of suicide
- Recent job or financial loss
- Recent loss of relationship
- Perceived rejection associated with personal meaning
- Access to lethal means
- Lack of social support and sense of isolation
- Stigma associated with asking for help
- Lack of health care, especially mental health and substance use disorder treatment

- Cultural and religious beliefs, such as the belief that suicide is a noble resolution of a personal dilemma
- Exposure to others who have died by suicide (in real life or via the media)
- Ability to be a part of one's personal rescue plan from past suicide thoughts/behaviors

HISTORICAL FACTORS Historical factors are unique to the client's life and include, but are not limited to, history of trauma or abuse, abandonment of social supports, family-of-origin risk factors, previous suicide attempts, family history of suicide, or humiliating past events that are still causing pain for the client.

Protective Factors

Protective factors make a client less likely to engage in suicidal behavior. Protective factors can also promote resilience and ensure connectedness with others during difficult times, thereby making suicidal behaviors less likely. Often, the factors listed as risk factors for some clients may also be protective factors. For example, a client's religious beliefs may be the reason the client is not following through on the plan to die by suicide. Consider this statement made by a client, "I have not followed through because I do not believe I would be joined with my other family members in heaven." A counselor would not say, "Yes, according to your beliefs, you may not go to heaven." Rather, a counselor may respond, "It sounds like being with your family after death is important to you. Tell me more about your religious/spiritual beliefs and the importance of being reunited with loved ones."

Another protective factor may be a single mother who says, "I have not acted on my suicidal thoughts because I would hate for the father of our children, who has been abusive to me, to be given custody and abuse the kids. I cannot do that to my children." Her desire to make sure her children are cared for is a protective factor. A counselor would not reply with, "Yes, that would be quite selfish to have your children experience the abuse you did because you decided to kill yourself." Rather, a response to support the protective factor may be, "It sounds like you do not want your children to experience the same abuse from their father that you did, and you are concerned that if you took your own life, they would not be cared for the way you want to take care of them. Tell me more about your children and what you want for them."

Other protective factors include, but are not limited to:

- Hopefulness
- Restricted access to highly lethal means of suicide
- Strong connections to family, friends, social, and community supports
- Skills in problem solving, conflict resolution, handling problems in a nonviolent way, and lack of impulsivity in past situations
- Ability to see alternatives, limited cognitive distortions, and less impulsivity
- Cultural or religious beliefs that discourage suicide and support self-preservation
- Medication compliance
- History of ability to solve problems and create solutions
- History of demonstrating skills for handling emotional crises

- Expression of concrete and detailed future plans
- Family commitments, such as raising children or caring for siblings
- Willingness to sign a no-harm contract
- Being resilient in past situations
- Having a sense of purpose or meaning

Warning Signs

Warning signs may be present prior to death by suicide but not recalled until after the fact. Survivors may experience guilt in not intervening in the suicide, which may cause more complicated grief issues. Other clients may not present with warning signs at all. Often, individuals who do not present with warning signs are those who truly want to die and do not want any intervention. In addition, some warning signs may be normal behavior for the person who died and therefore may not be seen as a warning sign by family, friends, and counselors.

Some warning signs may fall into risk factors. Aside from assessment of risk, counselors should be aware of various warning signs that a client may present and provide psychoeducation and information to family members and friends (as appropriate), such as including possible things to look for when they know their loved one has indicated they are experiencing thoughts of suicide. The following are common warning signs of suicide that may lead to acute suicidal thoughts or behaviors requiring immediate intervention (Rudd et al., 2006).

- Talking about wanting to die
- Researching methods of dying, preparations, or rehearsal of taking one's own life
- Talking about feeling hopeless or having no purpose; self-hate statements
- Talking about feeling trapped or being in unbearable pain
- Talking about being a burden to others; recent hospital discharge
- Increasing use of alcohol or drugs
- Acting anxious, agitated, impulsive, or reckless, especially if this is not the norm
- Sleeping too little or too much
- Withdrawing or feeling isolated
- Showing rage or talking about seeking revenge
- Displaying extreme mood swings
- Cognitive, behavioral, or mood changes

ACUTE VERSUS CHRONIC SUICIDE RISK

Learning Objective 6.3 Distinguish between acute and chronic suicidal risk.

Acute suicidal risk is defined as an individual who reports or presents with a desire to die in a foreseeable time frame—usually 24 hours or less—with statements about a specific plan, intent, means, and method (Kleespies, 2000). Some counselors may allow for a longer time period, but there is still a specific time frame. For example, a client who has been experiencing chronic and complex grief may state, "I just cannot get over my mother's death. On Mother's Day this year, I plan to end it all so I can be with my mother." It would still be appropriate to consider this as an acute risk situation and to take appropriate measures to start the preventative suicidal assessment and intervention.

Acute crisis refers to situations that require immediate professional intervention. For example, Jobes (2016) concluded there is a strong relationship between acute risk and acute crisis events, such as personal failures that may impact core meaning or situations that are perceived as humiliating to the client. Any recent events need to be assessed to determine whether the client perceives the events as humiliating. These acute crises alone may increase the suicidal risk and lethality to a high level, resulting in immediate and imminent risk. Examples include demotions at work, a relationship breakup, legal situations, discovering a betrayal, foreclosure or encountering adverse financial situations, educational failure, and medical issues that may be perceived to cause a financial burden on family members. Often, a humiliating event can cause an individual to not think clearly and, combined with impulsive personalities, can trigger suicidal behaviors.

Chronic suicidal risk is often connected with long-term patterns of relationship difficulties, interpersonal conflicts, poor decision making in various aspects of life, and repetitive destructive patterns causing life situation difficulties. These chronic risk situations may be associated with chronic suicidal thoughts and possibly chronic behaviors, but individuals may have less specific plans and less lethal intent or means. Thoughts of suicide may last over long periods of time and may not appear as imminent or immediate. Chronic risk must be assessed for the possible emergence of active or acute risk.

Individuals with chronic risk can be described as having "markers" of a potential suicide risk that is not currently imminent. In other words, individuals may present with some identifiers of persons who *could* become acutely suicidal but are not currently in imminent danger. Just as with other medical diagnoses, the client in your office with a family history of cancer (a risk factor) may not ever be stricken with cancer. A person who is chronically in suicide risk may thus have markers or risk factors for potential suicide, but there is a chance the person will never attempt suicide. Chronic risk should be observed closely for the possible emergence of active or acute risk. A few of these chronic markers might include the following:

- Being from a high-risk suicidal group
- Diagnoses including but not limited to mood disorders (depression or bipolar disorder), anxiety, eating disorder, thought disorder, substance use disorder or some personality disorders
- History of trauma, abuse, violence, self-injury, previous hospitalizations, low self-esteem, high self-hate, tolerance and acceptance toward suicide, oppression related to sexual orientation, perfectionism, divorced parents, family history of suicide attempts or fatalities, and other factors

In summary, chronic risk suggests the presence of longer-term potential of suicidality, but with less specificity of intent or plans (e.g., means or methods) to carry out an imminent attempt. The counselor has more time to help the client mitigate, manage, or resolve psychological pain (and the need to end the pain), can be less directive, and may not resort to immediate protection such as hospitalization as a treatment strategy. The counselor will instead reassess suicidality regularly, manage safety, and attempt to stabilize long-term risk with effective counseling. Table 6.2 provides further differentiation between the acute and chronic risk categories.

TABLE 6.2 Acute Risk versus Chronic Risk

	Acute Risk	Chronic Risk
Descriptors	Emergent, urgent, imminent behavioral indicators and warning signs; could lessen and possibly become chronic	More long-running risk, grounded in risk factors; could be precrisis state, in crisis, or transcrisis; may or may not become acute
Time frame for potential attempt	Hours, 24 hours to perhaps a few days	Days, weeks, months, years
Ambivalence	Little to none; believes death is the answer to ending the pain	Some to considerable ambivalence (e.g., 50-50); wants to live as much as seeking death to end pain
Clinical response	Clinician advised to get help for client immediately, arrange heightened vigilance, not let the client remain alone, provide emergency intervention, and use directive style	Clinician advised to monitor risk frequently and regularly, be aware that hospitalization may not be required unless risk becomes acute, develop and maintain therapeutic alliance, perhaps meet more frequently, prepare a safety plan, provide support, and help the client develop coping skills, problem-solving skills, and life skills
Likely trigger or precipitant*	A sudden or recent event, loss, humiliation, defeat, embarrassment, trapped feeling	Likely to be relationship based; risk can rise to acute with a sudden or recent event, loss, humiliation, or defeat

* See Jobes (2016) for additional information.

LEVEL OF SUICIDAL RISK

Learning Objective 6.4 Describe how counselors assess a client's suicidal risk as no, low, moderate, or high.

The gathering of information regarding risk factors, protective factors, warning signs, and acute versus chronic risk will likely render a decision connected with lethality. Most counselors assess a client's suicidal risk on a continuum of low, moderate, or high:

- Low suicidal risk includes the presence of suicidal ideation/behaviors or infrequent thoughts without a plan and means to carry out the plan.
- Moderate suicidal risk includes serious thoughts and desire to die with ideas of a plan and means, often no access, some existence of social support, but also some risk factors.
- High suicidal risk includes details of a plan and means to carry out the plan, presence of risk factors, and absence of protective factors including support.

Although many scales indicate low, moderate, and high, there should also be a note if there is no (or none) suicidal risk detected. Indicating "no or none" is a best practice and allows you and other referring counselors to have a better comparison of suicidal thoughts and behaviors over time.

Table 6.3 presents a sample rubric from an AAS-accredited crisis center utilized for crisis calls and distance counseling to assess the level of suicidal risk of callers

TABLE 6.3 Suicide Assessment Form – Checklist for Assessing Emergency Risk for Suicide

Indicators/ Risk Levels	None	Very low risk	Mild/ low risk	Moderate risk	Severe/ high risk	Extremely high risk
Method(s) Considered	None	Vague, no plan	Pills, slash wrist	Drugs, alcohol, car wreck, some specifics	Gun, hanging, jumping; some specifics	In progress; gun, hanging, jumping; very specific—knows how, when, where
Suicidal Plans	None	None	None	Some but no clear intent	Developed plan with considerable detail	Well-thought-out, lethal plans
Availability of Means	None	No plan; no availability of means	Not available—will have to get	Available but not close by	Available—has close by	Has in hand or in progress
Suicidal Behavior	None	No current ideation; may report past ideation	Ideation	Threat	Attempt low lethal	Attempt high lethal
When Attempt is Planned	None	No attempt planned	48 hours or more	24 to 48 hours	24 to 18 hours	Presently (in progress) or within 24 hours
Previous Suicide Attempt(s)	None	None	None	Yes (1–5 years ago)	Yes (3 months to 1 year ago)	Yes (3 months ago or less)
Feelings of Loneliness	None	Hardly ever	Sometimes, support system present	Usually, some support system	Always, limited support system	Always, no support system
Hope/ Level of Ambivalence	Hopeful	Hopeful; readily acknowledges desire to live	Some hope, aware of some desire to live	Hope and some desire to live present, but inconsistent or limited	Little or no hope for future; does acknowledge some ambivalence	No hope; does not consciously acknowledge any ambivalence
Intoxication (Use of Alcohol or Drugs)	None	Has not been drinking or using drugs	Limited use of alcohol	Limited use of alcohol or drugs; history of substance abuse treatment	Intoxicated, mixing drugs and alcohol	Mixing drugs and alcohol and evidence of intoxication
Chance of Intervention	Has clear supports	Others present	Others expected	Others expected or available	Others available	No one nearby; isolated

Source: Based on the Crisis Center, Birmingham, Alabama.

experiencing suicidal thoughts. Law enforcement "hostage negotiators" also use variations of this guide for crisis response teams.

In addition, the National Suicide Prevention Lifeline (NSPL; 2007) recommends following a rubric of assessing suicide risk based on the individual's desire, capability to attempt, and intent to die or attempt suicide.

Suicidal *desire* is indicated by:

- Desire to harm self or others
- Psychological pain such as hopelessness, helplessness, or feeling trapped
- Perceiving oneself to be a burden on others

Suicidal *capability* is evidenced by:

- History of suicide attempts
- Exposure to someone else's death by suicide
- History of or even current violent behaviors toward oneself or others
- Having the means to hurt oneself or others
- Current intoxication or excessive substance use
- Recent dramatic mood changes or loss of orientation to reality
- Extreme agitation, rage, increased anxiety, decreased sleep, and recent acts or threats of aggression

Suicidal *intent* is reflected by

- Attempt in progress
- Plan with a known method to hurt oneself or others
- Preparatory behaviors (e.g., giving away possessions, finalizing one's business affairs, saying goodbye [perhaps cryptically], counting pills, reading up on how to use the method, loading and reloading the gun, rehearsing the method) and stated intent to die

Now see the various scenarios in Case Study 6.1, which asks readers to assess potentially suicidal cases.

CASE STUDY 6.1

Potentially Suicidal Cases

After reading the following brief potentially suicidal cases, indicate the risk factors (health, environmental, historical), protective factors, warning signs, and other relevant information and indicate whether you believe the suicidal risk is acute or chronic or no, low, moderate, or high risk.

Case A

Kelson is a 22-year-old who has recently spent 27 months in a conflict zone. Kelson is headed home for an extended furlough and fidgets and is agitated on the 18-hour plane ride home. Kelson arrives home to discover their former partner is now involved with a high school friend. In addition, Kelson's dad has remarried a woman whom Kelson has never met. The marriage occurred when Kelson had been deployed for just 4 months. Kelson's new stepmother has moved into the house where Kelson lived before

deployment. Kelson's younger brother, in the meantime, graduated from high school and is now in college two states away. Kelson goes on a drinking binge the first night home and gets into a scuffle at the bar. As a soldier, Kelson knows how to use firearms.

Risk factors: _____

Protective factors: _____

Warning signs: _____

Other relevant information: _____

Acute risk or chronic risk: _____

No, low, moderate, or high risk: _____

Case B

Devon is a Black college student who has learned to cope by "doing more." If having one part-time job is good, then two part-time jobs are better. Devon is prone to getting overinvolved in extracurricular activities, thinking that overachievement will lead to being selected for preferred groups and clubs in college. Devon has been seeing a counselor due to lack of focus on class. In addition, Devon's grades are not quite good enough to meet the requirements of Devon's desired activities. Devon's counselor asked about depression. Devon was bullied as a teenager by others who mocked Devon's physical features, and admitted to lifelong gruff treatment by a step-father. Devon is bright and likeable but does not possess that self-view. Devon cannot explain all the crying and self-pity because of all the hard work to "keep it together." Devon feels like a burden to everyone and has daydreamed about taking a bunch of pills.

Risk factors: _____

Protective factors: _____

Warning signs: _____

Other relevant information: _____

Acute risk or chronic risk: _____

No, low, moderate, or high risk: _____

Case C

Charles is a White, 77-year-old retiree. During retirement, he plays a lot of tennis and spends time at his hunting cabin with friends. One of his long-time tennis buddies of over 30 years died suddenly from a heart attack on the tennis court only minutes after Charles had left the club. In addition, Charles's wife has been diagnosed with lymphoma and is undergoing treatment. While she is doing well despite the fatigue of the treatments, he is very worried about the possibility of her death before his own, and since his adult children have busy lives and reside in another region, he is primarily the sole caregiver for his wife. Together, they have decided to not get out and socialize as much as they had formerly, and they seem to be more socially isolated, except for Charles's occasional tennis matches. On a recent visit to his physician, the doctor became worried and asked him if he was thinking of suicide.

Risk factors: _____

Protective factors: _____

Warning signs: _____

Other relevant information: _____

Acute risk or chronic risk: _____

No, low, moderate, or high risk: _____

Case D

Jessie is a 34-year-old Native American who has struggled with substance use for many years. Because Jessie feels that drinking is a gesture of loyalty to friends, abstaining or understanding the risks of chemical dependence is difficult. Jessie's father died by suicide when Jessie was 16 years old; as the oldest child, Jessie became the head of the family. Last night, Jessie got a second drunk-driving charge so is in a panic about the shame and embarrassment that this will bring on the family. Jessie told a friend about going to the cliffs to "fly with the eagles."

Risk factors: _____

Protective factors: _____

Warning signs: _____

Other relevant information: _____

Acute risk or chronic risk: _____

No, low, moderate, or high risk: _____

Case E

Sandy is a 47-year-old White man who has just finalized a divorce from his third partner. For most of his life, he has placed a high premium on having a relationship with a partner, yet his commitments and marriages have seemed rushed and not very thought out, according to some friends and relatives. Sandy is devastated when things do not go well, and his pattern of relationships has been somewhat chaotic and often conflicted. Similarly, he has seen several counselors over the years. Sandy's current counselor, in talking to his most recent prior counselor, learned that he has never attempted suicide actively but has often talked about wanting to die "if it happened." Sandy is not sure how he would do it because he is afraid. He is an adult survivor of childhood sexual abuse. He is an active member of his church and does not think it would be "okay" to attempt suicide.

Risk factors: _____

Protective factors: _____

Warning signs: _____

Other relevant information: _____

Acute risk or chronic risk: _____

No, low, moderate, or high risk: _____

The field has produced a variety of acronyms to assist counselors in recalling risk and protective factors and warning signs during suicide assessments. These should not serve to replace a formal model and Q-tree (which we discuss next) but may be helpful in gathering information on risk and protective factors. The following is not an exhaustive list but provides examples:

- IS PATH WARM (Juhnke et al., 2003): Ideation, Substances, Purpose, Anxiety, Trapped, Hopelessness, Withdrawal, Anger, Reckless, and Mood
- SIMPLE STEPS (McGlothlin, 2008): S—Are you Suicidal?, I—Ideation, M—Means, P— Perturbation, L—Loss, E—Earlier attempts, S—Substance abuse, T—Troubleshooting the client's problem-solving ability, E—Emotion (hopeless, hapless, worthless, loneliness, depression), P—Parents or family history/exposure, S—Stress and life events
- MAN THIS ISN'T FAIR (Sankaranarayanan, 2007): M—Mental issues (depression, anxiety, agitation, guilt, shame, delusions), A—Attempts at self-destruction, N—No positive support from family/peers, T—Triggering stressors, H—Hopelessness, I—Ideas and intent, S—Substance abuse, I—Illness/chronic pain, S—Suicide in family history, N'T—(suicide) Note, F—Final arrangements/saying goodbye/giving away possessions, A—Access to means, I— Isolation, R—Recent psychiatric hospitalization

The above is only a guide to the factors that may affect risk and lethality. There is no set number of indicators someone may have to determine lethality, and a single indicator may not constitute higher risk. In other situations, an individual may have many indicators but may not be high in lethality. In still others, a few indicators together may be essential enough to qualify for high risk. For example, if a client says they have a plan and means, have no hope, and will follow through on their plan without seeking intervention, then the individual may certainly have a high suicidal risk. There are many assessments in the field that can be used to quantify the variables connected with suicide.

EFFECTIVE QUESTIONS AND ASSESSMENT STRATEGIES FOR SUICIDE INTERVENTION

Learning Objective 6.5 Describe effective questions and assessment strategies for suicide intervention.

A suicide risk assessment should always begin at the first session in counseling, and it should be the main assessment goal in any crisis. The counseling field has a plethora of assessments and models for assessing suicide risk. A standard of care in our profession is to not only provide a risk assessment of every person, regardless of the client's presenting problem, but also have a framework (model) upon which to base the assessment. Although the profession does not dictate a specific suicide assessment to use, it is a standard of care to use effective risk assessment to practice ethically and reduce negligence.

An effective suicide risk assessment relies heavily on clinical judgment and interpretation of available facts, indicators, evidence of past behaviors, rating of current mood and symptoms, and consideration of all these data against the backdrop of the field's knowledge of risk factors, protective factors, epidemiology, and causative factors that are known to increase the risk of suicidal death. Regardless of the model used, there are a set of questions that should be integrated to weigh the risk and protective factors and make a sound clinical impression of lethality to provide a treatment plan.

Question-Tree Approach for Suicide Assessment: Essential Questions

Regardless of the assessment used, certain questions should be part of the suicide assessment. Depending on the answers, follow-up questions should be asked for a comprehensive assessment. If a counselor only stays with a surface question and does not follow up with additional questions, the assessment is not complete and the treatment plan may be ineffective or inaccurate, leading the counselor to an incorrect lethality level. Using a question-tree (Q-tree) approach allows the counselor to weigh the risk and protective factors and formulate the most appropriate plan for a client experiencing suicidal thoughts or behaviors (see Figure 6.1).

Notice that the risk and protective factors are included in the Q-tree. In addition, some indicators may be difficult to assess by a question. *Desire* to die may not, by itself, indicate an individual's risk level. Rather, the counselor must assess how capable the client is to follow through on the desire. *Capability* is defined by a sense of fearlessness and impulsivity to follow through on a specific plan and the means available to follow through on the plan to die or attempt suicide. Neither desire nor capability predict active behaviors toward seeking death as strongly as intent (Kalafat et al., 2005). Strong *intent* to die may lead to reduced motivation or reasons for living and can result in a more lethal method selection.

The recommendations issued by the NSPL (2007) include exploring buffers, which are like protective factors to lessen suicidal risk. These may include (1) immediate supports, (2) social supports, (3) hopefulness and future plans, (4) engaging in the therapeutic process, (5) desire to live, (6) core values and beliefs that conflict with suicide, and (7) a sense of meaning/purpose.

TRIAGE APPROACH While using any suicide assessment or Q-tree, counselors must also be observant and use the TRIAGE approach. Counselors should observe a client's *affective factors* (e.g., nonverbals such as being withdrawn, uncooperative, distant, disinterested in taking part in a plan), *behavioral factors* (e.g., how likely a client is to act on their thoughts or follow through on a plan to stay alive), and *cognitive factors* (e.g., Does a client seem rigid in their thought process or open to exploring the situation and options for life? Are thoughts distorted?). In addition, *developmental factors* (e.g., the perception of death as related to a client's understanding of death by age, cognitive level, or developmental abilities and limitations) and *environmental factors* (e.g., various cultural values important to the individual) should be gathered and documented. See Chapter 1 for more details.

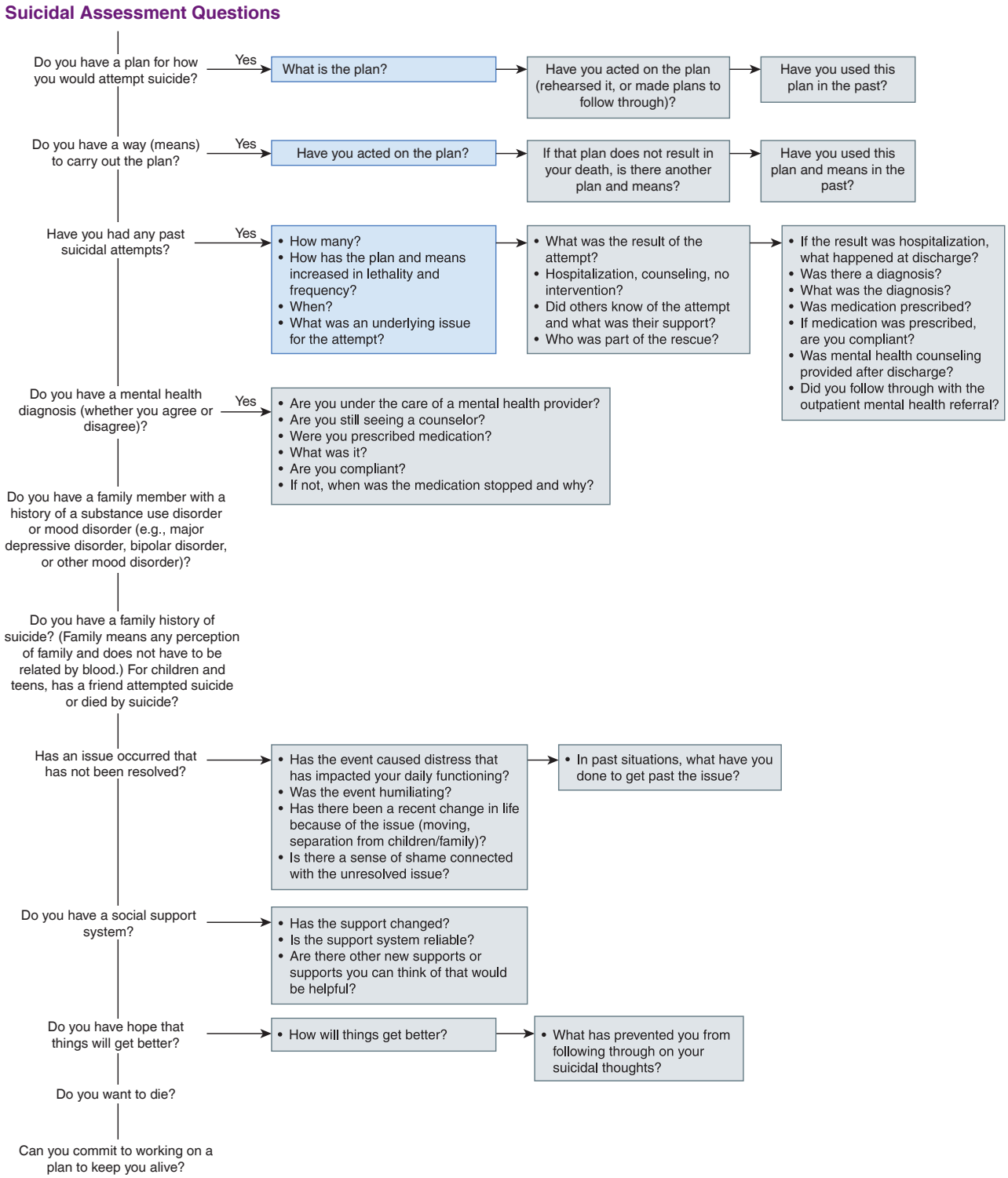

FIGURE 6.1 Sample Q-tree for Suicidal Assessment.

Suicide Lethality Assessments

Many assessments are available in the field and used as frameworks by crisis counselors in lethality assessment. It is a standard of care to integrate at least one model into your practice to provide documentation for each client. Regardless of the model selected, the Q-tree discussed above should be integrated into any of these models. Some models will indicate a section within the model for gathering risk and protective factors, although the Q-tree or essential questions are not written into the model. The following assessments provide a sampling from the field.

BECK SCALE FOR SUICIDE IDEATION The Beck Scale for Suicide Ideation (BSI) is a self-report instrument requiring 10 minutes to administer that evaluates a client's thoughts, plans, and intentions to die by suicide. The 21 items are administered by the counselor using a rating scale that includes items such as "I have made plans to die by suicide" and "I have access to lethal means." The instrument requires specific, detailed recall of suicidal experiences, such as "I was XX% sure I would die by suicide" and "I have only felt suicidal (number of) times."

CHRONOLOGICAL ASSESSMENT OF SUICIDE EVENTS The Chronological Assessment of Suicide Events (CASE) model was developed by a former emergency department (ED) doctor who observed firsthand the high prevalence of suicide risk and suggested there was a window of opportunity where professionals could intervene when a client was discharged from psychiatric admission to the ED or psychiatric hospitalization. His concern was with the high demands on ED medical professionals and being able to better assess suicidal risk. The CASE method focused on the immediate factors of the client's suicidal risk—the conditions immediately prior to the psychiatric emergency (6–8 weeks buildup or a pileup of distress), the influences prior to 6–8 weeks of buildup (e.g., long-ago exposure to suicide, family history of suicide, traumatic events)—and concludes with questions about the "now" of suicidal feelings and pending days ahead. The approach emphasizes five sets of question-asking:

1. Shame attenuation (i.e., normalizing feelings of psychological suffering and the immediate issues the client has experienced)
2. Behavioral incident specificity (e.g., "Where is the weapon?", "And then what did you do?", "And after that, what did you do?", "Who knows about your plans?")
3. Gentle assumption (e.g., "How have you thought of killing yourself?" rather than "Have you ever thought about killing yourself?")
4. Symptom amplification (e.g., "How much time do you spend thinking about suicide in a 24-hour period?")
5. Denial of the specific (e.g., asking clients if they have considered other methods—this will not give clients the idea of having more options)

COLLABORATIVE ASSESSMENT FOR MANAGING SUICIDE Jobes (2016) developed an assessment that combines the interview with the written instrument tool. Collaborative Assessment for Managing Suicide (CAMS) is evidence and theory based and requires close collaboration with the client to understand the details of the suicidal risk. CAMS integrates safety and treatment planning and has been integrated into a 4-hour online training. It can be found at *http://www.empathosresources.com* (Empathos, 2015).

COLUMBIA-SUICIDE SEVERITY RATING SCALE (C-SSRS) The Columbia-Suicide Severity Rating Scale (C-SSRS; Posner et al., 2011) includes detailed categories of suicide ideation, intensity of ideation, and suicidal behavior to assess lethality of suicide. The scale has been adapted and developed for various populations (e.g., general population, schools, and first responders) and has suggested actions coded as red, yellow, and green based on the responses of the individual.

MINI-MENTAL STATUS EXAMINATION (MMSE) The Mini-Mental Status Examination (MMSE) is a brief (10- to 15-minute), quantitative measure to assess cognitive impairment for those age 18–85 years, is available in over 70 languages, and is recommended as a screening tool for depressed individuals (Folstein et al., 2016). The MMSE demonstrated score validity and reliability in psychiatric, neurologic, geriatric, and other medical populations. It is used to evaluate five areas of mental status: orientation, registration, attention and calculation, recall, and language. The maximum score is 30, and a score of 23 or lower demonstrates a level of cognitive impairment indicating a referral for a more intensive cognitive evaluation. Orientation, place, and recall are not impacted by level of education and are important to assess prior to administering the examination. If only assessing for orientation, place, and recall, a total maximum score is 17. The MMSE is attributed to the following descriptors that are frequently referred to in many client charts:

- Oriented *3 (person, place, time)
- Oriented *4 (person, place, time, situation/event)

POSITIVE AND NEGATIVE SUICIDE IDEATION INVENTORY Positive and Negative Suicide Ideation Inventory (PANSI) is a 20-item self-report measure of positive and negative thoughts related to suicide. It contains statements such as "I am in control of my life" and "I think about killing myself," to which clients respond on a 1- to 5-point Likert-type response scale. This instrument is intended to be used as a screening tool for adults and high school students.

SUICIDE ASSESSMENT FIVE-STEP EVALUATION AND TRIAGE The Suicide Assessment Five-Step Evaluation and Triage (SAFE-T) can be used by mental health professionals during the initial contact with a person experiencing thoughts of suicide. The five-step assessment includes identification of risk and protective factors; conducting an inquiry about suicidality (implementing a Q-tree); determining level of risk and selecting an appropriate intervention; and documenting the process, including a follow-up plan.

TASK MODEL OF CRISIS ASSESSMENT AND INTERVENTION The task model of crisis assessment and intervention (see Table 6.4 and Chapter 1; see also Jackson-Cherry, 2018) can be integrated for a suicide assessment and focuses on *four important areas of crisis assessment* leading to a more effective and appropriate *four-task intervention plan* for individuals in virtually every type of crisis, regardless of setting. Assessment of safety/stabilization; biopsychosocial/spiritual elements; clarification of the problem(s); and coping skills, resources, and supports are essential first steps toward more accurately defining the actual crisis situation.

TABLE 6.4 The Task Model of Crisis Assessment and Intervention

Level I: Four Essential Crisis Assessment Tasks
Assessment Task 1: Safety, Stabilization, and Risk
Assessment Task 2: A Holistic Biopsychosocial/Spiritual Assessment Approach (Integrate Q-Tree)
Assessment Task 3: Clarification of the Problem(s)
Assessment Task 4: Coping Skills, Resources, and Supports
Level II: Four Essential Crisis Intervention Tasks
Intervention Task 1: Normalize and Educate
Intervention Task 2: Exploration of Options
Intervention Task 3: Develop a Plan and Obtain a Commitment
Intervention Task 4: Documentation, Follow-up, and Referral

Source: Jackson-Cherry (2018)

TREATMENT PLANNING AND CARE MANAGEMENT WITH A SUICIDAL CLIENT

Learning Objective 6.6 Describe appropriate treatment planning and care management with a suicidal client, including a safety plan, outpatient and inpatient treatment, and postvention.

After assessing a client's acute or chronic risk and determining the level of risk, many mental health professionals might again ask, "Now what do I do?" Assessment is only part of the intervention process. Rudd et al. (2004) and Jobes (2016), among others, provide qualified step-by-step recommendations for appropriate treatment planning and responding. Just as all treatment plans for clients seeking counseling for any condition are customized to meet their needs, the same is true for clients at risk for suicide. The assessment process yields valuable information to assist with the design of appropriate treatment interventions. This section reviews several ideas for counselors to consider, beginning perhaps with the most important intervention of all: the development of a customized safety plan.

Safety Plan

Developing a **safety plan** with the client is an industry standard and expectation, but many mental health professionals were trained prior to the emergence of research and data to write a "no-harm contract." Based on research, best-practice thinking, examination of the potential for false security regarding liability, and more evolved legal, ethical, and clinical reasoning about how much a suicidal client might be able to "agree" to such a "contract," the no-harm contract has been replaced with professional and empirical preference for the safety plan.

Even seasoned practitioners who are reluctant to do away with the no-harm contract should be advised that not only is it ill-advised but it is also in contradiction to many more modern insights about safety planning, and they could run the risk of *increasing* liability if using no-harm contracts even in conjunction with safety plans. Liability for knowing what to do in the case of a suicidal crisis is clearly the burden of responsibility on the professional and not the client, who would be potentially

considered not "competent" to direct treatment decisions if incapacitated with life-threatening depression or multiple symptoms of any diagnosis. Many acutely at-risk suicidal clients are not in a psychiatrically oriented or lucid condition to shoulder the burden of knowing enough about suicide or safety planning. Therefore, the attempt of the clinician to contract with the temporarily disabled client increases the counselor's liability through the use of a no-harm contract.

While some authors have referred to safety plans by varying nomenclature (such as *crisis response plan* and *self-care plan*), the NSPL's Standards, Training, and Practices Committee recommends the term *safety plan*.

> We would always use the term "safety plan" . . . other terms [crisis response plan or self-care plan] are too vague and do not explicitly reference the fact that keeping the individual safe is the primary goal in developing a plan—vagueness is not good. In addition, we would never reference a "safety plan" without explicitly stating the avoidance of anything that resembles a "no harm contract"—so would attempt to eliminate any confusion at the outset. (G. Murphy, personal communication, September 22, 2010)

Stanley and Brown (2009) have written extensively about the components of a safety plan and recommend that clients ask the following questions:

1. When should I use the plan?
2. What can I do to calm/comfort myself if I am feeling suicidal?
3. What are my reasons for living?
4. Who can I talk to?
5. Who can I talk to if I need professional assistance?
6. How can I make my environment safe?
7. What should I do if I am still not feeling safe?

Stanley and Brown addressed not only what should be included in safety planning but also how to talk through its use with clients. They are widely considered the field's experts on safety planning, and practitioners should explore their resources and seek training in their Safety Planning Intervention.

A comprehensive safety plan addresses specific behavioral actions, determined in concert with the client, and includes several key focal points including means restriction, self-soothing options, self-care options, family involvement options, resource development, and crisis response options (M. Gould & D. Jobes, personal communication, November 10, 2010), in addition to the recommendations that Stanley and Brown (2009) suggested.

One essential component of safety planning is addressing means restriction (Coombs et al., 2010; Mann et al., 2005). Since firearms are the most frequently used method with which to attempt suicide in the United States—and the most lethal with the least amount of time for active rescue or intervention—counselors developing safety plans should include a focus on means removal. It is best to engage a safety partner to assist with means removal to guarantee the means restriction more fully, rather than counting on the client to do so.

An example of a safety plan is presented in Table 6.5, written with samples of specific behavioral actions.

TABLE 6.5 An Example of a Multimodal Safety Plan with Specific Behaviors

I will . . . *(List specific behaviors such as the examples listed below)*. **Customize the plan with your client.**

1. Remove gun and ammunition from house, car, truck, cabin, barn.
   (Note to counselor: Means restriction)
2. Fill doctor's prescriptions weekly (not a month at a time).
   (Note to counselor: Means restriction)
3. Take my dog to play in the park.
   (Note to counselor: Soothing feelings of distress)
4. Call a friend.
   (Note to counselor: Soothing feelings of distress)
5. Eat three meals a day.
   (Note to counselor: Self-care)
6. Drink nonalcoholic and noncaffeinated beverages.
   (Note to counselor: Self-care)
7. Avoid lonely times by going to my sister's.
   (Note to counselor: Family support)
8. Spend the night at my cousin's house.
   (Note to counselor: Family support)
9. Go to a community support group for persons who have bipolar disorder.
   (Note to counselor: Using resources)
10. Call my sponsor or go to a meeting.
    (Note to counselor: Using resources)
11. Call 988 or contact https://988lifeline.org
    (Note to counselor: Crisis or emergency support)
12. Go to my primary care physician, psychiatrist, or the ED.
    (Note to counselor: Crisis or emergency support)

List the names and reliable phone numbers of several people that the counselor can call or who can call the counselor in the event of an escalating situation:

Name: _____ Phone: _____

Name: _____ Phone: _____

Name: _____ Phone: _____

Name: _____ Phone: _____

Name: _____ Phone: _____

Note: Specific behavioral ideas should be written in the affirmative. For example, in Item #6, instead of "Don't drink alcoholic or caffeinated beverages," restate it in the affirmative "Drink nonalcoholic and noncaffeinated beverages."

A well-facilitated assessment will encourage a client to trust the counselor and provide detailed information, which will allow the counselor to understand when, how, and under what conditions the client becomes vulnerable to suicide. For example, if a client reveals they want to end their life, particularly when experiencing conflict with a partner, the treatment plan might recommend couples counseling and conflict-resolution skills building. If an isolated 80-year-old widower reports being desperately vulnerable between the hours of 5:00 and 8:00 p.m. each day when imagining all the neighbors having dinner with their loved ones, a treatment plan should increase social support during that period of vulnerability.

Safety plans should always involve resources, actions, and coping mechanisms that the client is able and willing to put into place, that are specific to the situation, and that are written with clear behaviorally specific verbiage. In the examples provided in Table 6.5, note that there are behavioral recommendations for means restriction, self-soothing, self-care, family support, use of community resources, and crisis response options.

Outpatient and Inpatient Treatment Decisions and Management

Changes in the health care insurance industry and managed care have severely reduced the amount of hospitalized care that at-risk persons may obtain, even when suicidal risk remains high. However, when a client is in immediate risk to self, the decision for hospitalization may be required to keep the person safe. States and counties within states differ as to the process for involuntary and voluntary hospitalizations, and it is essential that clinicians understand the laws and policies in the state and county where their clients live. It is better to know what to do prior to a crisis for both the clinician and the client.

Voluntary hospitalization is preferred over involuntary hospitalization as it allows the client (and supports) to be part of the decision-making process. In many states, when a person voluntarily seeks hospitalization, they may have more control in their discharge. Although this may be preferred, voluntary hospitalization may leave the crisis worker with an uneasiness of not knowing whether the client followed through with this plan. When clients and families agree to voluntary hospitalization, the crisis counselor should obtain a release of information and gain as much information as possible as to where the client will be seeking admission so they can follow up. A well-being check can be requested if there is any thought the client (or guardians) did not follow through. The well-being check is a request of law enforcement to check on the "well-being" of the individual. Since well-being checks do not fall in the priority of most law enforcement priority calls, these checks may be at the discretion of the law enforcement officer. An involuntary hospitalization may follow a voluntary decision that is not followed through by the client.

Involuntary hospitalization differs from state to state, whereas some states require courts to mandate the commitment of an individual; others allow licensed professionals to initiate an emergency petition for involuntary hospitalization; some states integrate mobile crisis teams or crisis intervention teams, while others rely on a community service board that would make the final assessment and determination based on the clinicians' observations. The crisis counselors should also establish professional relationships with community service officers in their jurisdiction and become familiar with protocols. In these situations, once a client is involuntarily hospitalized, the client will likely not have further contact with the crisis counselor, and clients will be discharged to outpatient mental health counselors.

Partial hospitalizations are often referred to as a step down from a hospitalization when a person may need additional or more intense support than outpatient mental health. For some this may be residential support in the evening when risk is higher. For others, like children, partial hospitalization may be scheduled during the day where they are able to continue their schoolwork and access mental health services and be discharged to parents in the evening. Medical stabilization and pharmacological intervention are

the goals of hospital mental health administrators, and discharge planning begins at admission. No matter how difficult, uncomfortable, or disruptive a hospitalization is for clients, it does save lives. Discharge planning and collaboration is essential between hospital personnel and outpatient counselors to eradicate the high window of risk for suicide after discharge.

CRISIS FOLLOW-UP CONTACT The follow-up with a client who is at risk for suicide is critical practice (Murphy, 2010). Follow-up as a clinical standard is a form of active postvention that is postventive, preventive, and interventive. Follow-ups may include regularly scheduled phone calls, letters, or notes, and even texting or e-mailing, or they may include memoranda of agreements with provider organizations (e.g., between agencies or practices and hospital EDs).

Data about follow-up assessments with crisis center callers is very encouraging in that callers who sought out suicide intervention assistance from centers are very positive and motivated to reduce ambivalence about wanting to die. Clinicians may direct family care team members to obtain helpful booklets from agencies such as the Feeling Blue Suicide Prevention Council (*https://www.givepulse.com/group/2786-Feeling-Blue-Suicide-Prevention-Council?*) to assist family members following a suicide attempt. The Substance Abuse and Mental Health Services Administration also has several "after an attempt" resource booklets for medical providers, attempters, and family members of attempters that can be obtained at *https://store.samhsa.gov/product/A-Guide-for-Taking-Care-of-Your-Family-Member-After-Treatment-in-the-Emergency-Department/sma18-4357eng*.

DOCUMENTATION OF LETHALITY It is important that the counselor notes what a client reports and how they present. Not asking does not mean a client is not suicidal. It means the standard of care was not met sufficiently. How a client presents is also important documentation. Is the client agreeable to a plan? Are they more distant today than other sessions? Do statements made by the client and how the client is behaving conflict? Do they appear disoriented? Does the client have distorted thoughts? A common risk statement should show that a verbal assessment was made as well as an observation of nonverbal behaviors: "The client does not report or present with current suicidal or homicidal thoughts and behaviors. The client denies a history of suicidal behaviors or hospitalizations for psychiatric concerns." This statement implies that the counselor has asked the appropriate questions and the client has been evaluated based on their behavioral presentation. This documentation should also be extended to NSSI and homicide risk assessments. Now read Voices from the Field 6.1.

VOICES FROM THE FIELD 6.1

Crisis Intervention with Clients Who Have Low Cognitive Functioning

Amanda Evans

A common theme that I noticed when working at an inpatient psychiatric unit as a graduate student was helping professionals struggling to assist patients in crisis who performed at lower cognitive functioning. I can specifically remember several clients who were misunderstood by the hospital staff due to their level of cognitive functioning and were subsequently provided more restrictive care. It is very important as a counselor to be mindful that just because a client is an adult does not mean that the client can manage the responsibilities or cognitive complexity of adulthood. A patient who

appears to be 50 years old may in fact function cognitively as an adolescent or as a child. This can be challenging in a hospital setting when clients in crisis require immediate, effective, and appropriate medical attention.

Oftentimes, patients who were lower functioning were expected to cope and manage in the same way as their higher-functioning peers. This included maintaining personal hygiene, medication compliance, following the unit's rules, and establishing appropriate boundaries with their peers. In addition, I noted counselors who provided treatment to these clients struggled with gathering intake information and establishing aftercare goals. This difficulty in serving lower-functioning patients was most evident in the staff's attempt to deescalate crises. In my opinion, clients who are lower functioning are commonly misinterpreted by helping professionals, leading to unnecessary rapid escalation in crisis situations, physical restraints, and possibly medical restraints.

Based on my personal experiences, I want to encourage counselors-in-training to consider alternative methods to communicate with clients in crisis. When patients who have low cognitive functioning are in crisis, expecting mature, rational decision making may be unrealistic; however, helping clients to express themselves, calm down, or self-soothe is ideal. In addition to talking through a crisis, working with individuals using pictures, schedules, sign language, silence, or techniques specific to the client's needs can be very helpful in deescalating a crisis. At times, competent care requires stepping outside of the box and using alternative interventions to best assist clients.

Postvention

In recent years, postvention has grown to include the care practices that practitioners provide to persons who have survived a suicide attempt. Postvention with attempt survivors can include ongoing stabilization, long-term therapy, and improvement of help-seeking skills and overall self-care, with a goal of long-term improvements in quality of life and functioning.

Historically, postvention has been the type of care provided to survivors of suicide loss. Survivors of suicide or homicide loss can develop protracted symptoms of grief, depression, anxiety, PTSD, longer and more complicated grief and bereavement; more demands on medical and mental health services; and greater potential for lost time at work and at school. Survivors of suicide loss also have an elevated risk for suicide attempts. The remainder of this section focuses on survivors of suicide loss.

BEREAVEMENT AND OTHER ISSUES Bereavement from both suicide and homicide can remain complicated for many years. The DSM-5-TR (American Psychiatric Association, 2022) added a diagnosis of *prolonged grief disorder*; arguably, many survivors of suicide or homicide loss might be at risk for this diagnosis due to the vexing circumstances compared to other types of loss.

Given the prevailing prevalence of suicide in the United States, a conservative estimate of persons who become newly bereaved due to the loss of a loved one by suicide numbers over 275,000 per year (Suicide Awareness Voices of Education, 2022). The job of bereavement counselors is an important one, due both to the large number of survivors of suicide loss in the population and the misperceptions and myths about suicide both before and after death. Survivors of suicide loss often report feeling misunderstood, stigmatized, and marginalized. They are prone to bereavement and sorrow, complicated grief, and posttraumatic symptoms (Harrington, 2011). Their bereavement may be punctuated by the complexities of believing that their loved one "rationally chose" death, as contrasted with viewing suicide as a multifactorial process of debilitation and vulnerability (Survivors of Loved Ones' Suicides, 2006). They may have lost a child, a

sibling, or a partner within a cloud of unknown circumstances, such as losses or defeats that resulted in acute suicide risk, such as a bad business deal, a negative performance evaluation at work, or an act of infidelity that they could not work through, to name a few.

The circumstances surrounding the immediate scene of a suicide may include the involvement of law enforcement, investigators treating the scene as a murder, potential arrest warrants being issued, and limited or potentially gruff communication with personnel such as coroners or first responders, possibly punctuated with yellow crime scene tape. Of course, many survivors report exemplary conduct on the part of these professionals, and so it is not a universal problem, but for some survivors of suicide loss, the circumstances surrounding the immediate scene complicate the grieving process.

Adults and children alike are often witnesses to the actual suicide, they may discover the body, or they might be involved in clean-up. Many family-related and religious complications can ensue, with conflicts possible among stepfamily, family-of-origin, or in-law family members who may disagree about stating the cause of death openly to their communities. Sometimes the suicide may be like one domino in a row of dominoes that continue to fall. For example, consider the scenario in which a middle-aged person dies from suicide, leaving financial problems that result in foreclosure or bankruptcy for the surviving family members, leaving college-age children unable to continue their schooling, and a partner/parent who must reenter the workforce or sell the house. The suicide can become one crisis of many in a sequence of events, not unlike the ABC-X model of crises (Hill, 1949, 1958) or the Double ABC-X model of crises (McCubbin & Patterson, 1982).

Counselors providing postvention grief services to survivors of loss should become familiar with complicated bereavement and mourning, learn about counseling techniques and countenance directly related to working with suicide loss, and especially be aware of the feelings of helplessness, sense of responsibility, and internalized guilt that the bereft feel after a suicide. Many family members have knowledge of their loved one's struggles and may have assisted in ways that in fact prolonged the life of their loved one. Yet the traumatic circumstances, the stigma or belief that it was a rational choice, or the complicated mourning leave survivors clinging to a sense of guilt-framed attribution of responsibility.

FRAMEWORK TO INTERVENE WITH SURVIVORS Whether working with an individual or with a group, there are common factors to take into consideration that may be unique to those grieving the loss due to suicide: (1) prevention of further suicides, (2) reconstructing the "why," (3) commemorating the positive, (4) saying goodbye, and (5) an assessment of guilt/shame and absolution of guilt. The main priority throughout the time with survivors is to assess and prevent further additional suicidal behaviors. Often, suicide survivors have no closure and may not know why their loved one decided to take their own life. It is unlike the grief process during which a person is grieving the loss of a loved one with a known cause (e.g., terminal illness, accident).

In suicide cases, survivors often are stuck in the "why." Questions such as "Why did he not come to me for help?" and "Why did she take her own life?" are things that keep a survivor progressing through healthy grief work. Counselors may need to normalize the repetition of these questions and reconstruct the why to understand what the underlying meaning of these "why" questions is for the survivors. Meaning may relate to the perception of being a "bad" parent, grieving the loss of a role of being

a family member, or could uncover some hidden guilt in not being present when they now believe they could have assisted in suicide prevention.

Connected with being stuck in the "why," survivors are unable to have closure and move on with their own lives through letting go or saying goodbye. Counselors, when reconstructing the "why," will often help clients work toward moving on in their own life while still remembering the loved one lost. Similar to staying stuck in the cognitive "why," survivors may experience a range of emotions, including anger, sadness, confusion, numbness, and guilt (to name a few) toward their loved one. Being stuck in one emotion can also prevent survivors from moving on and healing in grief work and may lead to negative feelings toward their loved one. Being able to commemorate a positive trait or attribute can allow a survivor to start working through the loss.

Finally, in suicide cases, there may be a degree of guilt by the survivor, thinking they could have done something to prevent the suicide. In many cases, survivors can recall the warning signs of the loved one. However, these warning signs are often only evident after the fact and could not have predicted a suicide. Therefore, it may be important for survivors to know that whatever the situation or relationship leading up to the suicide, it was ultimately the decision of the loved one to take their own life—offering absolution of guilt. Having these aspects built into the grief process can help survivors move into healthy grieving.

When working with family and friends, it is imperative to determine who may have found the body. Often, a family member may not be forthcoming if they found the body. The human senses (visual, hearing, touch, or smell) connected with the suicide may trigger flashbacks for this individual; therefore, the individual who found the body should be a priority in counseling.

Counselors should become familiar with how to find suicide bereavement groups that exist in many localities around the country by visiting either the American Foundation for Suicide Prevention or the American Association of Suicidology websites for comprehensive listings. While survivors of suicide loss often report that they speak the same language, a language that other bereft persons do not fully understand, not all survivors will think that a group is the best option for them. Other suggestions for therapeutic responses that counselors can recommend include bibliotherapy and Internet resources, psychoeducation, family guidance, survivor outreach, cyber groups or chatrooms, survivor conferences, and activism, with activism marking a full circle from postvention to prevention.

Suicide bereavement counselors might provide psychoeducational insights to survivors of suicide loss. For example, the counselor may provide insight that experts do not believe that suicide is a "rational choice," that intent looks very much like a "choice" when clinically they are very different, that suicide was not done to punish the survivors but rather to end the pain or to alleviate the burden that the decedent perceives they were (often a mistaken perspective). Counselors should be prepared to support survivors of loss when survivors encounter misinformation, myths, stigma, and judgments about the type of death that occurred. Some people, even well-meaning friends, say hurtful and untrue things, such as "I can't believe he would do such a selfish and hostile thing to you" or "How tragic that she committed this sin; she'll surely go to hell."

Finally, some counselors become crisis advisors to employers, agencies, schools, faith-based groups, and communities. These professionals should be familiar with the national resources available for responsible communication with the media following

a suicidal death. All the prominent national suicidological organizations have such resources available on their websites for distribution to decision makers, spokespersons, and even media representatives to use the public news forum responsibly, prevent suicide contagion, and avoid glorifying suicide or misrepresenting knowledge about it. Now complete Activity 6.2.

ACTIVITY 6.2

Think about how society sometimes normalizes language that is violent or self-harming. Have you ever heard friends in casual conversation use phrases to describe their exasperation like "That's enough to make me wanna kill myself" or "I'm just going to shoot myself if he doesn't quit teasing me"? Perhaps you have seen seemingly humorous references to suicide in unexpected places. For example, a restaurant may have a dessert called "Suicide by Chocolate" or a corporate trainer may talk about "career suicide."

Public messaging is a powerful tool in promoting initiatives that are clinically responsible and in the interests of public health. It is unacceptable to refer to a fatal suicide attempt as "successful." Only prevention can be considered successful.

How we unwittingly toss around the words and concepts of suicide says something to the public. We may unconsciously promote stigma and even set up barriers for the public to learn and to trust that help is available. They will not have to worry about being crazy, a quitter, a loser, or a sinner if they understand that health care professionals know that self-directed violence is the result of involuntary and debilitating attempts to end psychological pain. To be more empowered and enlightened is to use language that is nonjudgmental and inviting to persons who may be struggling to be help-seeking.

What will you do the next time you hear a so-called humorous reference to suicide? How would you entertain the idea that generally people do not often joke about other life-threatening illnesses, such as cancer, cardiac arrest, or diabetes? We do not see "Cancer by Chocolate" on menus. People do not say "she cancered" herself. What small individual act of suicide prevention advocacy will you do when you have the opportunity?

NONSUICIDAL SELF-INJURY

Learning Objective 6.7 Describe assessments and treatment for NSSI, including the use of a Q-tree.

The DSM-5-TR added suicidal behavior and nonsuicidal self-injury into the "other conditions that may be a focus of clinical attention" section. The inclusion of NSSI should improve documentation of these behaviors to determine risk factors for suicidal ideation and actions and provide more effective treatment planning options. NSSI is the intentional, self-inflicted, immediate destruction of body tissue, without suicidal intent, that is not viewed as culturally acceptable behavior. Self-injurious behaviors can be performed on any part of the body but often injuries are done to the hands, wrists, stomach, and thighs. The severity of NSSI can range from superficial wounds to long-term disfigurement. In one study, 33% of people who reported engaging in self-injurious behaviors reported they should have sought medical attention; however, only 6.5% in the same study reported receiving medical attention for their wounds. Although the most common NSSI behavior continues to be reported as cutting, other common behaviors or methods of NSSI include, but are not limited to:

- Burning
- Skin picking
- Hair pulling

- Banging or hitting the head or other body parts
- Rubbing objects into the skin
- Skin erasing or carving
- Interfering with wound healing
- Bone breaking

Most of the literature suggests that NSSI increases substantially in early adolescence and decreases during late adolescence and early adulthood (Plener et al., 2015) with more incidents of NSSI behaviors by females (Fox, 2015). Considering the prevalence of self-injury among adolescents, the likelihood that counselors and medical professionals will encounter NSSI is quite high. Historically, NSSI has been viewed as not linked with suicidal intent, but most clinicians and researchers suggest that NSSI is a strong predictor of future suicidal behavior. Scott et al. (2015) estimated that suicide attempts increased fourfold for those who previously engaged in NSSI and suggested that both suicide ideation and NSSI are known risk factors for suicidal behavior with increased predictive risk for suicide attempts.

Assessments and Q-Tree

The high correlation of NSSI and future suicide attempts prompted the DSM-5-TR (American Psychiatric Association, 2022) to add suicidal behavior and NSSI into section 2, "other conditions that may be a focus of clinical attention." Furthermore, it is a standard of care to integrate an NSSI assessment as part of all suicide risk assessments. It is essential to understand the intentions of those who participate in NSSI to properly assess intent, implement the most effective treatment strategy, or appropriately refer to additional care. Every intake should have an NSSI assessment in addition to the suicide and homicide risk assessment, even if NSSI is not a presenting issue. It would be better to include a standard NSSI question than not ask and live with the consequences after the fact. This could entail simply including the following:

- "Have you ever engaged in any nonsuicidal self-injurious behaviors?"
  - ✓ If no, proceed to a suicidal risk assessment
  - ✓ If yes, complete the NSSI assessment

Kruzan and Whitlock (2019) adapted the transtheoretical model (TTM) of behavior change proposed by Prochaska and DiClemente to assess motivation for change. TTM integrates the following while assessing motivation of change: (1) *precontemplation* (with no intent to stop NSSI); (2) *contemplation* (intent to stop NSSI within 6 months); (3) *preparation* (intention of taking steps to stop NSSI within next month); (4) *action* (currently taking steps to stop NSSI); (5) *maintenance* (steps have been taken for more than 6 months); and (6) *termination* (cessation of NSSI for over 3 years).

We have integrated a Q-tree into the TTM as an example for crisis counselors to assess and identify a timeline of NSSI, risk factors, and motivation for change (protective factors) to provide referral for treatment.

- **Have you self-injured in the last 6 months?**
  - ➢ If YES (to the 6-month question), is self-injury something you would like to stop?
    - If yes, have you taken steps to stop NSSI?
      - ✓ If yes, explore ACTION STAGE and ACTION PLANNING

- If no, do you plan to take steps to stop NSSI?
  - ✓ If no, client is in the PRECONTEMPLATION STAGE (consider psychoeducation)
  - ✓ If yes, client is in the PREPARATION STAGE (explore ideas and establish plan)

➢ If NO (to the 6-month question), have you self-injured in the past YEAR?
- If yes, is self-injury something you would like to stop?
- If no, have you self-injured in the past 3 YEARS?
  - ✓ If yes, explore MAINTENANCE STAGE
  - ✓ If no, no action is recommended

A more comprehensive assessment should follow this Q-tree screening approach. The TTM approach assesses the timeline and motivations for change based on the motivation for change theory. It is limited to these assessment categories unless you implement additional elements needed for a more thorough NSSI assessment. We suggest that more elements should be included for a comprehensive NSSI assessment and have included the following adapted NSSI assessment, based on the research and clinical work of Wester and Trepal (2017), who also found a high correlation between NSSI and suicidal attempts. Thus, it is best practice to conduct *independent* suicidal risk and NSSI assessments.

According to Wester and Trepal (2017), the NSSI assessment should gather: (1) basic information, (2) contextual information, and (3) functional/reasoning information. Table 6.6 can serve as a Q-tree for NSSI, and we have included some Q-tree questions that connect to Wester and Trepal's indicators. Upon gathering this information and assessing NSSI risk, a crisis counselor can provide effective referral for a client.

TABLE 6.6 Q-Tree Questions for NSSI

Basic Information	Contextual Information	Functional/Reasoning
1. What was the age at onset? **2.** How did the NSSI begin? (Was there a triggering situation or event?) **3.** When was the last time the client engaged in NSSI? (Start with most recent, in past week, month, 3 months, year, over their lifetime) **4.** What methods are used to engage? Have methods changed? **5.** Where on the body is NSSI targeted? **6.** Is NSSI known by others or is it secretive? **7.** Are there any wounds that have needed medical attention? **8.** Has client ever been able to cease NSSI?	**1.** When engaging, are substances (drugs, prescription medications, alcohol) used? **2.** Has client ever found a wound they had no recollection of inflicting? **3.** Is client able to delay engagement? **4.** If not, has there ever been a desire to delay engagement? **5.** Is there a geographical location or setting where the NSSI occurs? **6.** Does client have thoughts or ideations of suicide as well?	What reason is NSSI used to: **1.** regulate emotions? **2.** feel better? **3.** connect to oneself (decrease numbness)? **4.** socially withdraw or isolate? **5.** engage or belong? **6.** control or influence others? **7.** fill a craving or addiction? **8.** communicate?

Just as with suicidal risk assessment, the counselor should weigh the risk factors and protective factors to gather all the information required for treatment planning and referral. As a note, often NSSI behaviors can serve as protective factors so the counselor is cautioned to not eliminate NSSI behaviors without evaluating the impact of stopping the NSSI behavior, or exploring the elimination, as this could increase suicidal thoughts or behaviors. This model is more comprehensive in gathering and reviewing several key pieces of information that facilitate referral for continued care.

NSSI assessment, like suicide and homicide risk assessment, often requires long-term care, which may exceed the time a true crisis counselor can offer, unless the NSSI treatment takes place in long-term counseling. Often, crisis counselors will gather the information and then refer the person to appropriate long-term counseling. Using the template above allows a counselor to gather the basic, contextual, and functional information needed to provide a solid referral for treatment.

Treatment Approaches

Understanding the most effective treatment approaches for NSSI is also essential. The most common therapeutic approaches for NSSI include dialectical behavior therapy, cognitive-behavioral therapy, problem-solving therapy (rational problem-solving therapy), and treatment for self-injurious behaviors (T-SIB). T-SIB was developed as a brief nine-session intervention to treat NSSI behaviors in young adults without borderline personality disorder (see Table 6.7). T-SIB can be used as a standalone treatment or an adjunctive intervention and includes the components in Table 6.7 with psychoeducation

TABLE 6.7 T-SIB Sessions and Program Overview

Session and Overview	Description of the Session Content
1. Psychoeducation and Addressing Motivation	Psychoeducation of NSSI, orientation to treatment, motivational interviewing, and other strategies to decrease ambivalence may be used by counselor
2. Functional Assessment (FA)	Assess NSSI behaviors, provide alternative coping strategies for NSSI if appropriate, and provide homework to address the functional assessment of NSSI (is the person getting what they intend from NSSI behaviors?)
3. FA	Explore times when NSSI is not engaged
4. FA and Differential Reinforcement of Alternative Behaviors	Explore behaviors that may be adopted as alternatives to NSSI and provide homework to *only* list alternative strategies with clear instructions *not* to try them yet
5–6. Differential Reinforcement of Alternative Behaviors	Evaluate impact of each alternative listed, revise list, homework assigned to implement alternative behaviors
7–8. Individual Modules	Individual sessions for each client depending on their function of events to NSSI. May integrate distress tolerance, interpersonal communication, cognitive distortions, and restructuring
9. Termination	Review progress and address obstacles moving forward

and therapeutic interventions. Counselors know how stressful this work can be and are reminded to receive training, practice self-care, and seek consultation and supervision as needed. Next, read Voices from the Field 6.2.

VOICES FROM THE FIELD 6.2

Treating Nonsuicidal Self-Injury

Amanda C. La Guardia

After completing my bachelor's degree, I started working in the mental health field as a case manager assigned to a small number of children and adolescents who had been identified as "at risk" for being removed from their homes and placed into foster care or long-term inpatient facilities. My job was to help ensure these youngsters stayed with their families. I was with each family for up to 12 hours per week, which was far more than their licensed counselors were. I frequently felt that I did not have the training necessary to help in the way that was needed (which is why I decided to pursue my master's in counseling).

One of the biggest challenges I faced with many of my clients was treating non-suicidal self-injurious behavior. I worked with several adolescent females who used self-injury as a way of coping with either the psychological impact of the trauma they had experienced or their emotional turmoil. I also encountered elementary school–age boys who engaged in self-injurious behaviors for similar reasons. Rather than perceiving these behaviors as a coping strategy, many of the counselors I worked with judged these behaviors to be the result of mental illness. For example, borderline behavior was a consistent assessment for the females and impulsivity because of hyperactivity was a common assessment for the boys. Thus, I noticed clinicians often did not focus on nonsuicidal self-injury as an important treatment concern. I knew better, but I needed training.

In many cases, the clients who engaged in the most frequent self-injurious behaviors were also contemplating suicide. They would injure themselves to deal with their overwhelming emotions (or lack of emotional connection) and to prevent acting out on their suicidal thoughts. This combination was tricky. I knew they needed a better way of coping, but I did not want to take away the method that they felt kept them from walking down a more dangerous path. Formal and informal ways of assessing their suicidal ideations as well as their need for self-injury became equally necessary. Self-injury and suicidal ideation are separate issues, but they are often interconnected.

Together with my clients, we discussed what situations (external and internal) tended to lead to self-injurious behaviors and worked on developing new ways of coping. I did not ask them to stop, but I did ask them to take care of themselves and encouraged them to find new ways of dealing with their pain because they deserved happiness, however they defined it; they were worth it. I let them know I would be with them as they struggled, and then I made sure that I was.

HOMICIDE

Learning Objective 6.8 Describe threat assessment and treatment options for intervention with people who are homicidal or are threats to others.

Most counselors are aware that they are likely to encounter suicidal clients in their counseling practice (Laux, 2002) and be exposed to persons experiencing thoughts of suicide. Still, many counselors underestimate the need for expertise in assessment and intervention skills with homicidal clients. There is a general assumption that clients with homicidal ideation are found primarily within the domain of forensic mental health and therefore are not a concern for most counselors in a variety of settings. Obviously, clients with a history of certain mental disorders and violent offenses do present an elevated risk to the public, and mental health professionals working with violent offenders should strive to balance public safety and client rights (Carroll et al., 2004; Large et al., 2011).

Ongoing reports from the media reveal that the potential for violence and homicide exists in families, schools, colleges, the workplace, and across all communities. Accordingly, it is essential that counselors in all practice settings be equipped to assess the lethality of clients with homicidal ideation and be capable of making decisions regarding the referral and treatment of dangerous clients. Van Brunt (2015) stated that while mental illness is often a contributing element, "the core of assessment must be based on threat assessment principles, not clinical pathology" (p. viii). Moreover, counselors in all settings may be called on to provide services to survivors of loss due to homicide. A homicidal crisis is intense and has long-lasting repercussions for individuals and communities who are left struggling to make sense of their traumatic loss and complicated grief (Currier et al., 2006). In this section, we explore the issue of homicidal ideation and systems of threat level assessment as well as treatment options.

Homicide can be defined as the willful killing of one person by another person. According to the CDC (2022b), in 2018 there were 24,576 homicides in the United States (7.5 per 100,000). The Federal Bureau of Investigation (FBI; 2019) reported 16,425 homicides for 2019, which reflected a rate of 5.0 per 100,000 in that year. Preliminary analysis forecasts that the downward trend will continue, though the trends will be studied closely given any pandemic-related influences on the increase or decrease in violence. It is also important to note that while overall homicide rates have been decreasing, the rate of mass shootings has been on the rise, with 40 mass shootings in 2020, up from 30 in 2019 (FBI, 2021).

Sixty-eight percent of homicides are committed with guns (Doucette et al., 2019). Mental health professionals as prevention advocates are faced with myriad complex variables related to public opinions, political and values-informed discourse, the availability of guns through legal and illegal means, conflicting social values between personal freedoms and personal safety, and other factors. At the very least, within the clinical relationship, clinicians can and should strive to prevent homicide by attending to clients' safety with means-removal, means-restriction, and safety planning. They should work with highly attuned standards for duty to warn and duty to protect.

"*I could just kill him!*" That is not an uncommon declaration from a client venting anger at a difficult partner, peer, or coworker, and counselors must be able to ascertain frustration from true risk of violence (Welfel, 2016). Such threats of harm toward another usually transpire when a client is experiencing extreme psychological anguish and is attempting to resolve feelings of distress; they are expressions of emotion without any real intent to carry out a threat (Warren et al., 2011). The counselor is then tasked with determining whether such pronouncements are simply common idiomatic expressions of anger or are legitimate lethal threats. Fortunately, in most instances, clients are merely indulging in hyperbole to give voice to intense anger or frustration; however, counselors must be prepared to recognize and assess potential lethality in cases where resentment and infuriation have escalated into homicidal ideation and possible violent actions.

Legal Issues

Working with potentially violent and homicidal clients presents a challenge to crisis counselors as they attempt to assess possible homicidal behavior, provide treatment, and protect potential intended victims and themselves (Meloy et al., 2011). When counseling potentially homicidal clients, the protection of the public (or individual) is the priority.

The noteworthy 1976 case of *Tarasoff v. Regents of the University of California* (refer to Chapter 3) clarified that the duty under California law to warn individuals who are in peril is of greater importance than the preservation of client confidentiality (Rothstein, 2014; Welfel, 2016). The *Tarasoff* case established the legal construct of duty to warn through case law that inspired a higher ethical standard, even though in many states it was not codified by law and still may not be used as a precedent in some states.

It is a good idea for crisis counselors to become familiar with the regulations of their own state laws regarding the duty to warn, as these issues are constantly being addressed by legislators at the state level. Welfel (2016) noted that while codes of ethics allow breach of confidentiality regarding dangerous clients and the burden of responsibility to warn targeted victims, there are state laws that require client protection to take place through hospitalization of the threatening individual (e.g., in Ohio) or contacting police rather than directly contacting the intended victim (i.e., in Texas) and breaching confidentiality.

Counselors must be aware of the terminology in their licensing states in relation to homicidal behaviors. For example, Welfel reports that some states clarify that the harm must be imminent before confidentiality can be breached and may require consultation with other mental health professionals. Legal consultation is strongly urged before confidentiality is breached.

Assessment of Homicide: Risk Factors

Certain basic indicators and risk factors tend to be associated with homicidal behavior and violence. However, a word of caution about risk factors: While they may assist crisis counselors in assessment, it is important to remember that the existence of risk factors does not mean the client *will* become violent, and, conversely, the absence of risk factors *does not* indicate a lack of homicidal intention. Remember, each case must be evaluated on its own terms. With or without the presence of certain risk factors, someone can become homicidal at any time for any reason. Just as the existence of a risk factor cannot predict suicidality, risk factors for homicide in clients cannot predict whether the client will act in a violent manner (Van Brunt, 2015).

The FBI Unified Crime Report (2019) reports the following:

- 78.3% of the 13,927 murder victims were male.
- When race was provided for the 13,927 murder victims, 54.7% were Black, 42.3% were White, and 3.1% were of other races. Data on race was not included for 234 victims.
- More than 73% of the homicides involved the use of firearms.
- 28.3% of homicide victims were killed by someone they knew other than a family member (acquaintance, neighbor, friend, boyfriend, etc.), 13.0% died at the hands of a family member and 9.9% were murdered by a stranger.

Berghuis and Klott (2015) noted that, among other behaviors, males who become homicidal tend to exhibit impulsivity and have prevalence of mental illness, family violence, job instability, and overall insecurity in their social contexts. They usually express a need to control intimate relationships and may exhibit possessiveness and rage. The need to control others is most clearly demonstrated in domestic violence cases where batterers control their partners through threats, physical violence, isolation from friends and family, and limiting access to money. Emotional reactivity and acting out

of jealousy and revenge were found to be common themes among domestic violence perpetrators (Juodis et al., 2014a).

Research provides information about the characteristics and risk factors associated with youth who commit acts of violence and homicide. According to the CDC (2023a), homicide is the third leading cause of death for youth between the ages of 10 and 24 and the leading cause of death for non-Hispanic Black youth. Sexual minority teens are more likely to experience multiple forms of violence compared to their heterosexual peers. Furthermore, the report outlines various risk and protective factors, although it cautions that risk factors are not direct causes of youth violence. A combination of individual, relationship, community, and societal factors contribute to the risk and prevention of youth violence.

- **Individual risk factors:** History of violence victimization; attention deficit or learning disorders; history of early aggressive behavior; use of drugs, alcohol, or tobacco; low IQ; poor behavior control; deficits in social cognitive or information-processing abilities; intense emotional distress; history of treatment for emotional problems; antisocial traits and beliefs; exposure to violence
- **Family risk factors:** Authoritarian parents; inconsistent disciplinary practices; low parental involvement; low emotional attachment to parents or caregivers; low education and income of parents; substance use or criminal incidents by parents; poor family functioning; lack of supervision of children
- **Peer and social risk factors:** Delinquent peers; involvement in gangs; social rejection by peers; lack of involvement in acceptable activities; poor academic performance; low commitment to school and school failure
- **Community risk factors:** Diminished economic opportunities; high level of transiency; high level of family disruption; low levels of community participation; socially disorganized neighborhoods

Protective factors would be the opposite of the risk factors listed above.

School Homicide and Violence

The Association for Supervision and Curriculum Development (ASCD; 2019) reminds readers and members of K–12 school personnel that there is not a simple or obvious profile of a student who might enact a school shooting. Though school shootings result in extensive media coverage and collective trauma within communities and beyond, the prevalence of gun violence off the school campus is still substantially higher than on campuses. ASCD reported that the average number of deaths from gunshot wounds between 2002 and 2014 was 1,300 per year, compared with 117 *total* fatalities in school locations between 2000 and 2014. Despite the tragic and vexing aftermath of school shootings, educational settings may be considered safer than other settings that children frequent. Everyone who cares about safer communities and schools wants to know who that child may be that could become a school shooter, yet the ability to predict or "spot the profile" of such a youth is elusive. ASCD reported that, while most school shooters are White male teens, their academic and behavioral histories were unremarkable. They experienced some successes in school, had friends or a social group, and were relatively typical, leading the FBI and the U.S. Secret Service to advise against using profile analysis as a prevention approach.

That said, in recent years, it has become tragically evident that K–12 schools and higher-education settings are vulnerable to heinous acts of violence. The FBI (2019) tracked active shooter events from 2000 to 2013 and found a steady increase over that time span. Of those incidents, 24.4% occurred in educational settings. Moreover, the incidents that occurred on campuses accounted for some of the highest casualty rates.

As with violent incidents in other settings, school shootings are precipitated by a complex interplay of personal issues and environmental circumstances. Again, it is important to note that the presence of certain risk factors cannot predict actual violent acts, but since most violent students communicate their intentions, ongoing threat assessment can help school officials prevent violence and provide intervention to troubled students. By using systematic threat assessment and being attuned to warning signs, educational settings can more clearly identify individuals with violent intent. According to Meloy et al. (2011), "warning behaviors are acts which constitute evidence of increasing or accelerating risk . . . acute, dynamic, and particularly toxic changes in patterns of behavior which may aid in structuring a professional's judgment that an individual of concern now poses a threat" (p. 256). School personnel and crisis staff are certainly encouraged to maximize any efforts within the social and emotional development initiatives schoolwide to equip students with increasing emotional self-awareness, strong coping skills, help-seeking skills, conflict resolution skills, and more.

Meloy et al. (2011) developed a typology of eight warning behaviors that can indicate amplified risk of targeted violence and should be utilized when assessing a threat. They state that "threat assessment is concerned almost wholly with the risk of targeted violence by a subject of concern and has a behavioral and observational policing focus" (p. 257). The eight warning behaviors are as follows:

1. *Pathway warning behavior*—the individual plans an attack
2. *Fixation warning behavior*—the individual becomes obsessed with a person or cause
3. *Identification warning behavior*—the individual identifies with attackers or warriors
4. *Novel aggression warning behavior*—the individual tries out violent behavior as a test
5. *Energy burst behavior*—the individual increases activities related to the intended victim(s)
6. *Leakage warning behavior*—the individual tells another person about the plans
7. *Directly communicated threat warning behavior*—the individual makes an explicit threat to the target or police
8. *Last resort warning behavior*—the individual displays an increase in distress and comes to believe that there is no other way forward but violence toward the target (Meloy & O'Toole, 2011)

It is hoped that this typology of warning behaviors can make threat assessment more systematized and usable.

According to Meloy and O'Toole (2011), individuals leak their plans for a variety of reasons, such as excitement, attention, anxiety, or fear. Leakage may be unintentional or intentional. If leakage occurs during counseling, the counselor should discern the need to follow duty to warn responsibilities. O'Toole (2014) noted that individuals who are withdrawn, detached from others, and think they have been attacked by another person (whether the attack is real or imagined) may begin to plot a revenge attack. Erford et al. (2019) also state that revenge for bullying or social rejection is a specific motivation for

many school shootings. Bullying prevention must be considered an important facet of any school violence prevention program (Duplechain & Morris, 2014). The U.S. Secret Service (2010) offered this insight into the prevalence of leakage:

> School-based attacks are rarely impulsive acts. Rather, they are typically thought out and planned in advance. Almost every attacker had engaged in behavior before the shooting that seriously concerned at least one adult—and for many had concerned three or more adults. In addition, prior to most of the incidents, other students knew the attack was to occur but did not alert an adult.

This information points to the need to educate students, school staff and faculty, and the public about leakage. Violence in school is usually planned, and school staff and faculty need to be particularly attentive to signs and symptoms of violence as well as to school environmental factors that give rise to bullying, rejection, and the promotion of violence (Miller et al., 2000).

Kanan (2010) highlighted the following 10 findings on violent incidents in schools:

1. Incidents of violence are rarely impulsive acts.
2. Prior to most incidents, other people knew about the attacker's ideas or plans.
3. Most attackers did not threaten their targets directly prior to advancing the attack.
4. There is no active or useful profile of students who engage in targeted school violence.
5. Prior to the attack, most attackers engaged in some behavior that caused concern or indicated a need for help.
6. Most attackers were known to have difficulty coping with significant losses or personal failures.
7. Many attackers felt bullied, persecuted, or injured by others prior to the attack.
8. Most attackers had access to and had used weapons prior to the attack.
9. In many cases, other students were involved in some capacity.
10. Despite prompt law enforcement responses, most shooting incidents were stopped by means other than law enforcement interventions. (p. 32)

A key point to consider in school shootings is the fact that attackers usually indicate their intentions to someone. Most of the time, other students had some hints that a fellow student was ready to lash out. Part of the difficulty that schools face is the fact that students are reluctant to tell adults their suspicions. Payne and Delbert (2011) noted that the use of an anonymous hotline for students to report threats, mistreatment, or bullying can be an effective deterrent to incidents of school violence. Prevention messaging, such as "if you see something, say something," or training youth to get help from an adult are important school context norms to advance.

In general, college campuses are safe environments, and there is a lower rate of crime on college campuses compared to off-campus settings (Carr & Wood, 2006). Horrific shootings on college campuses are more reflective of problems with mental health access and compliance than of problems with campus safety. Colleges and universities can combat violent incidents through training for faculty, staff, and even students. The implementation of threat assessment and prevention programs on campus and campus-wide warning systems could provide the means to thwart future acts of violence and protect student lives. Campus leadership can continue to improve

access to mental health care, foster social and emotional support in residential facilities, and improve morale across groups (Van Brunt, 2015). Case Study 6.2 addresses the importance of risk assessment in schools.

CASE STUDY 6.2

Risk Assessment in Schools

Peyton is a grade 10 student who is very withdrawn and has few friends. Dalton, the school counselor, is concerned about Peyton. Last year there was a case where some students were bullying Peyton, who then shared thoughts of suicidal ideation with Dalton. Peyton has been seeing a counselor for the past year and the bullies were punished; however, Dalton has a gut feeling that the bullying continues "under the radar" of school officials. Peyton's English teacher recently stopped by to show Dalton one of Peyton's tests. Peyton did not answer a single question, but instead drew pictures of guns all over the test.

Discussion Questions

1. What should Dalton do?
2. What questions should Dalton ask?
3. What process should Dalton follow?
4. Are there any special considerations since they are in a school setting?
5. Are there any gender considerations?

Workplace Violence and Homicide

In 2017, there were 458 workplace homicides (Doucette et al., 2019) and, according to the Bureau of Labor Statistics (2021), 20,870 workers in the private industry experienced trauma from nonfatal workplace violence in 2019. Of those victims who experienced trauma from workplace violence:

- 68% were female
- 65% were age 25–54 years
- 70% worked in the health care and social assistance industry
- 21% required 31 or more days away from work to recover, and 20% needed 3–5 days away from work

However, the number of workplace homicides has decreased overall since 1993, a year in which 1,068 workplace homicides occurred. Even with this decrease over the years, homicide in the workplace is the fourth leading cause of fatality at work and remains a disturbing reality. Most workplace homicides involve robberies, and individuals who work in retail and hospitality establishments are at greater risk, as are customers and bystanders in these settings. Approximately 80% of workplace fatalities result from shootings.

Just as a school can be held liable for not addressing bullying and threats, the workplace can also be liable for failing to address danger and threats (Lies & Simonsen, 2015). Lies and Simonsen recommend that workplaces have clearly stated policies

regarding violence and threats, a way for employees to communicate concerns, a system to promptly investigate concerns, a discipline policy, training for all employees on the signs and symptoms of violent intent, and an Employee Assistance Program that can counsel employees in distress to help prevent acts of violence.

According to the CDC (2021a), the most dangerous workplace risk factors involve the following:

- Contact with the public
- Exchange of money
- Delivery of passengers, goods, or services
- Having a mobile workplace such as a taxicab or a police cruiser
- Working with unstable or volatile persons in health care, social service, or criminal justice settings
- Working alone or in small numbers
- Working late at night or during early-morning hours
- Working in high-crime areas
- Guarding valuable property or possessions
- Working in community-based settings

The risk of workplace violence and homicide increases during times of extreme job stress. Understaffed job sites, overworked employees, and times of downsizing or labor disputes will create a tense and potentially dangerous context for the development of violent behavior. Additionally, poor management, a high number of grievances, and the lack of employee access to counseling can contribute to violence and homicidal threat in the workplace (O'Toole, 2000). It is essential that workplace employers, particularly in high-pressure workplaces or those undergoing turmoil, be mindful of the need for ongoing threat assessment and be proactive by providing support to employees.

Threat Level Assessment

Various models of threat assessment have emerged over the past years, and it is beyond the scope of this chapter to outline all of them. Counselors are encouraged to familiarize themselves with a variety of models of threat assessment.

When a client has overtly indicated intent to harm others or has indirectly exhibited predictors or behaviors of violence and harm to others, this constitutes a direct threat, and the counselor must assess the level of threat (Warren et al., 2011). Threat level assessment is critically important to ensure the safety of others and to appropriately guide treatment decisions. While most people who make threats are unlikely to carry out a violent action, all threats must be taken seriously and evaluated (O'Toole, 2000). Threat assessment is the evaluation of the lethality of a threat through an examination of motive, risk factors, intent, and the means and ability to enact the threat.

O'Toole (2000) explained that threat assessment should involve exploration of the following questions: What are the details of the threat? Are the victims identified? Are the details logical, plausible, and spelled out in a specific manner (time of day, method)? Does the client have the means to carry out the threat? What is the emotional state of the client? While emotionality does not specifically indicate lethality, knowing the affective condition of the client provides important assessment and diagnostic information related to lability of mood, dysregulation of feelings, ability to think clearly or not, rational

functioning or not. Are there stressors or triggers that might predispose a client to violence? Is the level of threat low, medium, or high? O'Toole, working with the FBI National Center for the Analysis of Violent Crime, delineated threat levels as follows:

> ***Low Level of Threat:*** A threat that poses a minimal risk to victim and public safety.
>
> The threat is vague and indirect.
> Information contained within the threat is inconsistent.
> The threat lacks realism.
> The content of the threat suggests the person is unlikely to carry it out.
>
> ***Medium Level of Threat:*** A threat that could be carried out, although it may not appear to be entirely realistic.
>
> The threat is more direct and more concrete than a low-level threat.
> The wording in the threat suggests that the person issuing the threat has given some thought to how the act will be carried out.
> There may be a general indication of a possible place and time (though these signs still fall well short of a detailed plan).
> There is no strong indication that the person who issued the threat has taken preparatory steps, although there may be a veiled reference or ambiguous or inconclusive evidence pointing to that possibility—an allusion to a book or movie that shows planning of a violent act or a vague general statement seeking to convey that the threat is not empty: "I'm serious!" or "I really mean this!"
>
> ***High Level of Threat:*** A threat that appears to pose an imminent and serious danger to the safety of others.
>
> The threat is direct, specific, and plausible.
> The threat suggests that concrete steps have been taken toward carrying it out—for example, statements indicating that the person who issued the threat has acquired or practiced with a weapon or has had the victim under surveillance. (pp. 8–9)

The process of threat assessment must be conducted in an environment that will calm the client, facilitate the de-escalation of intense emotions, and assist in the attainment of precise information regarding the level of threat intent. The assessment of violence is like the assessment of other symptoms, in that the client's history of violence, family and medical history, mental status, and drug use must all be investigated. Van Brunt (2015) underscored the need for a calm demeanor and the use of basic rapport-building and active listening skills. Using counseling skills, a relationship can be formed, and the clients will be more willing and able to open up about their thoughts and potential plans. Law enforcement representatives assert that the active listening skills that counselors learn in their interpersonal listening classes are very similar to the rapport-building strategies that crisis response or SWAT team members use in police or FBI work (R. Jackson, personal communication, April 15, 2015; C. Spears, personal communication, February 10, 2022).

Once rapport is established, the counselor can delve into precise questions to obtain detailed information about the intent of the client to harm another, the extent

and intrusiveness of the homicidal thoughts, and the ability of the client to acquire the means to implement homicide plans. When dealing with a client expressing homicidal intent, crisis counselors must always keep the ethical obligation of public safety and the duty to warn at the forefront of their minds, even with low-threat clients. Furthermore, while assessing homicidal ideation, the counselor should also assess for suicidal ideation because homicide and suicide may co-occur. Now see Case Study 6.3, which presents a situation where a counselor must assess a client's level of threat. Then read Voices from the Field 6.3 and complete Activity 6.3 on threat assessment.

CASE STUDY 6.3

Assessing for Level of Threat

RILEY: I hate my supervisor at work. My life would be better if he was dead.

COUNSELOR: You're so angry at your supervisor that you wish he was dead. Have you had thoughts about harming him?

RILEY: Sure, I think about killing him.

COUNSELOR: Have you come up with a plan to kill him?

RILEY: A plan? I mean I've got a 45 at home but no, not a plan exactly.

COUNSELOR: Are there times when you feel you might act on these thoughts?

RILEY: Ha, whenever I'm drunk.

Discussion Questions

1. What level of threat is indicated so far?
2. What other information does the counselor need to assess the threat level?
3. If the counselor determines the supervisor is in danger, what steps should the counselor take?

VOICES FROM THE FIELD 6.3

Leaving No Stone Unturned

Cheryl Lewellen

When assessing for homicidal ideation of potential clients to our nonprofit women's residential substance use treatment facility, we begin by using a state assessment issued by the Department of Mental Health. I am sure most states have a similar standardized assessment. It takes more than an hour to perform and assesses for multiple factors, with both suicidal and homicidal ideation included.

Given the nature of clients struggling with addictions, it is important to continue to verbally assess for both suicide and homicide during individual sessions throughout the 90 days they are here. One of the main factors for this is that many of the women come into

(*continued*)

the program insufficiently detoxed, and it takes a while for them to gain a clear perspective on their lives. As they begin treatment and open up to aspects of their lives that caused them to use drugs and alcohol in the first place, the clients become aware of the multitude of emotions they had attempted to numb. Many of the clients have suffered abuse on multiple levels either before or during their drug use. Therefore, it is always important to monitor their potential ideation of both suicide and homicide.

As with assessment of suicidal ideation, assessment of homicidal ideation involves asking the client questions to establish whether an individual has thoughts about harming or killing another person, whether they have a plan, and to what extent the plan could be enacted.

ACTIVITY 6.3

Think about counseling a client who requires a threat assessment. The literature emphasizes the need for counselors to maintain calm and use facilitative skills to build rapport while conducting an assessment for homicidal ideation. How difficult would you find that to be? Put yourself in the situation: What concerns you? What do you feel you need to master threat assessment?

Referral

Because of the danger to others and the possible existence of mental disorders in homicidal clients, counselors usually refer clients to or work collaboratively with other mental health providers, particularly psychiatrists, when treating clients who pose a threat to others. Violent clients often need the help of specialists in forensic mental health, and it is a good idea for crisis counselors to be familiar with forensic specialists in their local area.

Clinical assessment of low-threat-level clients may reveal the need for psychiatric evaluation and treatment with psychotropic medication; however, it is imperative that clients with homicidal ideation who are assessed at the high and medium threat levels be referred for psychiatric evaluation and possible treatment with medications. Because of imminent danger to others, high-threat-level clients require immediate law enforcement involvement and probable voluntary or involuntary commitment (O'Toole, 2000; Warren et al., 2011). Medium-threat-level clients need to be monitored closely and may need to be hospitalized.

In instances with out-of-control and impulsive clients, homicidal and suicidal ideation may coexist and, therefore, such a client poses a threat to both self and others. As with suicidal crises, practitioners should be familiar with their state and county protocols for voluntary and involuntary commitment hearings with potentially homicidal persons. The procedures for voluntary and involuntary commitment discussed previously in this chapter in relation to suicide should be followed for homicide cases, with the inclusion of law enforcement in high-threat-level situations. Personal safety of the crisis counselor should always be a priority. A counselor should never attempt to physically stop a resistant client from leaving. If the client is intent on leaving, it is best to get as much information as possible about the client and request law enforcement to intervene.

What is the relationship between mental illness and homicide? Individuals with diagnosed mental disorders are at a higher risk than the general population for violent or homicidal behavior (Laajasalo & Hakkanen, 2004), and individuals with untreated mental illness are more likely to threaten to kill (Warren et al., 2011). It should be noted, however, that this risk is not present across the board and that an elevated threat to

others is mainly associated with certain diagnoses (Eronen et al., 1998). An increased risk of violent behavior has been found with diagnoses of schizophrenia and other psychotic disorders, but the highest risk of violent and homicidal behavior has been found in individuals with a dual diagnosis of antisocial personality disorder and substance use disorder. Substance use repeatedly emerges as a major risk factor for violent behavior and homicide; the comorbidity of substance use and any mental disorder increases the likelihood of violence and should be considered when assessing and treating violent clients.

Treatment Options

As stated earlier, a therapeutic alliance is particularly imperative with potential homicide offenders, since client investment in treatment compliance is necessary for public safety (Kolski et al., 2015). Tishler et al. (2000) noted that mental health providers may struggle with extending empathy and may experience fear when working with homicidal clients. While such reactions are understandable, it is important to remember that homicidal behaviors are symptoms of a client's illness and that open displays of fear and distaste undermine rapport building and impair treatment outcomes.

Once the immediate homicidal crisis has been dealt with, the safety of others has been assured, and appropriate medical treatment has begun, long-term treatment planning can proceed. Ongoing assessment of the client is crucial to both treatment outcomes and public safety; therefore, assessment must remain foremost in the crisis counselor's mind. One concern in assessment is the tendency for violent offenders to be less than forthright about the facts surrounding violent acts (Carroll et al., 2004). Crisis counselors should thoroughly examine client history and obtain external substantiation of client facts. Suicidal ideation and homicidal ideation often co-occur, and it is important to assess the risk of both suicide and homicide in clients who have thoughts of harm to self or others.

By understanding how aggression may have been reinforced in the client's past and by exploring any inhibiting factors and situational contexts, crisis counselors and clients can identify thoughts and behaviors that may lessen violent behavior in the future. The client can begin to learn other ways to cope with anger and aggression. See Case Study 6.4, which presents a situation in which a counselor must assess a client's level of threat.

CASE STUDY 6.4

Assessing for Level of Threat: Judy

Judy is a 43-year-old single woman who has been placed on 2 weeks' administrative leave from her job as a research and development technician at a chemical company. Over the years, she has had many personality conflicts with other workers, but her expertise is valuable and these conflicts have always been smoothed over and worked out. Recently, Judy "lost it" with her boss and flew into a rage when she was denied time off in compensation for working late the previous week; this incident was the basis for her administrative leave. Her family doctor has prescribed antidepressants for her and has referred her to you for counseling.

Judy expresses anger and a sense of hopelessness about her situation. She is distraught and drinks wine every night just to go to sleep. She is convinced that she will be fired and that the boss is using these 2 weeks to build a case against her. Judy expresses to you how valuable she is to the company and how much knowledge she has, including her knowledge of tasteless but deadly poisons. She states, "Just one minute in the break room is all someone would need; just slip it in the coffee and they wouldn't know what hit them."

Discussion Questions

1. What level of threat is indicated in this case?
2. Are there factors that increase the homicide risk in this client?
3. What actions should you take?

Compliance with treatment—in particular, compliance with medication—must be stressed with clients who have indicated homicidal ideation or intent. Nordstrom et al. (2006) studied convicted homicide offenders who had been diagnosed with schizophrenia. Most offenders were found to have been noncompliant with medication and treatment at the time of their crimes, resulting in active hallucinations and delusions during the homicidal crisis. Therefore, it is essential that crisis counselors conduct ongoing evaluations of medication compliance throughout treatment.

Family involvement in the treatment of homicidal clients is critical as a support for ongoing treatment compliance. Families can communicate to counselors if clients stop taking medication and can also report observed behaviors of concern that might indicate elevated risk (Kolski & Jongsma, 2015). Additionally, families provide a social network foundation that can facilitate client coping. In some instances, family members need to be given information to ensure their own safety. Family members and friends, rather than strangers, were the most frequent victims of homicide offenders with a diagnosis of schizophrenia (Laajasalo & Hakkanen, 2004). This fact highlights the need for families to be an active part of collaborative treatment and ongoing evaluation of homicidal clients. Counselors should establish and adhere to the terms of confidentiality with the primary client and update any informed consent about how information is shared and for what reason and when those boundaries would result in exceptions.

Early environmental factors influence the development of mental disorders and violent behavior; thus, early intervention with children at risk and their families may deter the development of future antisocial behavior, including homicidal behavior (Laajasalo & Hakkanen, 2004). Schools provide the first opportunity for intervention with children who are exhibiting behavioral problems and struggling with academics, both of which are associated with violence. Farmer et al. (2007) recommend a service delivery structure that supports intervention and prevention to promote academic achievement and social skills, intervention with at-risk youth, and developmental systemic prevention strategies. It should be noted that such a delivery structure can be achieved by the full implementation of the *ASCA National Model: A Framework for School Counseling Programs* (American School Counselor Association, 2019). Also, recent research on adverse childhood experiences has provided more clarity about the debilitating impact of chronic, toxic stress on young children. Early intervention

in concert with pediatricians, early education teachers, and other community partners when possible is recommended.

Needs of Survivors of Homicide Loss

Homicide survivors may experience symptoms consistent with complicated grief due to the nature of the loss. According to the Mayo Clinic (2016), several symptoms may accompany complicated bereavement:

- Extreme focus on the loss and reminders of the loved one
- Intense longing or pining for the deceased
- Problems accepting the death
- Preoccupation with one's sorrow
- Bitterness about the loss
- Inability to enjoy life
- Depression or deep sadness
- Difficulty moving on with life
- Trouble carrying out a normal routine
- Withdrawing from social activities
- Feeling that life holds no meaning or purpose
- Irritability or agitation
- Lack of trust in others

To heal from violent loss, survivors need to feel able to openly express what has happened to them and to fight for change in a system where they have felt injustice. It is also important that survivors find meaning in the death of their loved one by working to benefit others experiencing similar circumstances; this gives meaning to the loss as well as purpose to the survivor (Rynearson, 2012). Saindon et al. (2014) examined the use of restorative retelling, a technique where clients deal with internalized trauma and commemorate the life of the lost loved one from a strengths-based and resilient stance. They found that restorative retelling showed promise as an intervention, particularly regarding easing depression, lessening avoidance, and decreasing prolonged grief. Support groups tend to provide much healing in feeling connected to others who share a similar experience of death connected with violence.

Crisis counselors responding to the crisis of homicide should be prepared for complex and varied reactions and symptoms from individual survivors. Individuals and entire communities can be altered by homicide. At college campuses where a homicide has taken place, students may no longer feel safe and the stress and anxiety may affect their academic achievement. Communities touched by homicide are altered by the brutal reality of murder within their midst; a coordinated community counseling response and activation of resources can serve to facilitate healing. Additionally, crisis counselors should be sensitive to cultural differences in grief expression. Clients from expressive cultures will show more outward signs of grieving. A lack of outward grief expression may be indicative of a more emotionally restrictive culture, and crisis counselors should not underestimate the inner grief state of less expressive clients (Cavaiola & Colford, 2018). See Case Study 6.5, which focuses on grief counseling for parents who have lost a daughter to homicide. Then read Voices from the Field 6.4. Finally, Tables 6.8–11 provide resources for helping people through suicide, NSSI, and homicide.

CASE STUDY 6.5

Treatment for Families of Homicide

Debbie, a 19-year-old college student, did not show up for classes or for her job one day. When she did not answer her cell phone or come home that night, her roommate was worried and contacted Debbie's family, who decided to report her as a missing person. After 3 weeks of searching and televised pleas for her return, Debbie's body was found in a wooded area 30 miles from her apartment. She had been sexually assaulted and strangled. DNA found on her body was a match with DNA from a man out on parole from a previous rape conviction. He was accused in Debbie's murder. The killer claimed that he had consensual sex with Debbie in his car and that she was fine when he dropped her off at campus. Debbie's parents were appalled when the killer's defense team attempted to portray Debbie as a wild, out-of-control college student who partied too much, had multiple sex partners, and put herself at risk. The trial ended with a hung jury, and there is uncertainty about when there will be a new trial. Debbie's parents have come to you for grief counseling.

Discussion Questions

1. What factors have contributed to the parents' complicated bereavement?
2. What emotions and reactions might you expect from the parents in this case?
3. What treatment goals and interventions are called for in this case?

VOICES FROM THE FIELD 6.4

Reacting in Crisis Situations

Gregory Pollock

The calm of the day was interrupted by the loudspeaker calling a code for violent behavior being exhibited by a client. I quickly dropped what I was doing and responded to the code. As I approached the area, the commotion of the situation became louder and louder, as my adrenaline levels increased quickly with each step closer to the area. I felt my breath become shallower, my stomach drop, and my body become tense. I had to consciously step back and take a breath and attempt to relax prior to entering the room where the client in question was out of control and becoming violent. How many of us as counselors have been faced with similar situations?

I have learned the hard way that approaching situations such as these while hyped up and excited only adds to the tension and worsens the situation. I think back to my early days in this field where I witnessed staff enter crisis situations in a frustrated state and saw how this exacerbated the situation in very negative ways, at times leading to the staff or client becoming injured.

It is imperative to remain calm when reacting in crisis situations so that we do not add to the situation at hand. I frequently hear from counselors-in-training that they do not feel that they could remain calm in a crisis and that when faced with a crisis, they seem to "forget" all their skills. On the contrary, their skills are not forgotten but often masked with anxiety, fear, adrenaline, and stress reactions. So how is it possible to remain calm and keep our wits about us when faced with a crisis? Our training programs do a great job of preparing us for a variety of situations using counseling skills and techniques. Not all counselors have equal opportunities to practice in crisis situations, and those working in situations where crises are more common will have a greater comfort level in dealing with crisis incidents.

It is helpful to remain mindful of ourselves and recognize what is going on internally when entering a

regular counseling session or a crisis. How aware are we of our feelings and thoughts when walking into crisis situations? It is important to control our emotions as we enter crisis situations with the best interest of the client at heart, much like a regular counseling session. There are great consequences to not remaining mindful lest we lose sight of what is needed to best remedy the situation we face.

I refer to a yogic phrase, *Ahimsa*, meaning to do no harm. This is a great term to keep in mind as we prepare for and enter crises. The best way to ensure that we do no harm to ourselves in yoga is by being aware of ourselves and staying in touch with our bodies to make sure that we do not strain ourselves. It may be difficult to understand the amount of influence that we have on situations and controlling our emotions goes a long way in enabling us to enter stressful situations in a calm manner.

Maintaining awareness daily becomes very difficult, especially considering our listening to and taking on the issues and circumstances of our clients, leaving us vulnerable to compassion fatigue, which in turn can lead to a loss of control in crises. Self-care is vitally important to help us deal with underlying frustrations so that we can maintain our emotional stability.

The most important thing to focus on in crises is maintaining safety of self, the client, and other staff. We also need to focus on what the client needs in crisis situations, and we need to be open to empathizing, understanding, and meeting the needs of the client in a respectful, nonharming manner while approaching these situations with compassion and a calm, even temperament.

TABLE 6.8 Resources for Suicide, NSSI, and Homicide

Organization	Website
American Foundation for Suicide Prevention Site for Loss Survivors	http://afsp.org/find-support/ive-lost-someone/
Alliance of Hope	http://www.allianceofhope.org
American Association of Suicidology (AAS)	www.suicidology.org. Click on the Resources tab. Scroll down to find Attempt Survivor and Loss Survivor resources.
AAS: Suicide Loss Survivors	https://suicidology.org/resources/suicide-loss-survivors/
Center for Deployment Psychology, Clinicians as Survivors of Loss	http://deploymentpsych.org/blog/staff-perspective-suicide-postvention-don%E2%80%99t-forget-clinician-survivors
Department of the Air Force Resilience	https://www.resilience.af.mil/Postvention-Tools/
Friends for Survival	www.friendsforsurvival.org
Harvard Medical School, Harvard Health Publications, Harvard Women's Health Watch	http://www.health.harvard.edu/newsletter_article/Left-behind-after-suicide
HEARTBEAT: Grief Support Following Suicide	http://heartbeatsurvivorsaftersuicide.org
National Action Alliance for Suicide Prevention	www.actionallianceforsuicideprevention.org. Use the search option to find resources. Try using key words such as suicide loss, suicide grief, after a suicide, suicide bereavement, postvention, and lived experience.
National Suicide Prevention Lifeline	https://suicidepreventionlifeline.org/help-yourself/loss-survivors/
Parents, Friends and Families of Suicide	http://pos-ffos.com

(*continued*)

TABLE 6.8 Resources for Suicide, NSSI, and Homicide (*continued*)

Organization	Website
Pennsylvania Adult/Older Adult Suicide Prevention Coalition	https://www.preventsuicidepa.org/?s=loss. Use the search option to find resources. Try using key words such as suicide loss, suicide grief, after a suicide, suicide bereavement, postvention, and lived experience.
Sibling Survivors of Suicide Loss	http://www.siblingsurvivors.com
SPRC website	www.sprc.org. Use the search option to find resources. Try using key words such as suicide loss, suicide grief, after a suicide, suicide bereavement, postvention, and lived experience. For clinician survivors: https://www.sprc.org/resources-programs/clinicians-survivors-after-suicide-loss
Suicide Awareness Voices of Education (SAVE)	https://www.save.org/what-we-do/grief-support
Suicide Resource & Response Network	https://srrn.net/aftercare/
Survivors of Suicide	http://www.survivorsofsuicide.com
Survivors of Suicide Loss – San Diego	https://www.soslsd.org
The Dougy Center for Grieving Children and Families	www.dougy.org; https://www.dougy.org/resources?how=suicide&who=&type=&audience=. Scroll down this page to find resources for suicide and grief.
The Guide (for schools)	http://theguide.fmhi.usf.edu. Note that this is primarily designed for schools and school systems, but their materials are easily transferable to other types of organizations. Use the search option to find resources. Try using key words such as suicide loss, suicide grief, after a suicide, suicide bereavement, postvention, and lived experience.

TABLE 6.9 Resources for Suicidology, Suicide Prevention, Best Practices, and Intervention

Organization	Website
Alabama Suicide Prevention and Resources Coalition (ASPARC) contains resource directory and materials for professionals and families	www.asparc.org
American Association of Suicidology (AAS)	www.suicidology.org
American Foundation for Suicide Prevention (AFSP)	https://afsp.org

TABLE 6.9 Resources for Suicidology, Suicide Prevention, Best Practices, and Intervention (*continued*)

Organization	Website
Be the 1 To (an initiative of the NSPL)	www.bethe1to.com
Centre for Suicide Prevention (Canada)	https://www.suicideinfo.ca/
Didi Hirsch MHS (premiere crisis center in California, closely related to the work of Dr. Edwin Shneidman, "grandparent" of suicidology)	https://didihirsch.org
International Association for Suicide Prevention (IASP)	https://www.iasp.info
Jason Foundation	http://jasonfoundation.com
JED foundation	https://www.jedfoundation.org
Kevin Hines (mental health advocate and survivor, lived experience)	https://www.kevinhinesstory.com/
Living Works A.S.I.S.T. Training and SafeTalk Training (a Canadian company that provides extensive training in the U.S.)	https://www.livingworks.net/asist
Man Therapy (addresses many mental health needs of men, suicide-related content among them)	www.mantherapy.org
National Action Alliance for Suicide Prevention (often referred to as the Action Alliance)	https://theactionalliance.org
National Organization for People of Color Against Suicide (NOPCAS)	http://nopcas.org/
Olweus Bullying Prevention Program	www.olweus.org
QPR Institute	https://qprinstitute.com/
Speaking of Suicide Elevating the Conversation; based on Freedenthal (2017).	https://www.speakingofsuicide.com/
Stamp out Stigma	http://stampoutstigma.com
StopBullying.gov	www.stopbullying.gov
Suicide Prevention Resource Center (SPRC)	https://www.sprc.org
Suicide Awareness Voices of Education (SAVE)	https://save.org/about-suicide/preventing-suicide/
Suicide.org, Suicide Prevention in the Military	www.suicide.org/suicide-prevention-in-the-military.html
The GUIDE (designated best–practice model for school-based suicide prevention, intervention, and postvention care by the University of South Florida)	http://theguide.fmhi.usf.edu
The Trevor Project	https://www.thetrevorproject.org

(*continued*)

TABLE 6.9 Resources for Suicidology, Suicide Prevention, Best Practices, and Intervention (*continued*)

Organization	Website
Terry Wise (the lived experience, award-winning author, speaker, advocate)	https://terrywise.com
United Survivors	https://unitesurvivors.org
Zero Suicide Alliance (Britain/UK)	https://www.zerosuicidealliance.com
Zero Suicide Institute	http://zerosuicideinstitute.com https://zerosuicide.edc.org

TABLE 6.10 Resources on Homicide

Center for Homicide Research	www.Homicidecenter.org
Justice for Homicide Victims	https://justiceforhomicidevictims.org/resources/
National Organization for Victim Assistance	www.trynova.org

TABLE 6.11 Resources for NSSI

Counselors may find numerous resources, journal articles, research, YouTube videos, and more to learn more about NSSI. A few helpful websites to explore are:

Help Guide	https://www.helpguide.org/articles/anxiety/cutting-and-self-harm.htm
S.A.F.E. Alternatives	https://selfinjury.com/referrals/programs/
SELF	https://www.self.com/story/self-harm-friend-help
Self-Injury Recovery Anonymous	https://www.thesira.org/
The Recovery Village	https://www.therecoveryvillage.com/mental-health/self-harm/related/self-harm-statistics/#gref

SUMMARY

Suicide, NSSI, and homicide are crises that counselors will encounter during their careers. Suicide rates are increasing among all segments of our population, and this trend is expected to continue despite the extensive prevention efforts ongoing at school, local, state, and national levels. Counselors and other response-oriented personnel may find that their own anxiety surrounding these risk assessments may affect their approaches to helping clients. In addition to ongoing training, periodic self-examination is therefore necessary to maintain an adequate degree of differentiation from the client's crisis, to prevent countertransference of the crisis counselor's anxieties onto the client, and to avoid damaging the potential for a positive counselor–client relationship.

No one universally recognized rubric for predictability of suicide, NSSI, or homicidal behaviors exists; however, it is a standard of care to approach these risk assessments with models for assessment to provide effective intervention. Three primary goals to help crisis counselors dealing with a person threatening suicide or

homicide are (1) ensure client safety, (2) assist the client in achieving immediate short-term mastery of self and situation, and (3) connect the client with formal and informal supports. A variety of models were presented to help the crisis responder achieve these goals.

Knowledge of suicide risk factors, warning signs, and protective factors is essential for the crisis counselor to make a qualified assessment of suicide risk in clients who are struggling with thoughts of death to end pain. Once the client is assessed, the counselor formulates the level of risk and actively monitors the client to make progress toward symptom and risk reduction and when subsequent events affect the client's disposition. Documentation of the entire process is an increasingly important part of the assessment and treatment cycle; documentation is the single most important factor in justifying and defending a particular course of treatment action. Implementing NSSI risk assessment should now be part of any suicide risk assessment. Counselors should be aware of basic, contextual, and functional aspects to include in a Q-tree for an NSSI assessment in order to gather the information necessary to refer clients to the most effective treatment.

Responding to incidents of homicide presents many of the same kinds of challenges as responding to a suicide incident, although many crisis counselors underestimate the need for readiness to respond to homicidal risk. In fact, since homicide risk is becoming more frequent, crisis counselors should be prepared to integrate a homicide assessment and determine risk level. Crisis counselors may find themselves in a position if they need to exercise the duty to warn, duty to protect, and duty to consult. Crisis counselors should be aware of their legal duties in these situations.

Survivors of homicide loss often experience complicated bereavement. They may feel stigmatized, isolated, and traumatized by the legal system. Treatment of survivors should address their complicated bereavement issues and help them find resolution and meaning. Support groups for homicide survivors have been found to be helpful in the healing process.

Finally, mental health counselors are reminded to practice effective self-care when working with the stressful situations of their at-risk clients and family members. Crisis counselors are often first responders who pivot a potential tragedy into an alternative narrative of improved well-being of their clients and who also can experience professional gratification for making a difference in the lives of their clients.

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CHAPTER

7 Understanding and Treating Substance Use Disorders with Clients in Crisis

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PREVIEW

This chapter provides an overview of substance use disorders (SUDs) and symptoms and the counselor's role in working with clients facing crisis situations. The etiological factors of SUDs and various models of addiction are reviewed. Sections in the chapter dealing with treatment are arranged to help counselors understand the sequential treatment process in a crisis setting. Special issues such as relapse, support groups, co-occurring disorders and crises, and cultural considerations are discussed. Case studies and activities are provided throughout the chapter to help the reader integrate and apply the material.

LEARNING OBJECTIVES

Upon competing this chapter, readers will be able to:

7.1 Understand the nature of and importance of substance use in society, including classification of drugs and current research.
7.2 Describe the etiology and risk factors of substance use and dependence.
7.3 Articulate procedures for the screening, assessment, and diagnosis of substance use disorders and symptoms, including screening procedures, commonly used assessment instruments, assessment, diagnosis, assessing a client's readiness to change, and treatment admission and placement processes.
7.4 Explain substance use crisis treatment, including relapse and crisis, harm reduction, support groups, couples and family treatment, and co-occurring disorders.
7.5 Explain multicultural perspectives on substance use in crisis situations.
7.6 Identify elements of crisis intervention related to substance use, including safety and self-care, diagnostic considerations during a crisis, and essential substance use–specific crisis intervention skills.

*We thank William R. Sterner for contributions to past editions of this chapter.

SUBSTANCE USE AND SOCIETY

Learning Objective 7.1 Understand the nature of and importance of substance use in society, including classification of drugs and current research.

Throughout history, substances, many of which are now considered illegal in a number of countries, have been used as an integral component of social activities, religious ceremonies, spiritual rituals, and cultural practices. Along with intended sociocultural purposes, substances have also been sought out to alter mood and enhance sensory experience and pleasure. People have continually experimented with mood-altering substances, as well as heuristic routes of administration that both maximize the delivery to, and enhance the effects on, the brain. The list of mood-altering substances is as varied as it is long and includes well-known substances such as marijuana, cocaine, heroin, tobacco, and ethyl alcohol (one of the most widely used and easily accessible mood-altering substances).

Data from the 2019 National Survey on Drug Use and Health (Substance Abuse and Mental Health Services Administration [SAMHSA], 2019) revealed that over 50% of Americans age 12 years and older (about 132 million people) are current consumers of alcohol, and over 21% of Americans age 12 years and older (nearly 58 million people) are current users of tobacco products. Survey results revealed that nearly 57 million Americans reported using an illicit drug in 2019, a proportion of 20.8% of all people 12 years and older. A majority (53%) of those reporting use of nonmedical prescription-type drugs indicated the illicit substances were acquired from family and friends. Of the 20.4 million who reported needing treatment for an illicit drug or alcohol problem, only 2.3 million (11%) received treatment (SAMHSA, 2019).

Based on these data, one may conclude that substance use has evolved beyond sociocultural practices to more self-serving and unhealthy usage. However, what is not clear is the primary intent for use. Reasons for using substances can be varied and complex, as evidenced by the numerous theories developed to explain use and addiction. Prevailing attitudes, however, may not fully acknowledge factors beyond pleasure-seeking motives—factors such as stress, psychopathology, crisis response, pain relief, and grief and loss responses to traumatic events such as natural and human-generated disasters, emergencies, and tragedies. Unfortunately, aggregate data do not delineate the extent to which substance use may in fact be as much about pleasure-seeking as it is about compensating for psychosocial, psychopathological, and sociocultural problems.

Classification of Drugs

The number of different drugs that exist worldwide is unknown. Anecdotal reports estimate that the number of different drugs worldwide may be as high as 100,000. To gain some perspective, Kinch et al. (2014), in their meta-analytic study, reported that by the end of 2013, a total of 1,453 drugs had received U.S. Food and Drug Administration approval. Rather than attempting to quantify drugs, a more effective methodology is to classify substances based on the specific physiological and psychological effects. Current drug classifications include cannabinoids (e.g., marijuana, hashish, dronabinol), depressants (e.g., alcohol, tranquilizers, barbiturates, sleeping pills, inhalants), stimulants (e.g., cocaine, amphetamines,

methamphetamine, caffeine), hallucinogens (e.g., LSD, phencyclidine, ketamine, mescaline, psilocybin), opiates (e.g., heroin, morphine, OxyContin, codeine, methadone), nicotine, and psychotherapeutics (e.g., Prozac, Zoloft, Ativan, Xanax, BuSpar, Cymbalta, Abilify, lithium, Geodon, Zyprexa, Risperdal). Other drugs can have multiple effects and may fall under several classifications. For example, Ecstasy (3,4-methylenedioxymethamphetamine or MDMA), a designer drug popular in the club and bar scene during the mid-1990s, has both stimulant and hallucinogenic effects. Dextromethorphan (also known as DX or DXM), a drug found in many over-the-counter cough syrups, can have both depressant and mild hallucinogenic effects if taken in large doses.

The Comprehensive Drug Abuse Prevention and Control Act of 1970 classified controlled drugs into five schedules based on a continuum of potential for abuse, accepted medical use, and potential for dependence. Drugs listed under Schedule I have the highest potential for abuse and dependence and have no current medical application in the United States (e.g., heroin, LSD), whereas drugs listed under Schedule V have the lowest potential for abuse and dependence and have an established medical application (e.g., cough syrups with codeine).

Research on Substance Use, Crisis, and Society

Several studies documented the relationship between natural and human-generated disasters and subsequent patterns of substance use. A meta-analysis of over 30 studies investigating substance use following terrorist attacks in the United States found an increased use of substances in the 2 years following terrorist events, and the rates of use were likely higher than estimated (DiMaggio et al., 2009). One month following the September 11, 2001, attacks on the World Trade Center, increased use of marijuana, alcohol, and cigarettes was reported in Manhattan (Vlahov et al., 2004). A random phone study of 1,507 New York City residents 6 months after the September 11 attacks showed that the incidence of alcohol problems for those who did not have drinking problems before the attacks was 2% (Vlahov et al., 2006). In addition, a workplace cohort study that took place before and after the September 11 attacks showed that women used more alcohol after the attacks compared with before the event (Richman et al., 2004).

Understanding how crisis situations affect those with preexisting conditions has important implications for addictions counselors. Examining the effects of substance use following the April 19, 1995, Oklahoma City bombing, a majority of survivors did not develop new SUDs; however, a majority of those survivors who did use substances excessively had a preexisting SUD (North, 2010). Compared with a control community, alcohol consumption among Oklahoma City bombing survivors in the year following the Oklahoma City bombing was 2.5 times greater than for the control community (Smith et al., 1999). For evacuees of Hurricane Katrina in August 2005, treatment for preexisting psychiatric illnesses, not increased substance use issues, was the main concern (North, 2010). Counselors should assess the types and quantity of substances used as well as the source(s) or provider(s) of the substances.

NDCHealth Corporation compiled data on the use of antianxiety medication following the September 11 attacks in the New York City and Washington, D.C., metropolitan areas, revealing that in the 2 weeks post–September 11, prescriptions for Xanax, Ativan, and Valium increased significantly (Okie, 2001). Regarding adolescent patterns of substance use, changes in usage status for adolescents exposed to

Hurricane Katrina were noted: 15% shifted from cigarette nonuse pre-Katrina to use post-Katrina, 9% shifted from nonuse to use of marijuana, and 25% shifted from nonuse to use of alcohol (Rowe et al., 2010). Adolescents not near the events of September 11 also experienced increased marijuana use (Costello et al., 2004).

Substance use can not only result from crisis situations, it can also affect crisis situations. Schneider (2009) found a strong correlation between SUDs and suicide. In a review of psychological autopsy studies, Schneider reported that substance-related disorders contributed to a high proportion of suicides. Individuals who use substances are nearly six times more likely to attempt suicide than those who report no substance use problems (Dragisic et al., 2015). Furthermore, Dragisic et al. (2015) found that those with SUDs who attempted suicide also had family members who had psychiatric disorders.

Darke (2009) estimated that 5% of the deaths of those who are substance dependent are due to homicide. He noted that homicide is a major risk factor for substance-dependent populations since substance use plays a key role in perpetuating this form of violence, with alcohol contributing the most to substance-related homicides. Substance use also contributes to other forms of violence, including intimate partner violence (IPV) and sexual assault, as many perpetrators of IPV against women resulting in homicide or attempted homicide were using alcohol, drugs, or a combination of alcohol and drugs. Juodis et al. (2014b) reported that 76% and 65% of perpetrators of domestic homicide had alcohol use and drug use problems, respectively. Regarding victim substance use, varying data exist as to the relationship between victim substance use and IPV. The National Institute of Justice (2009) highlighted findings from several studies that also indicate an association between victim substance use and domestic violence victimization.

The relationship between substances and sexual violence may in fact be "reciprocal in that sexual violence may be a precursor to or consequence of substance use, abuse, or addiction" (Dawgert, 2009, p. 21). A history of sexual victimization may in fact lead to substance use to cope with the pain and trauma, while substance use may increase the risk for victimization. To be clear, the victim of sexual assault is never responsible for their assault, regardless of whether they were under the influence of alcohol or drugs. However, alcohol is a behavioral disinhibitor and makes individuals less aware or in control of their reactions in social or interpersonal contexts. Gidycz et al. (2007) found that for women who reported a history of sexual victimization, alcohol use contributed significantly to revictimization during the 2-month follow-up period. Furthermore, as alcohol use increased, so too did the risk for sexual revictimization during this period; women with the highest risk had a history of rape or attempted rape and were also heavy drinkers. Gidycz et al. also found that decreasing alcohol use can reduce the risk for sexual assault. According to the National Institute of Justice (2008), sexual assault on campuses is more frequently associated with alcohol than drugs, with less than 1% of women reporting being incapacitated by date-rape drugs. In addition, the National Institute of Justice reported that while more women experience forced sexual assault before college, once in college the risk of becoming a victim of incapacitated sexual assault increases. Based on the National Crime Victimization Survey, the Bureau of Justice Statistics revealed that 47% of female students believed the offender was under the influence of alcohol or drugs when the sexual assault occurred, compared with 25% who did not believe substance use by the offender factored into the sexual assault (Sinozich & Langton, 2014). Complete Think About It 7.1.

THINK ABOUT IT 7.1

Think about a person with a SUD or significant symptoms. What are some of the problems that person may be facing? What images or biases come to mind when you think of a person who is addicted to a substance?

THE ETIOLOGY AND RISK FACTORS OF SUBSTANCE USE AND DEPENDENCE

Learning Objective 7.2 Describe the etiology and risk factors of substance use and dependence.

The most widely accepted belief is that addiction is biologically (neurobiologically, specifically) based; however, alternative hypotheses continue to be debated. One reason for the diversity of theoretical perspectives likely stems from the fact that while research continues to make inroads linking addiction to neurobiological mechanisms, no conclusive evidence of a specific biological relationship exists (Capuzzi & Stauffer, 2020; Hammersly, 2014), even though evidence of strong statistical associations exists between biological factors and alcoholism (Capuzzi & Stauffer, 2020).

Factors That Increase Risk for Substance Use Disorders

Numerous risk factors have been identified as contributing to initial substance use. Since initial use often occurs during adolescence, most of the commonly identified risk factors reflect events and issues relevant to that developmental stage. Some of the more common factors include poor academic performance, alcohol and other drug (AOD) availability, peer pressure, living in a dysfunctional family environment, and undiagnosed problems, such as learning disabilities. Even though initial use typically occurs in adolescence, AOD use can occur at any age and can be triggered by various issues such as trauma, disasters, or other emergencies.

Differentiating between risk factors for initial use and SUD is an important consideration given that a majority of those who report experimenting or engaging in recreational substance use do not develop problematic usage behaviors. If that were the case, AOD data would yield a much bleaker picture. For individuals who develop SUDs and symptoms, counselors need to be aware of other potential risk factors such as self-esteem and self-worth issues, isolation, intimacy issues and fear of rejection, poor coping mechanisms, and impulsivity and compulsivity patterns.

Counselors must also be knowledgeable of the co-occurring patterns of psychopathology and SUDs and symptoms. Key psychiatric disorders that co-occur with SUDs include major depressive disorders, anxiety disorders, schizophrenia, bipolar disorder, and certain personality disorders (e.g., borderline personality disorder, antisocial personality disorders). Other risk factors that can lead to abuse and dependency problems include low socioeconomic status, cultural and subcultural factors/expectations, criminality, history of abuse, and a family history of substance abuse and psychopathology.

SCREENING, ASSESSMENT, AND DIAGNOSIS OF SUBSTANCE USE DISORDERS

Learning Objective 7.3 Articulate procedures for the screening, assessment, and diagnosis of substance use disorders and symptoms, including screening procedures, commonly used assessment instruments, assessment, diagnosis, assessing a client's readiness to change, and treatment admission and placement processes.

A critical step in the treatment process is properly evaluating the degree to which substance use is problematic and the extent to which it has influenced the individual, especially those clients who are dealing with crisis or trauma situations. Careful screening, assessment, and diagnosis not only provide an accurate clinical picture but also help the counselor conceptualize applicable theoretical models, formulate preliminary treatment goals, and implement techniques and approaches that will align with the client's presenting issue(s) and motivation for change. In crisis settings where there is a time limit on the number of sessions, this comprehensive information is essential in order to determine the best treatment options for clients.

Success at this stage of the treatment process is greatly influenced by the therapeutic relationship that has been established. A strong therapeutic working alliance between counselor and client can help create a trusting and safe environment, enabling the client to feel more at ease when discussing substance history, especially sensitive, shameful, or embarrassing information. Creating a collaborative counselor–client relationship can reduce client resistance and ambivalence while increasing motivation to change. Counselors who instill a sense of optimism or hope and focus on the client's strengths and successes are more inclined to find clients open to exploring options. Seeing clients as more than their addiction is critical to this process.

Screening

The first step in the evaluation process is to rule out AOD problems. Screening clarifies whether there is sufficient evidence of problematic substance use and, if yes, whether this use warrants further investigation (assessment). Screening is brief in nature and often entails simple screening questions or brief measures related to substance usage. Questions are designed to determine patterns of use and often focus on frequency, quantity, and problem areas. Examples of frequency and quantity questions that might be asked during the screening process include "During the past 30 days, how many days per week did you drink alcohol or use drugs?" or "On a day when you did drink alcohol or use drugs, how many drinks did you have (or what quantity of drugs did you use)?"

Regarding the quantity question, the counselor must also determine the client's interpretation of a drink. The following example underscores the importance of clarification of the quantity question:

COUNSELOR: Daryl, during the past 30 days, how many days per week did you drink alcohol or use drugs?

DARYL: I've never used any drugs in my life but I do enjoy drinking alcohol. During the past 30 days, I would say I drank an average of 2 or 3 days each week, and that was mainly on weekends.

COUNSELOR: On those days when you do drink, how many drinks do you consume?

DARYL: I only have a couple beers.

COUNSELOR: By a couple do you mean two?

DARYL: Actually, three or four.

Note that if the counselor concluded the questioning at this point, the only information garnered is the number of drinks per episode and how many episodes occurred in the past 30 days. The counselor may surmise that there is no need for further evaluation. But what if the counselor probed further?

COUNSELOR: So on those days when you drink, it is often three or four beers.

DARYL: Yes.

COUNSELOR: Tell me about the number of ounces in a typical beer. (*The counselor notices a defensive shift in client's posture and his nonverbal facial expression is one of frustration. Client responds in an angry tone.*)

DARYL: Why do you need to know this? I like a few beers, and that's no one's business! (*A period of silence follows*) Okay, so I drink three or four 40s. It's not a big deal.

Making assumptions about the size of the drinks may lead to inaccurate information and miss a potential problem. In this example, the assumption may be that the client consumes three or four 12-ounce beers per occurrence, when in reality Daryl is consuming over three times that amount. Using the consumption question may be more telling than using more extensive questioning (Fleming, 2003). From a sample of men and women, Williams and Vinson (2001) found that asking the question "When was the last time you had more than four drinks (women) or five drinks (men)?" led to detection of problem drinking or criteria for abuse or dependence in nearly 9 out of 10 people sampled. Self-administered screening instruments can also provide valuable information about substance use patterns. Given the information presented in the example above, continued evaluation appears warranted.

Commonly Used Assessment Instruments

Counselors should carefully weigh the advantages and disadvantages of using assessment instruments based on the information gathered during the screening phase. If an assessment instrument is deemed necessary to gain additional information from the client on substance use, the counselor must decide which type will be most efficient in maximizing clinical information while minimizing client time and frustration.

The following considerations apply when using an assessment instrument:

- The client must sign an informed consent form (e.g., a form that addresses the purpose of the assessment, risks, benefits, how the information will be used, the client's right to refuse consent).

- The counselor should see the client as more than the results (the assessment is only one of many pieces of the clinical picture).
- The counselor should ensure that the assessment instrument is applicable and appropriate for the population it will be used to measure.
- The counselor should have training and competency in administering the assessment.
- The counselor must be able to interpret the results (Stauffer et al., 2020). Four commonly used assessment instruments are the Alcohol Use Disorders Identification Test (AUDIT; Babor et al., 2001), Michigan Alcoholism Screening Test (MAST; Selzer, 1971), Substance Abuse Subtle Screening Inventory–4 (SASSI-4; Lazowski et al., 2016), and the CAGE (Ewing, 1984).

The AUDIT was developed as a collaborative project by the World Health Organization from a six-nation study to identify early detection of problematic drinking patterns and behaviors (Saunders et al., 1993). The AUDIT comprises 10 items divided into three domains: questions 1–3 address alcohol consumption patterns, questions 4–6 focus on dependency issues, and questions 7–10 address problems associated with alcohol use. The AUDIT questions use a five-point Likert-type scale (0–4) and total scores can range from 0 to 40. A total score greater than 8 points indicates a pattern of harmful or hazardous drinking. Of those diagnosed with harmful or hazardous drinking, 92% had scores greater than 8 points. Of those who were not diagnosed with problematic drinking, 94% had scores less than 8 points. Based on total scores, counselors may incorporate certain interventions. The use of education and reduction approaches is suggested for those who score from 8 to 15. Individuals who score from 16 to 19 likely would benefit from brief interventions or counseling and follow-up. Individuals who score over 20 are at high risk and should be assessed for alcohol dependence and more intensive treatment (Babor et al., 2001).

The CAGE (Ewing, 1984) is a simple four-question screening tool that can be incorporated into a clinical interview. It is used only with adult and adolescent clients, and it is intended to identify behaviors related to alcohol use only. Each letter in the acronym *CAGE* represents one of the four questions: Have you ever felt the need to ***C***ut down on your drinking? Have people ***A***nnoyed you by criticizing your drinking? Have you ever felt bad or ***G***uilty about your drinking? Have you ever had a drink first thing in the morning to steady your nerves or to get rid of a hangover (***E***ye opener)? Each *yes* response to these questions receives a score of 1 point. Scores 0–1 do not reflect any apparent problems, and scores greater than or equal to 2 are a possible indication of alcohol dependence. One concern with the CAGE is the wide variability in accurately diagnosing alcohol use disorders.

Assessment

An assessment is conducted for the following purposes: (1) to ascertain the degree and severity of a problem; (2) to determine the presence of co-occurring disorders; (3) to make accurate diagnoses; (4) to assess the scope of the problem (i.e., whether it is specific to the individual or systemic, such as within the family or support system); (5) to guide the treatment planning process; and (6) to provide a mechanism for communication across treatment personnel (Erford, 2021).

The primary focus of substance use assessment is to gather detailed information on substance use patterns, including drug(s) used; progression of use (amount and frequency of use over time); route of administration; reason for periods of increased usage or abstinence; treatment history; and evidence of tolerance, dependence, and withdrawal. During the clinical interview, obtaining clients' history of substances used helps to establish a pattern and context of substance usage. The assessment should compile information and usage patterns on commonly used drugs such as alcohol, marijuana, opiates, hallucinogens, cocaine, amphetamines and related stimulants, inhalants, nicotine, and caffeine. With the growth of synthetic or designer illicit drugs, the assessment should include questions about use of drug variations such as flakka (a synthetic version of the stimulant cathinone), K2 (synthetic cannabinoids), bath salts (synthetic cathinones), wax (marijuana and butane), purple drank (cough syrup with codeine and promethazine mixed in soft drinks or made into candy), and krokodil (desomorphine, a derivative of codeine that is mixed with oil, gasoline, alcohol, or paint thinner and injected), to name a few (see Table 7.1 at the end of this chapter for resources on commonly used designer drugs).

Inquiring about use of any psychotherapeutic medications, prescriptions, and over-the-counter drugs, including herbal supplements, rounds out the comprehensive picture of usage. Including questions about these types of drugs is beneficial since most clients do not consider them to be problematic, especially if they have been prescribed. Gathering information on these substances provides valuable information on potential synergistic effects resulting from drug interactions. Furthermore, having knowledge of different drug interactions can help determine any potential contraindications that may exist. For example, clients with bipolar disorder who use lithium are advised to avoid certain drugs that are contraindicated, including alcohol, nonsteroidal anti-inflammatory medications such as ibuprofen, and illicit drugs such as Ecstasy that can result in changes in lithium levels.

Assessment also provides a comprehensive picture of the client across different psychosocial domains (e.g., medical, legal, education, employment history, family history, psychiatric history). Often information gathered on other psychosocial issues is directly connected to substance use. For example, clients with substance use issues may have an unstable employment history or a pattern of legal problems. It is not uncommon for those with a history of substance-related problems to also have conflicts in their relationships with family and friends. Medical concerns may be directly related to substance use, yet clients may not make the connection or resist addressing the problem.

When conducting a substance use assessment, counselors should also evaluate the client's current level of mental functioning, which is often done using the Mini-Mental State Examination (MMSE-2; Folstein et al., 2016) in crisis situations. The purpose of the MMSE-2 is to investigate signs and symptoms of impaired affect, behavior, cognition, and intellectual functioning related to one's mental state. The MMSE-2 can also be valuable in differentiating between substance and nonsubstance impaired mental symptoms and for ruling out co-occurring disorders (Rudd, 2006). Given that most clients who have co-occurring substance use and mental health issues are likely attending counseling due to acute symptom presentation or concerns associated with these disorders, counselors should evaluate whether a crisis assessment is warranted. For example, clients who have a previous history of psychiatric diagnoses (e.g., bipolar disorder, major depressive

disorder, schizophrenia) and substance use are at greater risk for suicidal behavior. Depending on the score related to any cognitive impairment not due to the substance use, a referral for a full mental status exam may be included in the treatment plan.

One of the more commonly used methods for conducting substance use assessment is the clinical interview, a process that includes extensive and detailed questions regarding various content domains. When counselors are conducting the clinical assessment, it is inevitable that clients indicate how they define their substance use. Some clients may be inclined to overestimate or underestimate use or information. Clients may indicate that they fit into certain categories (e.g., social drinkers, recreational users) despite information to the contrary. Therefore, counselors may need to ask additional questions or clarify client responses.

Semistructured clinical interviews are often preferred over structured ones because they allow the counselor to ask specific, structured questions in various domains and provide the counselor with the flexibility to expand on or deviate from questions based on client response. Domains that are commonly covered in a clinical interview are as follows:

1. Demographic information (e.g., name, age, race, height, weight)
2. Presenting complaint or reason for attending
3. Medical history (e.g., diagnoses, medications, hospitalizations, surgeries)
4. Substance use history
5. Legal history (e.g., specific charges, incarcerations, any charges that are pending, parole/probation status, any protection from abuse orders)
6. Employment history (e.g., job history, reason for leaving jobs, conflicts with coworkers or supervisors, work performance and evaluations)
7. Education history (e.g., degrees achieved, problems in school and reasons for them, difficulty with reading or writing)
8. Social and family history (e.g., childhood issues, trauma, or abuse; family discord/dysfunction; marital status; current living arrangements; history of relationship problems; family history of mental illness or SUD; history of child abuse or neglect for those with children)
9. Recreational history (e.g., activities of interest, activities impacted by substance use, hobbies, memberships in clubs or groups)
10. Religious/spiritual/cultural history (e.g., current and past religious/spiritual beliefs, practices, attendance; spiritual injuries that cause people to question their faith; cultural/heritage values, beliefs, considerations)
11. Psychiatric/psychological history (e.g., any psychiatric or personality diagnoses, hospitalizations, inpatient/outpatient treatment, suicide attempts and history)
12. Sexual history (e.g., sexual identity, sexual disorders, sexually transmitted infections)
13. Mental status exam

Scenarios 1 and 2 provide some guidance on clarifying client presenting information.

Scenario 1: Challenging a Client's Definition of Social Drinking

COUNSELOR: Finch, you mentioned earlier in our session that you drink daily and often drive when you know you shouldn't. Help me understand how you see your drinking.

FINCH: Well, I don't see it as a problem. I never had a DUI [driving under the influence] and had only minor issues with my partner when I've gotten home late or with my boss when I've missed time at work because I'm hung over. I guess I see myself as a social drinker because I only drink when I go out after work with my friends.

COUNSELOR: So you classify your drinking as social because of the location, and you have not had any significant consequences.

FINCH: That's how I see it.

COUNSELOR: Tell me how you might view this definition if you got arrested for a DUI, the relationship with your partner got much worse, or you got fired from your job because of your drinking. (*This probe helps the client examine personal usage, knowing that any one of these scenarios is a real possibility even as a "social drinker."*)

Scenario 2: Differentiating Between Recreational Use and Substance Use

HAYES: I only smoke pot recreationally as a way to relax and have some fun.

COUNSELOR: How would you classify your pot use?

HAYES: I share a joint or two with some friends once a month. It's not a big deal.

COUNSELOR: You mentioned that you only use marijuana occasionally, yet when you were discussing your medical and legal history, you alluded to using it more regularly.

HAYES: Just 'cause I have breathing problems and had a few run-ins with the law doesn't mean I have a problem!

COUNSELOR: I sense you're upset right now because we're talking about whether your use is really recreational. My intent is not to judge or be critical of your choices around marijuana. Rather, I'm concerned that your use may be causing problems in other areas of your life.

HAYES: Sometimes I use it more as a way to have fun.

Diagnosis

A well-conducted, thorough assessment should yield sufficient data and documentation to make accurate decisions and establish the degree of impairment. With most other mental health diagnoses, providing a full diagnosis during a crisis phase would be premature and inappropriate due to the symptoms and period of time needed to accurately offer a valid diagnosis. However, documenting prior diagnoses, onset, relapse, current client reports, and presentations are essential to provide for the next phase of the treatment so the next clinician can formulate a diagnosis. Pure SUDs, not including induced disorder

categories (e.g., substance-induced mood disorders, substance-induced bipolar disorder) rely on self-reports and can aid the treatment planning and referral process.

The purpose of diagnosis is to identify the specific condition(s) or disorder(s) that are causing the impairment in order to provide a basis with which treatment interventions can be applied or to make an appropriate referral for long-term substance treatment. The main diagnostic tool used for SUDs is the *Diagnostic and Statistical Manual of Mental Disorders*, the most current being the fifth edition, text revision (DSM-5-TR; American Psychiatric Association, 2022). In DSM-5, substance use and addictive disorders are organized into a number of categories. The two categories of primary interest are alcohol use disorder and SUD, the latter covering a variety of addictive substances, such as amphetamines, cocaine, and inhalants.

Prior to the latest edition, DSM classified substance use disorders as either substance abuse or substance dependence. As a result of the ambiguity and confusion surrounding dependence and addiction, DSM-5-TR does not use the terms *substance abuse* and *substance dependence*. Instead, it uses the umbrella term *substance use disorders* in relation to recurrent alcohol and drug use (American Psychiatric Association, 2022). *Dependence* and *addiction* were often used interchangeably, and tolerance and withdrawal were criteria central to a dependence diagnosis. However, tolerance and withdrawal are common and expected responses to prescription medications that act on the central nervous system and as such do not imply one is addicted, especially if these are the only symptoms evident. The reclassification to SUDs and the use of severity specifiers (mild, moderate, severe) provide counselors with a reference point for the progression of the substance use pattern, as well as a perspective on how best to approach treatment and which interventions may be most applicable.

According to DSM-5-TR, at least two of the following criteria must be met for an individual to be diagnosed with a SUD: (1) significant problems exist at home, school, or work due to use; (2) physical hazards occur due to repeated episodes of impaired driving or operating machinery while under the influence; (3) substance use results in recurring legal problems (e.g., public intoxication); (4) tolerance increases; (5) withdrawal occurs; (6) increasing amounts of the substance are taken over time; (7) usage levels are not being reduced; (8) a lot of time is spent acquiring the substance; (9) social or work activities are sacrificed due to use or recovery from use; (10) use continues despite interpersonal and physical effects; and (11) there is craving, a strong urge, or desire for the substance. Severity is determined by the number of presenting criteria (i.e., mild: two to three criteria; moderate: four to five criteria; severe: six or more criteria). Complete Activity 7.1, which focuses on SUD diagnosis.

ACTIVITY 7.1

When assessing a client for a SUD, which issues and factors do you believe should be considered to make an accurate diagnosis?

Assessing a Client's Readiness to Change

A client's readiness to change is an essential function to consider during the assessment phase. Evaluating resistance to change allows the counselor to match treatment techniques and motivational approaches to the level of client ambivalence. Several change models have

appeared since the 1970s (Connors et al., 2015), but Norcross et al.'s (2011) five-stage change model focuses mainly on change patterns associated with health behavior change and reflects identifiable stages individuals go through as they are attempting to change substance use behaviors. Norcross et al. identified the following five stages of change:

1. ***Precontemplation***—characterized by resistance to change or denying that a problem exists.
2. ***Contemplation***—evidence of ambivalence exists and the client is contemplating the effects of substance use as well as weighing the advantages and disadvantages of continued use.
3. ***Preparation***—resistance has waned and the client has made a decision to change but is uncertain how to go about this process.
4. ***Action***—the client is committed to making a change and has demonstrated specific goals and behaviors toward this end.
5. ***Maintenance***—the client has maintained a period of abstinence and is working on maintaining recovery and addressing relapse issues and triggers.

Transitioning across stages is rarely a smooth, straightforward process. Clients may move from one stage to the next or move across several stages only to experience a setback. Moving back to an earlier stage is not unexpected given that change is difficult, and clients will likely experience much frustration and discouragement during the process. Motivational support becomes an integral component to assist in the change process.

Understanding the importance of instilling motivation as an antidote for ambivalence and resistance, Miller and Rollnick (2013) integrated their motivational interviewing (MI) approach into the stages-of-change model based on the belief that motivation is not intrinsically driven as part of one's personality or personal traits but rather is developed through interpersonal interactions. They identified MI as a client-centered, directive, transtheoretical approach emphasizing empathic communication to bring about a client's intrinsic motivation to change that aligns with values and beliefs. MI was developed as an alternative to the directive and confrontational approaches that were widely used and accepted in treating addiction for decades.

Miller and Rollnick (2013) developed four central processes that serve as the basis for MI: (1) *engaging*—establishing a connection between counselor and client that helps to build the working alliance; (2) *focusing*—involves the counselor developing and maintaining a focused direction in the discussion about change; (3) *evoking*—assisting the client to take ownership for their motivation for change; and (4) *planning*—establishing a commitment to change with a specific course of action where talk shifts from whether and why it happens to when and how to change. Read Voices from the Field 7.1.

VOICES FROM THE FIELD 7.1

Working with Substance Use

Rachel M. Hoffman

"Addiction is an issue with which I never want to work! I don't want to work with clients who don't want to change!" That quote, or something very close to it, was uttered by me at some point (or maybe more) during my graduate counselor training experience. Before being exposed to the field of addiction counseling, I mistakenly believed that addiction was a disorder

(*continued*)

from which people really had to "hit rock bottom" before they could recover. Now, many years and a lot of education later, I have a completely different view of addictions, and I honestly could not imagine working with any other population.

Addiction is a complex phenomenon. Many researchers now believe that addiction is a brain disease that results in neurological adaptations with prolonged substance use. After spending the past several years working in an agency that specialized in addiction, I have learned that it is important to use interventions appropriate for a client's stage of change. Assessing a client's stage of change (i.e., precontemplation, contemplation, preparation, action, and maintenance) allows the counselor to develop interventions based on the client's readiness to change a particular behavior or situation.

In my work with clients, I have appreciated the ability to use motivational interviewing techniques to help the client move through the stages of change. I respect my clients' abilities to make changes that align with their stated goals and desired outcomes. Adopting this strengths-based approach to working with clients has helped me avoid frustration, burnout, and anger in my work. Although I did not initially intend to work with this population, I find my daily work with addictions immensely satisfying and rewarding, and I am thankful that my career path has led me in this direction.

Treatment Admission and Placement

Information gathered during the assessment and diagnosis phases assists the counselor in determining not only treatment planning but also the appropriate level of care for clients in crisis. Clients who present with significant substance use issues will likely require an intensive treatment protocol compared with clients who are misusing a substance. The American Society of Addiction Medicine (ASAM; 2013) provides comprehensive treatment criteria for patient placement, continued stay, and discharge. The latest ASAM criteria put greater emphasis on co-occurring mental health disorders and SUDs. ASAM continues to divide the continuum of treatment into increasing levels of care. The five broad levels of care, now designated by numbers instead of Roman numerals, are as follows:

- ***Level 0.5*** (Early Intervention) involves screening, brief intervention, referral, and treatment; early intervention services were expanded as a way to reach out to the nearly 20 million individuals who are pursuing addiction treatment
- ***Level 1*** (Outpatient Services) is considered a gateway for substance use treatment as well as a level of care that can focus on ongoing chronic addiction and abstinence
- ***Level 2*** (Intensive Outpatient/Partial Hospitalization Services)
- ***Level 3*** (Residential/Inpatient services)
- ***Level 4*** (Medically Managed Intensive Inpatient Services)

Each level also has a withdrawal management designation that aligns with what the counselor believes is needed to manage the physiological and psychological symptoms of withdrawal.

ASAM has developed six dimensions that practitioners can use to assess clients at each level and provide a rationale/justification for a particular level of care. The dimensions also serve as standardized assessment language for general health care and mental health practitioners. The six dimensions are as follows:

1. Acute intoxication and/or withdrawal potential—assessing withdrawal management across different levels of care and making preparations for continued addiction services

2. Biomedical conditions and complications
3. Emotional, behavioral, or cognitive conditions and complications
4. Readiness to change
5. Relapse, continued use, or continued problem potential—greater focus on addressing early stages of change
6. Recovery/living environment

The ASAM assessment dimensions should be incorporated into the continuous evaluation of the client's progress. Based on the outcome of an assessment, the counselor should identify the most appropriate care and adjust the treatment plan accordingly (Mee-Lee, 2013). When assessing all six dimensions, counselors want to select the treatment level that is least intensive but the safest level of care based on the client's presenting issue and assessed dimensions. Case Study 7.1 highlights the ASAM assessment process.

CASE STUDY 7.1

A Client Assessment Using the Six ASAM Dimensions

Jules is a 42-year-old brought to the city hospital following a DUI accident that resulted in breaking both a collarbone and arm. Jules's blood alcohol level (BAL) was 0.42 at the time of arrest, and this was Jules's second DUI in the past 12 months. Jules's BAL for the first DUI was 0.12, and Jules was approved for Accelerated Rehabilitative Disposition, a pretrial intervention program for nonviolent offenders with no prior record. During the trial for the second DUI, the judge requested a substance use evaluation and is considering remanding Jules to treatment in lieu of incarceration. The counselor completed an assessment of Jules using the six ASAM dimensions and made a treatment recommendation.

Dimension 1: Acute Intoxication and/or Withdrawal Potential Jules demonstrated increased tolerance. Based on reported usage, Jules is at elevated risk for physiological withdrawal. Concerns exist given the pattern of continued use, high BAL, and inability to stop drinking. Jules meets criteria for Level 3.2-WM (Clinically Managed Residential Withdrawal Management).

Dimension 2: Biomedical Conditions and Complications With the exception of current medical complications, Jules did not present any specific health problems or complications due to alcohol. Jules is not on any medications. Jules meets criteria for Level 1 (Outpatient Services).

Dimension 3: Emotional, Behavioral, or Cognitive Conditions and Complications Jules has problems managing anger and often fights with partners and coworkers, especially when drinking. Jules lacks coping skills and can be quite reactive when confronted about alcohol use. Interpersonal skills, specifically communication skills, are poor. Jules does not take responsibility for personal actions and tends to blame others for problems. Jules lacks self-efficacy, motivation, and cognitive awareness to make the necessary changes at this point. Jules meets criteria for Level 3.3 (Clinically Managed Population-Specific High-Intensity Residential Services [adult level only]).

Dimension 4: Readiness to Change Jules is at the precontemplative stage of change. Jules does not see personal use as a problem and believes no assistance is needed with the drinking. Jules feels misunderstood and is in denial about usage and amount. Jules is defensive and angry when discussing alcohol use and meets criteria for Level 3.3.

Dimension 5: Relapse, Continued Use, or Continued Problem Potential Jules's potential for relapse is very high without intensive treatment. Jules lacks skills and resources to maintain sobriety and denies having any friends who do not drink or who provide positive support systems. Without intensive treatment and an established support network, the prognosis for maintaining sobriety is very poor. Jules meets criteria for Level 3.3.

Dimension 6: Recovery/Living Environment Jules has alienated partners and family and does not care. Jules has refused to attend 12-Step meetings and thinks they are a waste of time. The current support system cannot assist Jules in helping to maintain recovery. All social activities revolve around drinking. Jules meets criteria for Level 3.3.

Recommendation to the Judge Jules meets criteria for Level 3 for five of the six dimensions. Treatment recommendations are as follows: complete inpatient/residential treatment detoxification protocol followed by Level 3.3 residential treatment (adult level only). Length of stay will depend on progress on all six dimensions and should be reevaluated after 4 weeks. Step down to Level 2.1 intensive outpatient treatment following successful discharge from Level 3 care.

Discussion Questions

1. What treatment approaches might you consider using with Jules?
2. As the counselor, what issues would come up for you as you work with Jules?

SUBSTANCE USE TREATMENT AND CRISIS

Learning Objective 7.4 Explain substance use crisis treatment, including relapse and crisis, harm reduction, support groups, couples and family treatment, and co-occurring disorders.

Treating SUDs and symptoms becomes more complex when the client is also experiencing a crisis. Clients can be in crisis at any point along the substance use continuum and may require different interventions depending on their level of motivation, stage of change, the significance of the crisis event, intensity of the crisis response, coping capacity, resilience, and existence of co-occurring disorders. Before continuing, complete Think About It 7.2 and read Voices from the Field 7.2.

THINK ABOUT IT 7.2

Think about your personal characteristics and skill levels. What counselor characteristics and counseling skills/techniques do you believe are important when working with clients who use substances as a means to manage a crisis response?

VOICES FROM THE FIELD 7.2

Engaging the Client Who Uses Substances

Steve Zappalla

One of the most common traits of substance use clients is their denial system and strong habit to protect their substance of choice, often at any cost. For the counselor, it is important to relate to and understand their desire to use a substance. Using language, descriptions, and techniques they understand is helpful to gain their trust, respect, and confidence quickly. Meeting clients where they are is extremely important.

Listen carefully to the client's resistance and use calm, reflective statements. Determine the main theme that is bothering the client and avoid attacking the drinking or using problem. It helps to hear what they are saying. Try to see things from the client's point of view, no matter how unreasonable or how irrational it seems. Your goal is to create a space to help clients see that what they are doing is not working and try to get them to want to do something about it, not to win a confrontation. It could prove more beneficial to acknowledge different viewpoints.

Try to be encouraging and understanding. Clients often become resistant, even hostile, when they feel frustrated or confused. Clients with SUDs and symptoms tend not to have a very mature emotional coping system and will react quickly with what they know best. The most important thing to keep in mind is that there is a good chance the client does not know any other way. While small steps may seem a success early on, it is critically important to help clients feel and see the long-term benefit of this new behavior. It is most important to help clients see that what they are doing is not working and begin to help them take steps to do something about it.

Counselors should be aware of a multitude of treatment options when working with clients identified with SUDs and symptoms. Over the years, numerous treatment modalities have been implemented to address SUDs. Sorting out which treatments are effective has been no simple task. Decisions for treatment approaches have often been based on personal interest or counselor familiarity rather than on empirically supported evidence. Efforts to understand which treatments are effective have increased significantly over the past several decades. Traditional approaches such as confrontational approaches, aversion therapies, insight-based psychotherapy, and educational approaches have generally been ineffective (Miller et al., 2003). A comprehensive review of various treatment modalities revealed that approaches such as brief interventions, motivational enhancement, gamma-aminobutyric acid agonists and opiate antagonists, various behavioral treatments, community reinforcement, and social skills training were ranked the highest in terms of treatment efficacy.

Given that brief interventions and motivational enhancement approaches were two of the highest-ranked, most effective approaches to address SUDs and symptoms, integrating a crisis model that aligns with these approaches allows the counselor to effectively address both the crisis response and SUD (see the task model of crisis assessment and intervention presented in Table 1.1). Case Studies 7.2 and 7.3 provide practice in applying the model of crisis assessment and intervention.

CASE STUDY 7.2

A Single Crisis Session

Lenora is a 23-year-old woman who was transported to the hospital during the early-morning hours by her sister, Mary. Lenora presented as visibly upset, very angry, shaking, disheveled, and crying uncontrollably. Mary told the attending nurse that

Lenora had been out drinking earlier that night and met a man. Mary explained that he was buying Lenora drinks and at one point must have slipped her a "roofie." Lenora did not recall what took place but when the drug wore off, she found herself alone and unclothed in a hotel room several blocks from the bar. Lenora reported that she had been raped. Mary also shared that Lenora "has been drinking a lot for the past 6 months" following the termination of a long-term relationship. You are an addictions counselor working for a local mental health agency that contracts with the hospital for evening and weekend crisis response calls.

Discussion Questions

1. Explore how you could integrate the task model of crisis assessment and intervention (see Table 1.1).
2. What would be the first thing you would do to address this crisis situation?

CASE STUDY 7.3

Multisession Crisis Intervention with a Substance Use Client

Francis is a 39-year-old married man who recently learned that his wife of 10 years is filing for divorce because of his marijuana and cocaine addiction. He also learned she is having an affair with his best friend. Francis has been distraught for several days upon hearing this news and is contemplating suicide. At his mother's insistence, Francis begrudgingly attends the initial appointment she made for him with a counselor who specializes in substance use and crisis issues at the local mental health agency. Francis is aware that his drug use was causing problems at home and has tried several times to stop, but he has been unable to maintain abstinence for more than a day. He reports that he was in counseling 2 years ago when his wife threatened to leave him but was not motivated to change and dropped out after a couple of sessions. He also did not feel the counselor listened to him and instead told him what to do. Francis acknowledges his drug use has caused problems and is upset over his wife's affair with his best friend. He is coming to the realization that it is too late to save his marriage. He states that he has no reason to live. Let's examine how the counselor may structure the first six sessions.

Session 1

The counselor must undertake several tasks simultaneously. Understanding what was helpful and not helpful from the client's previous counseling experience can provide valuable information as to how the client views counseling, approaches that should be avoided or enhanced, a sense of motivation, and ways to conceptualize how best to establish a working alliance. Establishing a working alliance is essential given the client's fragile emotional state. While the counselor is working to establish rapport and create an environment where the client can feel supported, effort must be directed toward the client's message of desperation and suicide ideation. The counselor must assess the severity of the client's statement around self-harm. During this process, the counselor must assess the level of intent and determine both the subjective (client statements) and objective (specific concrete behaviors) messages. Using direct language

(direct questions vs. reflective responses) is necessary to evaluate intent, means to carry out the intent, and a clearly developed plan. Conveying this information in an empathic manner creates a sense of care and concern that increases the likelihood the client will come back next session. Let's look at the counselor–client dialogue related to examining intent.

COUNSELOR: Francis, I appreciate your willingness to seek help and for being open with me. I want to ask you a few questions to help me better understand your emotional state.

FRANCIS: OK.

COUNSELOR: Tell me about what you mean by not wanting to live anymore.

FRANCIS: My world has fallen apart. My wife has been upset about my drug use for some time, and I think it drove her to have an affair. I don't see any reason for living. I really screwed up.

COUNSELOR: A couple times today you mentioned or alluded to the fact that you don't want to live. Before we talk about what's going on right now for you, let's discuss any previous suicidal thoughts or attempts. Have you made any suicide attempts previously?

FRANCIS: No, never, but I've thought about it from time to time, especially after my wife and I got into fights about my drug use.

COUNSELOR: At that time, did you have a plan to hurt yourself and, if you did, did you have the means to carry it out? (*Give examples of methods*)

FRANCIS: No, I didn't have a plan. It was just thoughts running around my head that I couldn't let go of. I got really scared of my thoughts because I didn't feel I had any reason to live. I felt that I was no good to her or anyone. I felt overwhelmed and the pain was unbearable.

COUNSELOR: You felt so distraught and worthless because nothing seemed to matter anymore. The only solution in your mind was to hurt yourself. Describe your intention to hurt yourself at that point.

FRANCIS: I thought a lot in those moments about killing myself, but I didn't because I would have hurt more knowing my children and my mom would have to live with this.

COUNSELOR: Francis, you were aware of the painful outcomes killing yourself would have on loved ones. Help me understand more about your state of mind during that time. Did you think of or have a plan in place and, if so, did you have a means to carry out that plan?

FRANCIS: No plan. I just wanted the pain to go away. I started to use more drugs to numb out.

COUNSELOR: Did you plan to use more drugs with the hope of killing yourself? (*Exploring drug use as a possible means to carry out suicide is important to assess as it can be easily overlooked with the focus on more acute means of lethality*.)

FRANCIS: No. I never planned or intended to overdose or use drugs to kill myself. Only to take away the pain.

COUNSELOR: So, you had no specific plan or means?

FRANCIS: That's right. I guess I chickened out.

COUNSELOR: Choosing to live when you were facing incredible pain takes a lot of courage and strength. (*The counselor continues to use empathy to build support, rapport, and trust*.)

FRANCIS: I guess, but I'm not feeling a lot of courage right now.

COUNSELOR: Describe your current thoughts about wanting to kill yourself. (*The counselor may use a 1 to 10 scale to help assess, with 1 being no intent, plan, or means to kill myself and 10 being a definite plan and means to kill myself*.)

FRANCIS: Kind of like what we talked about before. Feeling really overwhelmed. Maybe a 3 or 4.

COUNSELOR: So a 3 or 4 would indicate feeling overwhelmed like what you felt before, or is it more than before?

FRANCIS: Pretty much the same.

COUNSELOR: Thank you for helping me understand. Francis, given these recent painful experiences, do you have a plan or means to kill yourself right now?

FRANCIS: No. I just feel really awful. I just wish the pain would go away.

COUNSELOR: The pain feels really unbearable, so much so that you think that the only way to reconcile it is by not existing anymore. (*If the client mentioned a plan and intent, the counselor would explore in detail the plan and means. The counselor would ask about specific sources to carry out suicide and continue to examine multiple methods until the client states that no other methods have been considered*.)

FRANCIS: Yes. (*Francis cries for several minutes*.) And I don't know what to do next because nothing has worked out and my wife is leaving and screwing around with my best friend because of my drug use.

COUNSELOR: It sounds like you don't want to die, you just want your life to make sense. Right now, you're uncertain what direction to go to alleviate the pain. You know that drugs have not helped the situation, but they have been the only thing that has helped to numb the pain.

FRANCIS: Exactly! I know this is no way to live my life. I'm tired of feeling this way, but it's the only thing I know that works and I'm not sure I want to let go of using drugs at this point. (*The client's affect brightens a little and seems a little hopeful that he is being listened to, something he complained his wife never did*.)

COUNSELOR: It sounds like the relationship with drugs is causing a lot of pain and problems, but you're not ready to let it go. Maybe we can work together to examine this relationship and the costs and benefits of using versus not using.

FRANCIS: It's something I've been thinking about. I've been trying to figure out why I keep doing this when it causes so many other problems, but I need it and I'm not sure I can give it up right now because it hurts too much when I don't use drugs.

COUNSELOR: Let's not focus on all-or-nothing options related to use. Would you instead be open to examining your use and understanding what purpose it's serving for you?

FRANCIS: I'd be open to talking about that.

COUNSELOR: To help start that conversation, I have some materials that others have found helpful as they attempted to understand similar issues you've been facing. Would you be interested in looking these over between now and our next session?

FRANCIS: Yes, I'd be interested.

The counselor established that there was no chronic suicidality or previous attempts. Client responses indicate mild risk, and the counselor should evaluate and monitor risk on an ongoing basis. The client has difficulty when facing challenging situations, feels overwhelmed, often uses substances impulsively to manage emotional instability, and has a history of fleeting thoughts of suicide but no established intent, plan, or means. During the remainder of the session, the counselor will assess the client's drug use and motivation to change. The counselor is still concerned about the client's strained emotional state and will work to establish a plan in the event that the client's condition worsens before the next session. The counselor may consider using the commitment to treatment statement (CTS), an agreement to commit to the treatment process and to living, and a crisis response plan that outlines specific behaviors the client will engage in if the crisis escalates (see Rudd, 2006).

The counselor and client also work together to examine a realistic plan for substance use. It is evident the client is in the contemplative stage, and the counselor wants to continue to enhance client understanding and awareness. The counselor also commits time in session to discussing the marital issues and explores the dynamics that have triggered this recent crisis situation. The client is feeling better at the end of the session, and they agree to meet again in 4 days. At the end of this session, the counselor must thoroughly document the session and provide specific content related to the suicidal ideation, established patterns, and current condition. The use of direct client quotes strengthens the record.

Session 2

The counselor continues to build rapport and establish the therapeutic working alliance using basic reflective skills and advanced skills such as clarification and immediacy as needed while helping the client color in the details related to the presenting problem, including the implications of the affair. In this session, the counselor assesses the client's emotional state and explores behaviors and activity during the past several days and evaluates whether different interventions are necessary. Motivational enhancement approaches continue to serve as a building block for exploration of the client's substance use and assist in shifting the client's ambivalence toward change.

The exploration and implementation of treatment options will align with the client's stage of change. Assessing cultural factors and their role in understanding a client's motivation level is important during the second session. Also, the counselor discusses the bibliotherapy assignment and specific reactions, thoughts, and insights and how the client has integrated this information and any newly acquired knowledge into his situation. The counselor and client discuss additional options, including continued bibliotherapy options and the possibility of listing advantages and disadvantages of current substance use for the next session. At the end of the session, the counselor and client both summarize salient issues and the counselor reminds the client of the behavioral plan in the event the crisis reemerges.

Sessions 3–6

Evidence of a stronger therapeutic alliance is emerging with ease of rapport and signs that the client is trusting the counselor. The exploration of presenting problems continues. The counselor notices that the client has chosen options to help explore ambivalence, including a handwritten list of the advantages and disadvantages of drug use. The client reports that he knew his substance use was a problem but did not understand the magnitude until he made this list. He reports that he wants to make changes and wants to end this relationship with drugs. He also realized how much he alienated his wife and, while still very hurt and upset over her affair, understands how his drug use contributed to this process. He reports that he wants to change but does not know where to begin. He also reports that he has not used drugs for the past 12 hours and has been feeling a little rough but indicates he would like to be drug-free in the next month.

The client's attitude has shifted from the first session, and his behaviors indicate that he is preparing to move toward making a change. The counselor recognizes this shift and works with the client to develop a plan to change and specific treatment options that align with the preparation. During this stage, the client's motivation aligns with specific behavioral approaches and the opportunity for the counselor to use other advanced skills such as challenging, advanced empathy, and immediacy. During these sessions, working with the client to establish a support system is critical.

At this point, the counselor would discuss various types of supports and assess the client's willingness to explore each. It is important that the counselor be knowledgeable of different support options and help the client navigate this process. For example, integrating 12-Step support groups is quite common, yet many counselors may not take the time to process the client's apprehensions or fears. Explaining what 12-Step groups are about, the types of meetings available, expectations, and what a typical meeting entails can do much to allay client fears. The counselor may suggest to Francis that to

help ease into this process, it may be beneficial to see whether a friend would be willing to accompany him the first time. Support is an essential step in enhancing motivation and creating an alternative to relying on drugs as a means of coping.

Discussion Questions

1. What other issues warrant consideration in this case?
2. What other considerations would you integrate into the session 1 dialogue?
3. What counselor responses would you change or revise and why in the session 1 dialogue?

Relapse and Crisis

One of the more critical aspects of substance use treatment is addressing relapse. Relapse is sometimes inevitable, especially for clients who have a chronic, long-term history of substance dependence. Despite efforts to address relapse as part of treatment goals, it continues to be problematic for many clients. Various studies report 40–75% of clients with SUDs relapse in the 6 months following treatment (Hasin et al., 2013; Kabisa et al., 2021; Maehira et al., 2013; Moradinazar et al., 2020; Sapkota, 2019). Even those who maintain abstinence beyond a year struggle. Dennis et al. (2007) found that 36% of respondents with less than 1 year of abstinence (any substance) were able to maintain it at year 8. For those with 1–3 years of abstinence, rates improved to 66%, and they increased to 86% for those with 3 or more years of abstinence. Dennis et al. concluded that the first 3 years of abstinence yield the highest risk for relapse. Numerous factors may contribute to this wide variance in relapse rates, such as treatment not addressing relapse issues, clients not following through with treatment goals, and clients dropping out of treatment. Those with SUDs and significant symptoms may be at a higher risk for relapsing when facing crisis situations.

During treatment, counselors need to prepare for relapse by helping clients explore triggers and learn new cognitive-behavioral strategies. Even in short-term crisis centers, these triggers can be identified, explored, and documented when clients are referred for treatment. Examining the client's high-risk behaviors, self-monitoring activities, and coping skills (Moss & Cook, 2020) as well as supporting the client in progressive relaxation techniques can help counselors address relapse concerns (Lewis et al., 2019).

Counselors need to help clients differentiate when use is viewed as a "lapse" or a "relapse." A *lapse* is a single episode of using a substance following a period of abstinence. The individual often takes responsibility for this lapse and examines the specific experience or trigger(s) that resulted to build on recovery. A *relapse* is a return to sustained use of the same substance or another mood-altering substance following a defined period of abstinence. Counselors need to be extra vigilant to potential warning signs, including client rationalization or denial (or using other defense mechanisms); the client not showing up for treatment; the client not attending 12-Step meetings or working with a sponsor; the client demonstrating changes in attitudes, behaviors, or thoughts around substance use; and the client reconnecting with unhealthy people or environments. An established support system is an essential piece in addressing relapse during a period of crisis. Clients should be encouraged to seek support from 12-Step groups, be aware of the need to commit more time to meetings, and work closely with sponsors. Complete Think About It 7.3.

THINK ABOUT IT 7.3

Think about relapse. What are your thoughts, attitudes, and beliefs about relapse? What approach would you take if a client reported a lapse or relapse?

Harm Reduction

For those with SUDs and symptoms, the main goal of treatment is to establish and maintain abstinence, while incorporating strategies for harm reduction. Getting to a place of abstinence is often a complex and difficult process. Counselors using various approaches may also find using harm reduction approaches is a valuable tool within the treatment protocol. Harm reduction approaches are tools to help the client reduce the negative consequences associated with use, regardless of one's usage pattern. Clients in the early stages of change are not going to be open to hearing about severing their relationship with their drug. Harm reduction can be helpful to find a common ground and meet clients where they are. Harm reduction is often used during the early stages of change when clients are expressing resistance. Harm reduction is often misunderstood, and some believe it may encourage continued use. In reality, the ultimate goal of harm reduction is to achieve abstinence and prevent future crisis situations.

Marlatt et al. (2011) identified five underlying principles of harm reduction:

1. Strive for abstinence while considering alternative options to help minimize harm.
2. View prevention as a way to reduce the negative consequences or harmful outcomes associated with use but not focusing specifically on usage.
3. Focus approaches on where the client is at that point in their use or addiction and develop approaches accordingly.
4. View harm reduction as a community-oriented process where action is taken at the grassroots level to advocate for change specific to the needs within that community.
5. Focus on minimizing harmful consequences and not about judging or criticizing client behavior.

Using harm reduction approaches helps establish rapport because the counselor is working with client resistance and not targeting complete abstinence but, rather, focusing on ways to reduce the harm associated with use. Clients are also more likely to buy into alternatives to reducing harm that can lead to lowered resistance. Harm reduction approaches may lead to an increased motivation for change while examining usage patterns. The dialogue that follows illustrates how a counselor can use harm reduction with a resistant client.

COUNSELOR: Sam, you mentioned that you like to drink and have no intention of stopping. Spending time out with your friends at a bar is an important part of your social life.

SAM: Yes, so it's not really a good use of your time or mine if these sessions are going to be about you preaching to me about the ills of drinking. I don't have a problem and see nothing wrong with how I spend my time.

COUNSELOR: I appreciate your honesty. My intention is not to preach to you about your alcohol use. Rather, I was wondering if we might examine transportation options so that when you are out drinking you can avoid getting another DUI.

SAM: Well, now that I got my license back, I really don't want to risk losing it again. I need my license so I can get to work. I'm willing to discuss this, but I don't have an alcohol problem.

COUNSELOR: Fair enough. Let's brainstorm some possible transportation options together for the next time you go out with your friends. (*The counselor is not focusing on alcohol reduction at this point. The emphasis is on reducing the negative consequences associated with drinking and driving. By working together, the counselor is not only establishing the therapeutic alliance and working to reduce harm but also helping the client feel comfortable and laying a foundation for later trust and change.*)

Support Groups

Support groups serve a valuable function within the treatment process. Clients with SUDs or significant symptoms often find that as the condition progresses, they become alienated and isolated from family, friends, and coworkers as they spend more time in relationships with their substances and those who enable the behavior. When clients enter treatment, some are often faced with the hard and painful reality that life is empty and too much damage has been done to repair some relationships.

Another obvious issue is that clients often have poor social and coping skills, so engaging in social contact can be difficult. A common treatment goal is to learn to establish social and coping skills, which are critical components in developing and maintaining abstinence. Support groups play an instrumental role in facilitating and practicing these skills. The most common and best-known support groups are Alcoholics Anonymous (AA) and Narcotics Anonymous (NA), which are both based on a philosophy that addiction is a disease and individuals are powerless over its effects. In order to recover, they come to understand that they are powerless over the effects of the substance and cannot recover under their own volition and must turn their will over to a higher power.

The underlying mechanism of AA and NA are the 12 Steps, a series of sequential action steps that help individuals acknowledge their powerlessness over alcohol or other drugs and move toward accepting responsibility for their recovery. Several common misconceptions exist with respect to 12-Step meetings, including the belief that AA and NA are religious based and that these meetings serve as treatment for a person's addiction. Counselors encouraging clients to attend 12-Step meetings should educate clients on the purpose and mission of these meetings, as well as the variety of group options, meeting formats, and meeting structures.

Encouraging support group attendance should align with the client's stage of change. Clients may also report feeling discouraged or frustrated after attending a meeting and should be encouraged to try out different meetings to find out which best match their needs. Clients may find other groups like SMART (a four-point program that emphasizes self-reliance and empowerment), Double Trouble (for those dealing with mental health and SUDs), Al-ANON (for families of those with substance addiction), Overeaters Anonymous, Sex and Love Addicts Anonymous, and Gamblers Anonymous as options. Complete Activity 7.2, which focuses on enhancing your knowledge of 12-Step meetings.

ACTIVITY 7.2

Clients unfamiliar with 12-Step meetings are often reluctant or resistant to attend because they have misconceptions and fears about what to expect. Counselors can help allay client concerns by educating them about these meetings. For this activity, attend an open meeting of the 12-Step group of your choice. Note the sequence of events, format, and structure of the meeting and other pertinent information. How would you explain these meetings to a client?

Marital and Family Treatment

Marital and family treatment (MFT) has been an integral part of substance use treatment for decades. During this period, research findings have demonstrated a reciprocal relationship between problem drinking and family and marital discord (Merkouris et al., 2022). Stress and problems within the marriage and family can contribute to the development and continuation of problematic alcohol use. In cases where abstinence has been achieved, those individuals who continue to deal with unresolved marital and family conflicts may be at a higher risk for relapse. Merkouris et al. (2022) highlighted client characteristics that contribute to successful completion of MFT, including being older, entering counseling following a crisis, being employed, having a history of serious alcohol problems, having graduated from high school, and living together with one's family.

Counselor attributes that tend to lead to successful outcomes include addressing client and family members simultaneously; focusing on alcohol problems first; using empathetic listening and dealing effectively with intense anger during the initial stages of counseling; developing a structured, directive approach that includes establishing rules, boundaries, and consequences; and viewing the change process as long term. Treatment goals for MFT include eliminating substance use and addressing marital and family dysfunctional patterns. Initial sessions address negative interactions and feelings, and later sessions focus on increasing positive interactions and communications, developing healthy coping skills, implementing behavior change contracts, and integrating problem-solving skills.

Co-occurring Disorders and Crisis

Substance use disorder is the most common co-occurring disorder with severe mental illness. SAMHSA (2022) defines a *co-occurring disorder* as the concomitant occurrence of a substance use and mental health disorder. Co-occurring disorders were first identified in the early 1980s, and treatment outcomes often had poor results, as traditional approaches

for treating substance use were integrated into existing mental health services (Drake et al., 2004). Clients who are diagnosed with a co-occurring disorder may find that crisis situations not only present challenges to maintain abstinence from substances but also trigger symptoms associated with a mental health diagnosis. Counselors working with clients with co-occurring disorders must be competent in understanding each disorder as well as how to integrate treatment so as to ensure that the disorders are addressed simultaneously. Unfortunately, integrated treatment options within many communities may not be widely available (National Alliance of Mental Illness, 2017).

Counselors need to understand how substance use can present symptoms that mimic mental illness and other medical conditions. As discussed, effective treatment requires counselors to make an accurate diagnosis, but doing so is often unrealistic in crisis settings with short-term exposure to the client. Careful consideration should be given to conducting differential diagnosis, especially if there is evidence that mental health disorders or medical conditions may be present. Focusing on the evident symptoms is critical. For example, clients who use alcohol may also present with symptoms of depression. If alcohol is the reason for the depression yet the counselor fails to connect this information to a SUD diagnosis, the counselor may incorrectly diagnose the primary cause as a depressive disorder.

Similar scenarios involve clients who use stimulants such as amphetamines or cocaine, with symptom presentation manifesting as manic episodes or, depending on the amount of usage, psychotic episodes. If there is no co-occurring psychotic disorder, stimulant-induced psychosis will be temporary and subside as the individual comes off the high. When assessing individuals who present with psychosis, counselors should be aware that while symptom presentation for certain substances may mimic certain psychotic disorders, some differences exist. For example, clients with amphetamine-induced psychosis may present symptoms similar to those of clients with paranoid schizophrenia. However, clients with amphetamine-induced psychosis tend to present with visual hallucinations, whereas clients with paranoid schizophrenia tend to report auditory hallucinations.

More time is needed to make most other mental health diagnoses that are not SUDs. A client who presents with possible co-occurring mental health disorders while in a crisis center or with a crisis counselor can be documented as follows: "Client [demonstrates, presents, or reports] with symptoms consistent with an anxiety-related disorder." This documentation allows the next treatment provider to be aware of possible differential or co-occurring disorders.

Withdrawing from a substance can also create a crisis situation. For clients who establish tolerance and dependence on alcohol and benzodiazepines, withdrawal can have potentially life-threatening consequences. Complications from withdrawal of alcohol can result in serious medical conditions, such as seizures, stroke, tachycardia, and death. Counselors also need to be aware that overdose or sudden death due to substance use (e.g., alcohol poisoning, inhalant use, heroin overdose) can also result in acute crisis situations and trauma for family and friends. Educating clients and their families about drug actions, interactions, and synergistic effects, as well as symptoms of withdrawal, is an important treatment consideration.

As we now transition to multicultural perspectives related to substance use, complete Activity 7.3, which asks you to consider the clients with whom you possibly feel uncomfortable or fearful.

ACTIVITY 7.3

With what types of clients, characteristics of clients, or specific cultural differences would you feel most uncomfortable or even fearful? Discuss the reason(s) for your concern.

MULTICULTURAL PERSPECTIVES, SUBSTANCE USE, AND CRISIS

Learning Objective 7.5 Explain multicultural perspectives on substance use in crisis situations.

One of the many issues a counselor must consider is the significance of culture in shaping a client's identity. *Culture* refers to the values and beliefs that shape a person's worldview. Pedersen (1991) emphasized the importance of cultural diversity within the counseling relationship and the recognition that all counseling is multicultural counseling. Counselors working with clients with SUDs or significant symptoms and in crisis need to understand how the culture of substance use influences each client's worldview. Historically, treatment has focused on reduction or abstinence goals, often with little consideration for understanding the role that culture plays in etiology or maintenance of use. Relapse considerations often emphasize exploring both intrapersonal and interpersonal factors yet may not fully explore the implications of culture on substance use behavior.

Counselors must be knowledgeable about and implement multicultural counseling competencies in their practice. Ratts et al. (2015) outlined four multicultural counseling competencies—counselor self-awareness, client worldview, counseling relationship, and counseling and advocacy interventions—that each comprise four dimensions—attitudes and beliefs, knowledge, skills, and actions. Addictions counselors should develop self-awareness of attitudes and beliefs that may interfere with their work with clients, including any attitudes or beliefs associated with specific substances and substance usage patterns or behaviors. For example, if a counselor has specific attitudes or beliefs regarding clients exchanging sex for drugs, which may be a common practice within some drug-using subcultures, judgment and biases may spill over into counseling.

Addictions counselors should also be aware of the client's cultural worldview and the role substances play as part of that identity. For example, the counselor working with adolescent boys should understand whether certain substance use coincides with cultural expectations or rites of passage. Integrating cultural awareness into substance use treatment better aligns the treatment with the client's particular cultural worldview and context.

Counselors should also consider the significance of the client's cultural worldview during the assessment phase, when evaluating motivation and ambivalence. Motivation for continued substance use involves a complex and varied set of factors and circumstances that often manifest as client ambivalence and resistance. In some instances, ambivalence may be less about mood altering and getting high and more about maintaining a client's beliefs, customs, or rituals associated with substance use as it relates to cultural identity or tradition.

Assessing cultural meaning as it relates to substance use provides a broader understanding of the cultural dynamics that may help explain ambivalence. Counselors should maintain a cultural focus when working with clients through the five stages of change. Moreover, triggers for relapse behavior should be examined from a cultural perspective. Case Study 7.4 explores multicultural considerations when working with a client who uses substances.

CASE STUDY 7.4

Multicultural Considerations with a Substance-Using Client

Van is a 26-year-old multiracial man. Over the past 6 months, his use of alcohol and marijuana has increased significantly. His usage has coincided with his disclosure to his parents that he is gay, which conflicts with their beliefs and values. Van thinks he has let his parents down and feels depressed and isolated. He has been attending counseling for the past month and has experienced suicidal ideation. The ideation issue was dealt with in session 1 and determined to be low risk: no plan, specific means, and low motivation to self-harm. In this session, the discussion focused on understanding substance use and cultural expectations.

COUNSELOR: Van, we've been talking the last several sessions about your coming-out process and your parents' reaction. You knew telling your parents that you are gay was going to be hard, but you didn't think it was going to be this painful. They are very angry and disappointed.

VAN: I didn't want to disappoint them, but I couldn't keep living this lie. They were pressuring me to settle down, get married, and have a family. They believe I'm making a choice to be gay as an act of defying them. The weight of their anger and disappointment sits heavy in my heart.

COUNSELOR: I'm sensing your pain as we talk and the heavy toll it's taking on you. Tell me more about your parents' cultural beliefs about being gay.

VAN: In my culture, being gay has always been viewed as some sort of defect and has been looked down on. My parents don't support this lifestyle and believe it's sinful. They made it clear that they would never accept this lifestyle in any of their children. I was always fearful because I knew I was gay for a long time.

COUNSELOR: You knew deep down that identifying as gay meant you'd never be accepted by your parents, so you turned to alcohol and drugs as a way to numb the pain.

VAN: Well, it's a lot easier to drink and get high to take away the pain than to feel it. Yet I still feel it and sometimes it only magnifies the feelings. Maybe I should just give them what they want because I see how much it is hurting them and me. It's so confusing and I don't know what to do.

COUNSELOR: You're questioning yourself and thinking giving them what they want will make everything better. (*Van looks dejected, like he is feeling lost and confused. It is clear he is struggling with his truth versus family and cultural expectations.*)

VAN: Yeah.

COUNSELOR: (*Using an advanced empathic statement to make the implicit message explicit*) Van, as we've been talking today and over the last several sessions, might it be that you can't accept yourself as a gay man unless your parents can also accept you in that way? That you have struggled for a long time with the realization that being true to who you are means alienating your family? Knowing that you're disappointing them has led to alcohol and drugs as a way to numb the pain. Yet the pain never goes away.

VAN: (*Van sits for several minutes in silence reflecting on the counselor's response*) I just wanted the pain to stop, but how can it as long as I keep denying who I am? I can't control how they see me, but I can control how I see myself and how I want to live my life. I'm really tired of feeling like I have to be something I'm not. I need to find other ways of dealing with this problem, and I realize using alcohol and drugs is not solving it.

COUNSELOR: You want to be true to yourself and find healthier ways of dealing with the pain. We can use our time together to explore what cultural expectations and messages you want to embrace while finding healthier ways to deal with painful emotions.

A multiculturally competent counselor examines any value conflicts and biases that arise as clients tell their stories and addresses those conflicts and biases. The counselor also attempts to see the presenting problem from the client's worldview. Strategies for change will likely require exploration and resolution of conflicting cultural messages and healthier ways of coping as clients move toward embracing their identity.

Discussion Questions

1. What other strategies for change should be integrated into the work with Van?
2. How should the counselor go about dealing with the substance use concerns?
3. What other cultural issues do you believe should be considered and why?

OVERVIEW OF THE ELEMENTS OF CRISIS INTERVENTION RELATED TO SUBSTANCE USE

Learning Objective 7.6 Identify elements of crisis intervention related to substance use, including safety and self-care, diagnostic considerations during a crisis, and essential substance use–specific crisis intervention skills.

The purpose of this final section is to review several specific elements of crisis intervention related to substance use, including safety and self-care, diagnostic considerations during a crisis, and essential substance use–specific crisis intervention skills.

Safety and Self-Care

Clients with SUDs or even symptoms can be particularly vulnerable to the effects of crises. Crises can result in greater risk for increased use of substances or lapse/relapse behaviors. Those who engage in substance use tend to be at a higher risk for crisis or trauma while under the influence (Zinzow et al., 2010). When individuals with SUDs or symptoms experience crisis or trauma, the recovery process and the work of the counselor can be challenging (SAMHSA, 2014). Driessen et al. (2008) reported that individuals with SUDs and symptoms who experience trauma tend to have poorer treatment outcomes than those who lack a history of trauma. Clients who increase substance use because of crisis or trauma create potentially dangerous outcomes for themselves (e.g., overdose, suicide) and the community (e.g., homicide, DUI-related accidents).

During a crisis, the counselor simultaneously addresses a client's crisis or trauma response and the potential risk for or actual substance use. Ensuring that the client is safe is a critical first step in crisis intervention. To effectively address safety concerns in relation to the client, the counselor establishes rapport, trust, and genuineness and creates a safe environment. Creating an environment where the client is protected and out of harm's way can help deescalate the crisis situation. The counselor assesses the risk of suicidality or instrumental suicide-related behavior and takes a directive role when the client in crisis presents danger to self or others. Voices from the Field 7.3 describes one counselor's experience with substance use and crisis issues.

VOICES FROM THE FIELD 7.3

Substance Use and Crisis Issues

Meghan Brown

Recently, while sitting at my desk, one of the counselors on my team came to my office and noted that a 15-year-old girl in her group was "acting out" and refusing to leave the women's restroom. I could hear the young girl yelling at her counselor and to all in the group: "Leave me alone . . . my life is crap!" The counselor returned to her group while I stood outside the restroom, nonchalantly pouring myself a cup of coffee. After a few moments, the young woman came out of the restroom, joined me at the kitchenette, and clearly stated, "No one can understand me . . . no one is me. I'd like to be a butterfly because butterflies are free." I immediately recognized that she was under the influence.

I asked the young woman to go into an open group room with me. I told her that she did not have to talk to me, but she was more than welcome to talk about anything as I was interested in what had made her so upset. During the time she spoke, I was able to conduct an informal mental status exam, explore her judgment, assess her potential for suicide, and observe her behaviors to further assess what substance she had taken. The conversation was very interesting but frightening due to hearing the girl repeatedly note that she has no reason to live, she wants to die, she will overdose to die, she has a history of psychiatric hospitalizations, she does not like her parents, and she enjoys her relationship with drugs. She remained disorganized in her thoughts, glared at me, was agitated, and demonstrated trouble regulating her breathing. Every few moments, it appeared as though she was gasping for air. I observed that her trouble breathing was not related to her crying but potentially more a result of her using a system depressant. I quickly asked her counselor to give the rest of the group a break and to come and sit with her in the open group room while I contacted her parents.

The young woman's parents arrived at the facility within minutes of our phone call. They were concerned and immediately defensive about their daughter's condition. The mother repeatedly noted that she does not think her daughter uses alcohol or other drugs, despite her being involved in chemical

(*continued*)

dependency treatment and testing positive in her most recent drug screen. I informed the parents that an ambulance had been called for their daughter due to her difficulty breathing and her high risk for suicide. The primary counselor and I worked with the client and the parents to try to keep everyone calm and provide the emergency medical technicians (EMTs) and police officer with the appropriate information.

The young woman repeatedly refused to cooperate with the EMTs and the police officer. As a result of her refusal, the police officer told the young woman's parents that he could not take her to the hospital. Her parents, although initially resistant to the idea that their daughter was under the influence, noted that they were afraid to take her home. They informed me that the police officer refused to "pink slip" their daughter and was going to send her home with them. It was at that point that I reminded the police officer what I had already told the EMTs, that this young woman was clearly under the influence, stated she had no reason to live, and had the means to overdose if she wanted to die. It was only then that the officer agreed to mandate the young woman be taken to the hospital for a 72-hour hold.

The entire length of this event was a total of 45 minutes, but looking back on it all, it seemed more like 3 hours. The situation required legal knowledge, sound ethics, and clinical judgment. I needed to be able to recognize the symptoms of a client under the influence of alcohol and/or other drugs, the risk factors and signs of suicide, and how to keep a client calm in a crisis situation to get the information needed to obtain appropriate care for the client. Had I not spent some time with the client to hear the words "I want to die" and understand the means she had to kill herself, I would not have been able to gain the support of law enforcement to have the young woman mandated to receive care in the emergency department.

Assessing and placing the client in the appropriate level of care can help reinforce the importance of safety by aligning the client's needs with applicable treatment and interventions. Determining where the client is in the stages of change can be valuable so the counselor can begin to conceptualize appropriate and applicable interventions. Use of personal support and empathy can be instrumental throughout the crisis. Employing harm reduction measures may be valuable for clients who are in the precontemplative or contemplative stages of change.

Once the crisis situation has deescalated, the counselor can enhance safety in a variety of ways, including employing grounding exercises to help clients establish some control when they feel overwhelmed; creating a structured counseling setting with specific routines or rituals; creating opportunities for safe discussion; identifying triggers or behaviors that create feelings of not being safe both in the moment and in general; and developing a safety plan (SAMHSA, 2014).

Counselors also take steps to ensure their own safety when dealing with clients who are in crisis (see Chapter 2). Counselors need to be aware of the dangers of working with clients who pose a threat to public safety or who have a history of violence, especially those who may also be under the influence of a substance (e.g., phencyclidine or other forms of hallucinogens resulting in distortions in one's perception of reality). Counselors should gauge the risk in working with clients who pose safety concerns and can take steps to minimize the risk by screening high-risk clients, avoiding seeing clients when there is a lack of physical security available, recognizing warning signs of violence, avoiding working alone during in-home counseling, and establishing rules prohibiting weapons in session.

Working with clients who have SUDs and significant symptoms creates unique challenges for counselors. Counselors who constantly face client resistance, ambivalence,

motivational concerns, and relapse issues may question their effectiveness in facilitating change. Working with this population requires counselors to integrate self-care techniques into their daily routine to minimize the potential for burnout. Self-care techniques that can be effective when working with clients with SUDs or significant symptoms include engaging in interests and hobbies; having a strong support network; establishing work boundaries; attending to one's physical, mental, and spiritual needs; learning effective time management; engaging in continued professional development and training; attending counseling to address any maladaptive patterns or interpersonal issues that hinder emotional or psychological well-being; and seeking out supervision.

Diagnostic Considerations During a Crisis

A crisis situation can exacerbate and confound the diagnostic process, especially if the client has co-occurring issues. The first consideration is to differentiate among symptoms that are due to the acute crisis situation and symptoms that present as part of the pathology. Differentiating crisis symptom presentations from pathological symptoms helps counselors explain to clients that their experiences are normal by associating those symptoms with the crisis, which can help minimize the client's stress. Explaining to clients how crisis situations can manifest physically and psychologically helps clients better understand the symptom presentation. Furthermore, explaining to clients how crisis situations can magnify affective states and trigger increased substance use may help normalize their experience.

As mentioned, the purpose of diagnosis is to determine an appropriate course of treatment. Differentiating between crisis symptoms and other presenting mental health and substance abuse symptoms enables the counselor to properly assess and triage the situation while also identifying appropriate interventions and normalizing the client's experience. Caution is warranted when providing a mental health diagnosis that may require more information than can be obtained in an assessment during crisis intervention.

Essential Substance Use–Specific Crisis Intervention Skills

The intervention skills used for clients with SUDs and symptoms tend to also apply when these clients experience crises. Use of attending behaviors and active listening skills is important in crisis situations. Creating warmth, expressing genuineness, and creating an atmosphere of trust, safety, and acceptance not only target the motivational factors inherent with treating SUDs and symptoms but also help clients in crisis deescalate from the acute stressor and feel more at ease and comfortable with the counselor. Using a directive approach, conducted in a compassionate and empathic manner, allows for clarification and understanding of the crisis threat so that appropriate courses of action can be taken.

Table 7.1 provides a number of online and print resources that will expand your knowledge of the interplay between crises and substance abuse.

TABLE 7.1 Resources on Substance Use and Crisis

Online Resources

Harm Reduction Coalition: https://harmreduction.org

National Alliance on Mental Illness: https://www.nami.org/Home

ProjectKnow: https://projectknow.com

Substance Abuse and Mental Health Services Administration: https://www.samhsa.gov

Print Resources

American Society of Addiction Medicine. (2013). *The ASAM criteria: Treatment criteria for addictive, substance-related, and co-occurring conditions* (3rd ed.). Author.

Miller, W. R., & Rollnick, S. (2013). *Motivational interviewing: Helping people change* (3rd ed.). Guilford Press.

Publishers Group. (2022). *Street drugs: A drug identification guide 2022*. Author.

Rudd, M. D. (2006). *The assessment and management of suicidality*. Professional Resource Press.

Substance Abuse and Mental Health Services Administration. (2022). *2021 National Survey on Drug Use and Health (NSDUH) Methodological Resource Book, Section 2: Sample Design Report*. https://www.samhsa.gov/data/report/2021-nsduh-sample-design-report

Substance Abuse and Mental Health Services Administration. (2014). *Trauma-informed care in behavioral health services* (Treatment Improvement Protocol [TIP] Series 57, HHS Publication No. [SMA] 13-4801). Author.

SUMMARY

Substance use has been an integral part of the human experience and cultural identity throughout time. The reasons that people use drugs vary; however, specific biological, psychological, and sociological circumstances are key factors. Unfortunately, as the data reveal, only a fraction of those needing substance use treatment actually receive care. Failing to address SUD issues or significant symptoms can be particularly problematic for clients who are also dealing with crisis issues. Numerous models have been developed in an attempt to explain the complex phenomenon of addiction. Still, there is no consensus on the causality of addiction. Counselors having knowledge of different models and theories of addiction are better equipped to address the array of etiological considerations.

To understand and treat clients with SUDs or significant symptoms, especially those who are dealing with a crisis, counselors must be competent in all aspects of assessment and diagnosis. Proper assessment and diagnosis are instrumental in establishing an effective treatment protocol and resources for the client. Numerous screening and assessment instruments exist, with the clinical interview being one of the most commonly used assessment tools. Treatment must consider both the level of motivation and the nature and severity of the crisis situation. Treatment considerations, especially for clients facing crisis situations, need to factor in admission and placement decisions as outlined by ASAM's level of care criteria.

Understanding the stages of change within the crisis context can assist the counselor in developing both an effective crisis response and a quality therapeutic alliance once the crisis is managed. The implementation of crisis models helps the client return to a precrisis level of functioning and is the first step in the treatment process, especially when a client's crisis response involves an increased use of substances or a relapse.

Furthermore, counselors should think about how to structure subsequent sessions based on response to the initial crisis session. Establishing a strong therapeutic working alliance between counselor and client is critical throughout the crisis period. Other

considerations include assessing client and counselor safety, client support, risk (on a continued basis), how substance use factors into risk behavior, specific plans once the crisis subsides, and motivation to change.

Also, counselors must understand that the risk of relapse is high when dealing with SUDs or symptoms and can become magnified when clients are in crisis. Helping clients identify and address triggers is a key part of treatment, as is helping clients understand the difficulty and inherent challenges and obstacles in maintaining abstinence in both the short and long term.

Other matters that must be considered during counseling include marriage and family concerns, co-occurring disorders, the integration of a harm reduction philosophy and approach into the overall treatment model, and the involvement of support systems such as 12-Step groups. Throughout the treatment process, counselors must apply multicultural counseling competencies to ensure that the counselor's cultural attitudes, values, and biases do not interfere with care; the client's worldview is recognized; and culturally appropriate intervention strategies are implemented.

This chapter highlights a number of considerations when working with clients with co-occurring crisis and substance use issues. Substance use can not only lead to crisis situations but also exacerbate them, so counselors need to navigate crisis situations and substance use issues simultaneously. Effectively assessing client motivation to change means counselors understand applicable and appropriate strategies, skills, and approaches that align with the client's current state of change. Furthermore, assessing the stage of change and motivation can have important implications once the crisis has been addressed, especially if substance use precipitated the crisis event. Finally, counselors who can create a therapeutic environment based on genuineness, trust, acceptance, and warmth go a long way toward establishing therapeutic rapport while building a foundation that enhances short- and long-term client change.